



basic education

Department:
Basic Education
REPUBLIC OF SOUTH AFRICA

DRAFT LEARNING PROGRAMME FOR CHILDREN WITH SEVERE TO PROFOUND INTELLECTUAL DISABILITY

17.6.2017

Department of Basic Education

Sol Plaatje House
222 Struben Street
Private Bag X895
Pretoria 0001
South Africa
Tel: +27 12 357 3000
Fax: +27 12 323-0601

120 Plein Street
Private Bag X9023
Cape Town 8000
South Africa
Tel: +27 21 465-1701
Fax: +27 21 461-8110

<http://www.education.gov.za>

Submit comments to:

Mrs Marie Schoeman

Schoeman.m@dbe.gov.za

EXECUTIVE SUMMARY

LEARNING PROGRAMME PACKAGE

The Learning Programme (LP) forms part of the Draft Policy for the Provision of Quality Education and Support for Children with Severe to Profound Intellectual Disability (SPID). The Learning Programme can be used by anyone (parents, teachers, caregivers, therapists, etc.) involved or working with children with severe to profound intellectual disabilities (SPID).

The Learning Programme (LP) for children with SPID assists teachers/caregivers to;

- Assess the learning needs (strengths and weaknesses) of children with SPID;
- Plan integrated programmes and activities to ensure optimum development of skills and community integration; and
- Integrate the LP into the Daily Programme by means of descriptive examples.

The use of the LP is supported by the designated District Based Support Teams in each of the Provinces. Each education district would be responsible for the training, implementation, monitoring and evaluating of the LP where ever it would be used. (See the above mentioned Policy document for children with SPID). The LP is further supported by a practical manual.

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ACRONYMS

AAC	Augmentative and Alternative Communication - various methods to enhance communication by using speech, gestures, signing, objects, pictures, etc.
ADL	Activities for Daily Living
CP	Cerebral Palsy
DBE	Department of Basic Education
DOH	Department of Health
DSD	Department of Social Development
EWP 6	Education White Paper 6 on Inclusive Education and Training System (2001)
ISP	Individual Support Plan
LP	Learning Programme (for Children with Severe to Profound Intellectual Disability)
NCF	National Curriculum Framework for Children 0 to 4 years
NCS	National Curriculum Statement
NELDS	National Early Learning and Development Standards for children birth to four years
PID	Profound Intellectual Disability
SIAS	Policy on Screening, Identification, Assessment and Support
UNCRPD	United Nations Convention on the Rights of Persons with Disabilities
WHO	World Health Organisation

GLOSSARY

Activities for Daily Living (ADL)	A series of basic activities performed by individuals on a daily basis necessary for independent living at home or in the community also known as self-care skills, such as: independent toileting; personal hygiene; (bathing, oral care); dressing; eating; etc.
Baseline Assessment	Determines what the child is able/unable to do
Bilateral	Both sides of the body working together, for example bilateral movement of the arms and hands in order to clap.
Child centred	Focus being on the needs, strengths and interest of each child as an individual.
Cognitive development	Refers to thought and learning processes and how these develop dependent on the development of the brain.
Co-morbid medical conditions	One or more additional disorders co-occurring with primary disability.
Constant support	The child can be involved in an activity, but needs support through-out, i.e. Can assist in dressing self, but needs help to perform task.
Duty of care	In the context of children with SPID, the duty of care encompasses an awareness and avoidance of the dangers of inappropriate physical handling, feeding and other care practices.
Education	Children with SPID are able to develop skills, learn and apply knowledge with the appropriate input and benefit from educational stimulation aimed at developing their maximum potential. The focus of this education is on the development of communication, personal and self-care, practical and conceptual skills within their natural daily environment, rather than academic skills.
Fully dependent	Cannot care for self, is fully dependent on caring, i.e. feeding, cleaning, etc.
Hand-over-hand support	The person teaching the skill places his hand over the hand of the child and may actually manipulate the hand of the child in performing the wanted action.
Holistic	Not focusing only on learning, but also taking in account the child's socio-economic circumstances, health, school environment, etc.
Inclusive Education	Is broader than formal schooling and occurs in all settings (home, school, community) within formal and informal modes and structures. All children can learn and have different learning needs. All children have differences whether due to age, gender, ethnicity, language, class, disability or HIV

status. Education structures, systems and learning methodologies should meet the needs of all learners. Attitudes, behaviour, teaching methodologies, curricula and the environment have an influence on learning outcomes. Maximising the participation of all children in the culture and the curricula of educational institutions and uncovering and minimising barriers to learning. By developing children's individual strengths and enabling them to participate critically in the process of learning, can empower them.

Integrated approach The subject components communication, life skills and visual perception and cognitive skills are not presented in isolation when teaching, but is integrated within programmes, i.e. during the morning ring all subject components are addressed.

Kinaesthetic learning A learning style in which learning takes place by the child experiencing concepts with the body and senses.

Laterality Developing of awareness that the body has two (2) sides, namely a left and right side.

Locomotor movement Locomotor movement is when you actually move from one place to another, such as crawling, walking, running, swimming, flying, sliding,

Non-locomotor movement Is moving on the spot without going anywhere, such as twisting, bending, swaying, stretching or turning body.

Midline crossing An individual's ability to efficiently move and use his/her hands, feet and eyes of the one side of the body in the space of the other side of the body – thus crossing the imaginary line that divides the body into a left and right side.

Minimal support The child is for the most part independent, but needs support from time to time, i.e. the child can dress him/herself, but needs support from time to time, such as fastening buttons.

Multi-disciplinary approach Having more than one person involved in assessing, planning and intervening. Not just a carer or teacher's responsibility, but could have speech, occupational and physiotherapist, psychologist and parents involved.

Muscle contracture A muscle contracture is a permanent shortening of a muscle or joint.

Perceptual development The ability to be aware/perceive and make sense of the world.

Person centred planning An approach designed to assist someone to plan their life and support. It is used to enable people with disabilities to increase their self-determination and independence.

Proprioception Any movement of muscles and joints. The sense of movement through space, e.g. one is able to close ones eyes and still be aware of the

position of each body part as well as movement of the limbs through space.

Responsive environment

A responsive environment is one in which the child can obtain responses both from people and from seeing things happen as a result of what they do.

Sexuality

Incorporates characteristics of who a person is as a human being as a man or a woman. It includes biological, psychological, social and cultural components.

Shared exploration

Exploring activities with support, for example hand-over-hand feeding – where adult holds child's hand to support.

Spatial orientation

Defines our natural ability to maintain our body **orientation** and/or posture in relation to the surrounding environment (physical space) at rest and during motion. Genetically speaking, humans are designed to maintain spatial **orientation** on the ground.

Structured free play

The child with SPID's play environment should be structured, whereby appropriate toys/objects should be provided, whilst supporting the child throughout the play activity with the necessary verbal/ physical guidance, also allowing spontaneity from the child.

Supported participation

The child needs support in order to participate, for i.e. supported seating and/or hand-over-hand support to enable participation in table activities.

Systematic instruction

A well-designed plan of how a skill will be taught.

Transversal team

Referring to inter-sectoral collaboration as well as transdisciplinary teams of health and education professionals to address the complex needs of children with SPID.

Vestibular movement

Movement for regulation of muscle tone, posture, balance, spatial experience, eye movements and emotional stability. It includes movement patterns such as forwards/backwards; sideways and up/down.

Visual perceptual skills

A set of skills used to gather information from the environment which are integrated with the other senses. This is done while incorporating all the integrated information with other things, such as past experiences, motivation and development in order to derive understanding and meaning from what are being experienced.

SECTION 1

INTRODUCTION TO THE LEARNING PROGRAMME (LP) FOR CHILDREN WITH SEVERE TO PROFOUND INTELLECTUAL DISABILITY (SPID)

1. BACKGROUND

The South African Constitution, Act 108 of 1996, protects the right of every child to access quality basic education. For learners who experience barriers to learning and development, equal access requires the provision of appropriate support services depending on the type of disability. The Learning Programme (LP) needs to be read in conjunction with the Policy Framework for the Provision of Quality Education and Support for Children with Severe to Profound Intellectual Disability (SPID). The LP has been developed to address the needs of children with SPID to ensure their meaningful participation in education.

Children with SPID function at a very low level of adaptive functioning and have high support needs. They exhibit significant developmental delays and frequently experience multiple disabilities. It is recognised that children with SPID may only encounter educational activities and develop awareness of their surroundings, which forms the minimum functioning level as portrayed in the LP.

Education and skills development focus on increasing the individual's capacity for learning self-care and communication skills. There will also be a need to develop motor control, as well as skills of social interaction with adults and peers according to their abilities. Furthermore these children will frequently require assistive devices (i.e. wheelchairs, standing frames, and specialised seating etc.) that need to be fitted and maintained. These devices must be available to the child at school/centre and at home. The processes for these designated devices are described in the Policy Framework for children with SPID.

The LP is aligned with the National Early Learning and Development Standards (NELDs) for children birth to four years and the National Curriculum Framework for children (NCF) from birth to four years old. These frameworks adopt a holistic approach and includes education, health and social services. Children develop individually and have different support needs, which may include restricted sensory and motor functioning, and will therefore need constant support and supervision. In line with the National Policy on Screening, Identification, Assessment and Support (SIAS), an Individual Support Plan (ISP) must therefore be developed for each child to customise his or her learning programme.

These children require a multidisciplinary approach because of their high support needs. There is also a significant impact on families, who should be supported and consulted throughout the education process. One of the greatest supports to family is the community, but this is only effective if the child is accepted and integrated within the community. Therefore the LP outlines opportunities for community integration where possible. Health professionals, social workers, teachers, caregivers

and parents should work as a team in a collaborative manner to address health, social and educational needs.

The LP incorporates self-care, communication, social and all aspects of learning that are integrated into a structured daily programme, thus creating a responsive environment.

Progression is likely to be very slow. In this LP there is an emphasis on practice and repetition within learning, creating the opportunity for constant growth. Many of these children find it difficult to generalise learnt skills to different situations, and therefore need exposure to practice these skills in different settings. The guiding principle must be to encourage and motivate each child, irrespective of the degree of their disability.

2. OVERVIEW

Through this LP, read in conjunction with the Policy Framework for children with SPID, the Minister of Basic Education prescribes the minimum norms and standards for children with SPID.

This LP is in line with the Constitution, the National Curriculum Statement (NCS) (2011), National Early Learning and Development Standards for Children Birth to Four Years (NELDS) (2004), the National Curriculum Framework (NCF) for Children from Birth to Four (2015), Education White Paper 6 on Special Needs Education: Building an Inclusive Education and Training System (2001), the Policy on Screening, Identification and Assessment and Support (SIAS) (2014), Integrated School Health Policy (ISHP) (2012) and The Care and Support for Teaching and Learning (CSTL) Programme (2008).

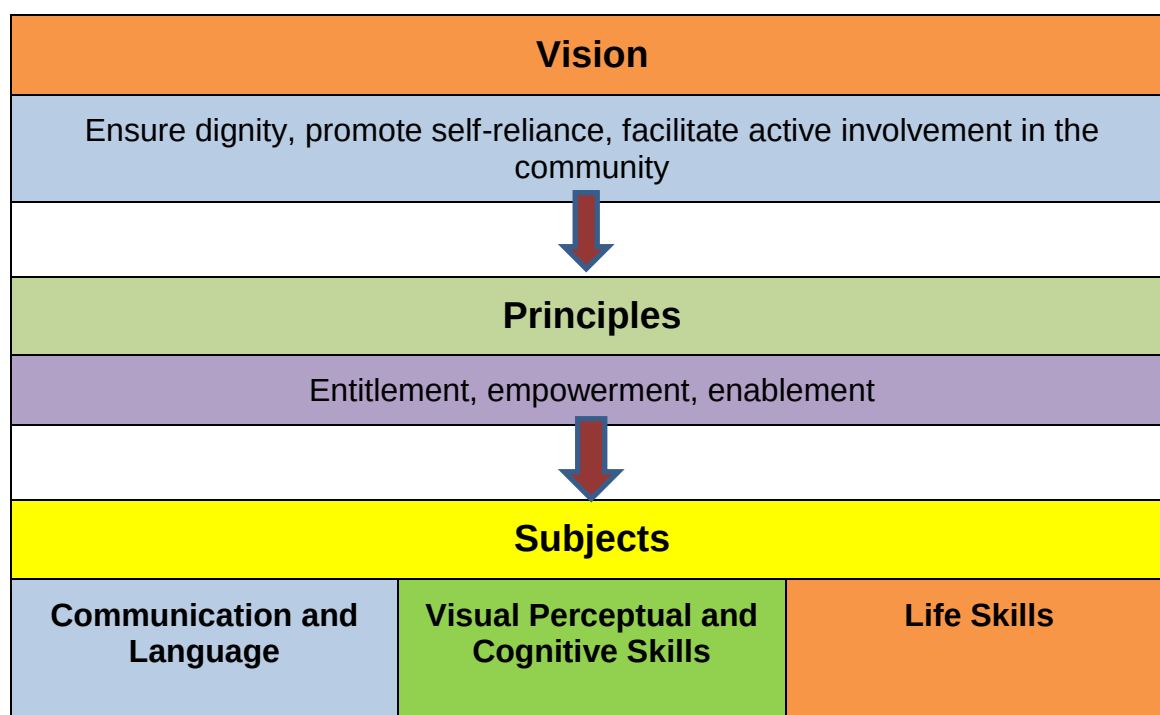
It also translates the key principles outlined by the United Nations Convention on the Rights of Persons with Disabilities (2007), specifically Article 10, 23, 24 and 26. These Articles emphasise the rights of children to:

- be educated in an inclusive education system, close to their home;
- have access to appropriate health services including assistive devices;
- be integrated into family and community life; and
- be protected against cruel and inhumane treatment.

All children have the opportunity for developing to their full potential and to have a sense of dignity and self-worth.

The policy also guides the practical implementation of the following sections of the Social Development and Health legislation, namely:

- a) The Mental Health Care Act (Act No.17 of 2002) which states that people with SPID need partial self-maintenance under close supervision, limited self-protection skills in a controlled environment.
- b) Section 11 of the Children's Act (2007) which makes provision for children with SPID, indicating that they need to be provided with parental, family or special care where appropriate.
- c) Chapter 5, Section 76 of the amended Children's Act (2007) which identifies the support related to children with SPID as an issue that is relevant for Partial Care. It is the role of the DBE to collaborate with the DOH and DSD to ensure that children with SPID have access to education programmes in schools and/or Partial Care Centres.
- d) It furthermore provides a response to the High Court judgement in the case of the Western Cape Forum for Intellectual Disability (WCFID) vs. the Government of the Republic of South Africa and the Government of the Western Cape Province, that upholds the rights of children with SPID to receive:
 - basic education;
 - protection from neglect;
 - equality; and
 - basic human dignity.



3. VISION

The policy recognises the right of every child with SPID to enjoy a full and decent life in conditions which:

- are safe;
- ensure dignity;
- promote self-reliance, and
- facilitate the child's active participation in the community.

(Adapted from Article 23 of the Convention on the Rights of the Child, 1989)

4. GENERAL AIMS OF THE LEARNING PROGRAMME (LP)

The general aim of the LP for children with SPID is to guide the planning and development of child and group specific learning programmes that would enable these children to meet their potential. These children require extensive support for the management of their disabilities and complex medical needs.

(a) The National Curriculum Statement (NCS) Grades R-9 specifies the knowledge, skills and values worth learning in South African schools. This LP is aligned to the NCS and aims to ensure that children with SPID acquire and apply knowledge and skills in ways that are meaningful to their own lives. In this regard, the LP promotes knowledge in local contexts, while contributing to lifelong learning.

(b) **The Learning Programme serves the purpose of:**

- equipping children with SPID, irrespective of their socio-economic background, race, gender, physical ability or intellectual ability, with the knowledge, skills and values necessary for self-fulfilment and meaningful participation in society as citizens of a free country;
- developing children's understanding of the world around them, social interactions and relationships;
- integrating learning, therapy and health needs through an interdisciplinary approach;
- facilitating the transition of youth with SPID from education settings to opportunities for lifelong learning and community participation;
- providing a profile of a youth's competencies to family, caregivers and possible employers to enable meaningful participation;
- being sensitive to issues of diversity such as poverty, inequality, race, gender, language, age and other factors;
- valuing indigenous knowledge systems: acknowledging the rich history and heritage of this country as important contributors to nurturing the values contained in the Constitution; and
- providing quality education that is credible and efficient and in line with international best practice for children with SPID.

(c) The LP for children with SPID is based on the following **principles**:

- **Entitlement**: the right to high quality learning experiences and interaction;
- **Empowerment**: the recognition of their basic human need to realise their self-worth and self-esteem;
- **Enablement**: the recognition of their potential to learn from a quality interactive learning environment;
- **Social transformation**: ensuring that the educational imbalances of the past are redressed, and that equal educational opportunities are provided for all sections of the population;
- **Active learning**: encouraging an active and critical approach to learning, rather than rote and unbiased learning of given truths;
- **Adapting knowledge and skills** in the LP to enable maximum development;
- **Progression**: showing progression from simple to more complex tasks in finely gradual steps to allow plotting of progression;
- **Values process**: The process of learning is as equally valued as the achievement;
- **Strengths-based approach**: the focus is on an asset-based approach that recognises strengths and capabilities of children and build on these;
- **Human rights, inclusivity, environmental and social justice**: infusing the principles and practices of social and environmental justice and human rights as defined in the Constitution of the Republic of South Africa.

(d) **Inclusivity** should become a central part of the organisation, planning and teaching at learning centres. This can only happen if all teachers and caregivers have a thorough understanding of how to recognise and address barriers to learning, and how to plan for diversity. The key to managing inclusivity is ensuring that barriers are identified and addressed by all the relevant support structures within the learning centre community, including caregivers, teachers, District-Based Support Teams, Institutional-Level Support Teams, parents and Special Schools as Resource Centres. Teachers and caregivers should create an enabling and responsive learning environment which recognises and builds the strengths and skills of individual children in a gradual manner (see the Department of Basic Education's *Guidelines for Responding to Diversity in the Classroom*, 2012).

5. EXPECTED OUTCOMES OF EDUCATION FOR CHILDREN WITH SEVERE TO PROFOUND INTELLECTUAL DISABILITY (SPID)

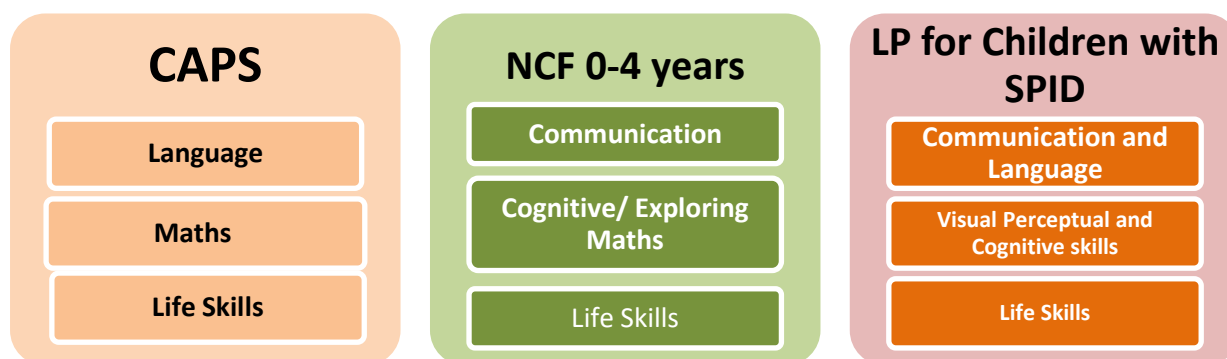
Through the use of this LP children with SPID will have improved or maintained:

- Access to lifelong learning.
- Maximum family and community integration.
- An optimal level of independence and communication.
- An optimal level of social and emotional well-being.
- An optimal level of participation in activities of daily living.
- A measure of progress through the performance criteria stages of encounter, awareness, attention, response, engagement, participation, involvement and attainment.
- Acknowledgement and respect of their rights and dignity.

6. STRUCTURE OF THE LEARNING PROGRAMME

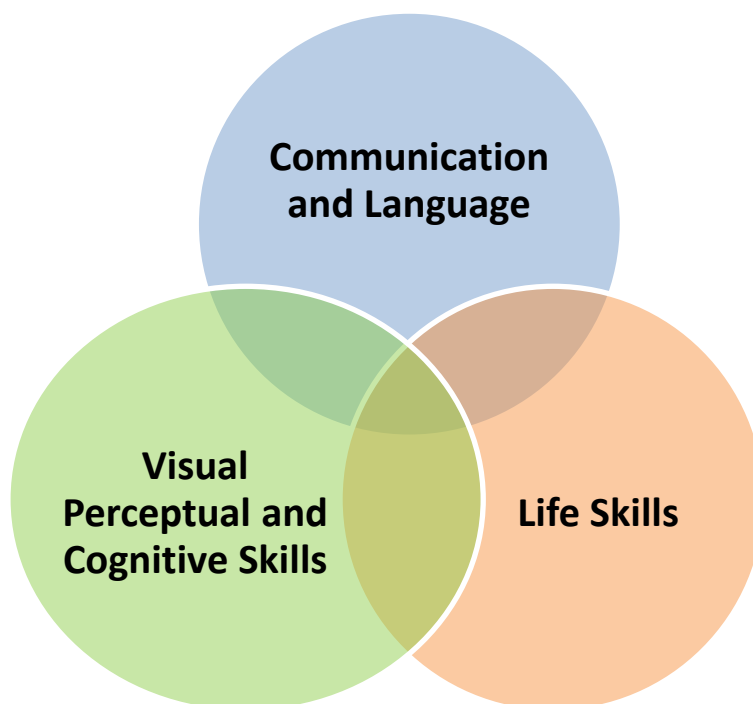
The LP for children with SPID focuses on the care, development and learning needs of this group from the age of three until the age of 18 years. The LP makes reference to typical child development at a Pre-grade R level (from birth to four years) with the focus being on the acquiring of basic developmental skills rather than reaching developmental milestones. The LP framework is further guided by the Curriculum and Assessment Policy Statement, Grade R-9 (CAPS), (2011); National Early Learning Development Standards (NELDS) (2009) and National Curriculum Framework 0-4 years (NCF) (2015) to ensure alignment with national education standards.

The 3 Subject components within the LP are guided by, aligned to and adapted from the CAPS, Gr R-3 and the NCF 0–4 years. It is further suggested that in the planning and development of child and group specific learning programmes, the LP should be used in conjunction with the NCF 0–4 years, because children with SPID often function at this level of development. However, their chronological and physical age must also be taken into account, hence the need for this LP specifically for children with SPID.



GUIDING PRINCIPLE OF THE LEARNING PROGRAMME

Development of skills, learning and applying of knowledge takes place through the continuous integration of communication, visual perceptual and cognitive skills and life skills that the child is exposed to daily, including experiences in his/her natural environment or daily programme.



7. SUBJECTS AND TIME ALLOCATION

7.1 Communication and Language

This subject component addresses the following:

- (a) Continuous development of both verbal and symbolic communication in speech or gesture.
- (b) Understanding of simple instructions or gestures.
- (c) Expression of own desires and emotions largely through non-verbal, non-symbolic communication.
- (d) Enjoyment of relationships with well-known family members, caretakers and familiar others through the use of communication.
- (e) Initiating and responding to social interactions through the individual child's mode of communication.
- (f) Encourage choice making.

7.2 Visual Perceptual and Cognitive Skills

- (a) Development of conceptual skills that involves the physical world rather than symbolic processes.
- (b) Use of objects in a goal-directed manner for self-care, play and vocational/domestic tasks.
- (c) Basic perceptual skills, such as matching and sorting based on physical characteristics.
- (d) Sensory, motor and perceptual skills to explore and participate in their environment.
- (e) Basic problem solving skills.

7.3 Life Skills

- (a) Improve the level of independence in all aspects of self-care, health and safety through encouraging participation.
- (b) Use of objects in a goal directed manner for Activities of Daily Living (ADLs) (personal management, school work, play and social). Assisting with some basic activities of daily living at home and at the learning centre, like carrying dishes to the table).
- (c) Developing basic skills to encourage vocational activities with appropriate levels of support and supervision.
- (d) Encouraging the participation in recreational activities and play exploration, that are developmentally and age appropriate.
- (e) Improving and encouraging behavioural modification to reduce maladaptive behaviours.
- (f) Education regarding appropriate sexuality and interpersonal relationships.
- (g) Encourage interpersonal relationships with family, caregivers community members and peers.

7.4 Instructional time

The time allocation is divided equally across subjects as there will be integration and play-based learning. The daily programme is structured to provide numerous interactive experiences that give opportunities to develop skills, learn and apply knowledge in all subjects. A total of 27.5 instructional hours is prescribed per week.

Subjects	Hours
Communication and Language	9
Visual Perceptual and Cognitive skills	9
Life Skills	9.5
Total	27.5 hours

SECTION 2:

METHODOLOGY AND INTEGRATION IN THE IMPLEMENTATION OF THE LEARNING PROGRAMME

1. UNDERSTANDING THE INDIVIDUAL NEEDS OF THE CHILDREN WITH SPID

Education of children with SPID is based on a *holistic and integrated approach* that is child centred and individualised to each child's development and learning needs. It is based on determining the level of functioning, with the focus on the child's strengths and interests. Children grow and develop through exposure to daily life experiences and learn in interaction with their environment in an integrated way across the developmental areas. The three subject components of this LP are delivered in an integrated way within a daily programme. It is therefore better to highlight the broad competencies that children with SPID are expected to acquire from a programme and activities, rather than the specific skills and abilities from a particular domain of development.

Children should experience a wide range of learning encounters. It is important to believe in each child's potential for further development and learning so that achievement and attainment are always made possible. All experiences could not result, and should therefore not be expected to result, in immediate attainment. What is important is that all children are entitled to become involved in a rich and wide range of educational and learning experiences. No child should only be limited to a watered-down practical programme of routines simply because their potential for attainment is observed to be limited.

A variety of different settings is needed to create all the *experiences* that children with SPID will have. What they feel, do, hear and see, form an important part of the LP. These experiences must be planned for within the context in which they are being cared for and educated. Continuous reflecting on and evaluating of the child's performance in the learning context would determine adjustments needed in the planning of child specific programmes. This should ensure that each child is given the best opportunities to be able to develop according to his or her own pace and abilities, providing the best suited support needed.

In a group setting (special care centre classroom; special school classroom; etc.) the baseline assessment of each child would provide relevant information to allow grouping of children according to level of functioning. When children are grouped together in groups of similar ability, it allows for better implementation of learning programmes, which allows differentiation within the group.

1.1 Early intervention

Right from birth, children are continuously developing and learning. Children with SPID also have that ability, but their disability acts as a serious barrier in the process, resulting in delayed attainment of developmental stages. In order for children with SPID to develop skills and learn, they need specific support which could enable them to improve further. It is therefore extremely important to identify the barriers that they experience at a very early age, so that the necessary support and early intervention can be provided to ensure that development of necessary skills and learning can take place.

1.2 A responsive environment

Children with SPID need a **responsive environment** and the **opportunity to make choices** in order to develop social, communicative and cognitive skills. A responsive environment is one in which the child can obtain responses both from people and from seeing things happen as a result of what they do.

Sometimes children elicit limited responses from their environment because they are unable to communicate. The educational environment needs to be structured in such a way that it is sensitive to small responses. Children learn to communicate through meaningful social interaction. A responsive environment embodies the respect and dignity with which all people, whatever their age, abilities or disabilities, ought to be treated. Studies show that parents and other caregivers find it more difficult to respond to children with disabilities than to non-disabled children, and are more likely to experience negative or unsatisfying interactions with them. This may result in them being isolated with little opportunity for interaction. Continuous repetition of encouraging actions might just result in a slight smile/ twitch of the mouth or he/she might turn his/her head towards the sound. But if no opportunities are created that could result in a response, no improvement is likely to occur.

1.3 Making of choices

To give someone the opportunity to make choices shows respect towards that person and enhances their dignity, because it shows that what matters to this individual has meaning and they do have some control over their own world.

1.4 Learning through play

“Play is an activity that a child **CHOOSES** to do and has **FUN** with whilst doing. Through play, a child uses his/her **SENSES** to explore and learn” (Case-Smith, J. 2005.)

Play enables children to develop the following skills:

- Gross motor skills: balance, posture, ball skills, eye hand coordination;
- Fine motor skills: manipulation skills, different hand grips, eye hand coordination;
- Sensory development: hearing, vision, touch, movement, smell, taste;
- Cognitive (thinking) skills: Basic concepts, perceptual skills, problem solving;
- Emotional development;
- Social skills, e.g. turn taking;
- Self-care skills, e.g. tooth brushing and dressing; and
- Social inclusion and community integration.

Types of play:

Children's play activities change over time and reflect their development (Case-Smith, J. 2005).

1. Sensorimotor and exploratory play: during infant years, and is most applicable to the child with PID;

- learning new skills and exploring their own body and surroundings; and
- developing a sense of cause and effect

2. Constructive play: Develops from pre-school years throughout life

- play activities have a definite outcome, e.g. arts and crafts; and
- cognitive skills develop, including emerging problem solving skills.

3. Symbolic play: begins at 2 years and peaks at 5 years

- dramatic and fantasy play (difficult for the child with SPID)

4. Social play: Begins very early as interaction between infant and mother

- partake in complex social games by age 3 years;
- role play to learn about cultural norms, e.g. pretend to be a doctor or mother; and
- games with rules during school going years.

Each child with SPID is dependent on and entitled to supported play-based learning to ensure the best possible opportunities to develop to his/her maximum potential. It is important for the carers and family to structure the child with SPID's play environment, and provide appropriate toys/objects

whilst supporting the child throughout the play activity with the necessary verbal/physical guidance, also allowing spontaneity from the child – this is referred to as **Structured Free Play** in the Daily Programme.

Each subject component of the LP includes integrated play activities, aimed at enhancing previous mentioned skills.

**A WORTHY EDUCATIONAL ENVIRONMENT IS RESPONSIVE, STIMULATES
SOCIAL INTERACTION AND PLAY-BASED LEARNING AT ALL TIMES**

2. BASELINE ASSESSMENT FOR PLANNING

When planning for children with SPID, it is necessary to know the specific needs of each individual. A baseline assessment of each child will determine what the child is able/unable to do. Emphasis is placed on the child's ability rather than his/her inability, within a strengths based approach. This baseline assessment will be used as the point of departure for determining the support needed to enable further improvement and learning. Section 5 on Assessment gives more detail about methods and available tools. (See Annexure 5: Example of an assessment tool).

3. DEVELOPMENT OF THE INDIVIDUAL SUPPORT PLAN

Each child with SPID has unique needs, strengths, interests, background and current circumstances. Planning for each child should be individualised by means of an individual support plan (ISP).

3.1 Developing an ISP

- (a) The ISP is developed using the baseline assessment and all relevant information from the child's file as well as information from relevant people in the child's immediate support system. It is essential that a multidisciplinary approach, that includes the parents, is adopted.
- (b) The ISP should have specific goals/objectives for the child's development. **These goals should be Specific, Measurable, Achievable, Realistic and Time-specific (SMART).** It should outline responsibilities of different role players in supporting learning. For example, transport and the procurement and maintenance of assistive devices might also be included in the ISP. It is better to select a small number of achievable goals at each stage of developing the ISP and then to evaluate and build on successes during ISP revision.
- (c) An ISP is developed over a period of time and is continuously reviewed. An ISP supports the child's individual needs and guides specific planning to address these needs on an ongoing basis. It should be part of the lifelong planning of goals as the child enters different

phases of his/her life. The ISP can be used to monitor the success of the intervention and to ensure accountability for those who have been given responsibility for support tasks.

Examples of specific needs to be addressed in the ISP, for children with SPID:

- **Gross motor:** The infant, toddler and young child with SPID and multiple physical disabilities require child specific intervention in order to optimize their ability to acquire postural control, develop optimal levels of voluntary control of movement and develop an array of physical skills. Individualized programmes with specific goal setting could include carrying and handling, massage and mobilizing, positioning and specific activities to develop the identified physical skill/s. The content of such a programme should be determined by a Physiotherapist, keeping in mind that in a transversal team, the Occupational therapist could also be consulted. Close collaboration between all professionals from all sectors involved in service delivery, is of utmost importance (see policy).
- For children with **physical disabilities**, correct seating and positioning are critical. Not only in promoting physical well-being, but to maximize their opportunities to engage and participate in all situations where learning and applying knowledge takes place. Care should be taken that caregivers use safe seating and positioning methods that have been approved and supported by physiotherapists/ professionals.
- Children with SPID and multiple physical disabilities have an array of **co-morbid medical conditions**. Contractures and deformities lead to pain and it becomes an increasing barrier to optimal level of independence as well as to caring and positioning. Poor conditions of care leads to infections, pressure sores and further disabling diseases. Many of these co-morbid health conditions are mental and behavioural in nature and could prevent a child from optimal participation to structured physical activities and from refining physical skills. The behaviour can become a physical hazard to the child as well as others.
- **Optimal nutrition** as well as **social and emotional well-being** impacts on the acquisition and development of physical skills. Appropriate screening and monitoring in this regard should be the role of all role players involved in service delivery to children with SPID. Referral to and collaboration with the Department of Health (DOH) and the Department of Social Development (DSD) should be a priority to all stake holders.
- **Sexuality:** Appropriate social and sexuality behaviour.
- **Self-care:** Gaining maximum independence with regards to toilet training and routines.

- **Communication:** Many children with SPID are unable to communicate verbally. It is vital to get a picture of what the child can understand and to observe the child carefully to see what behaviours they use in order to communicate. They might be able to use eye movements or different noises which at first seem meaningless but might be used by the child to convey certain messages.
- **Community integration:** Every ISP needs to include planning that promotes the inclusion of the child within their family and community.

4. WORKING IN A CLASSROOM SETTING

- a) As mentioned in the above section, it can be seen that children with SPID need ISPs to be able to develop to their optimal potential. This can be challenging in a classroom setting. It is therefore important to group children according to their level of functioning, but also keeping in mind their chronological age. This does not mean that all children with e.g. Cerebral Palsy should be grouped together. Keeping in mind their cognitive ability and physical abilities, they can still be included and exposed to all classroom activities with appropriate adaptations and differentiation of activities.
- b) Baseline assessment would provide the necessary information to group children according to their level of functioning. Within a group of similar ability, differentiated teaching methods should be used to address specific needs. Planning needs to be class or group specific, which means that within a special care centre with for example 5 groupings, the same programme (e.g. "My body") would be adapted according to the needs and level of functioning of the specific group, which would include the level of support needed.

EXAMPLE OF HOW TO ARRANGE GROUPS:

AWARENESS GROUP Children fitting into this group are in the very early stage of awareness. Focus should be on encouraging them to move from being passive to becoming aware. All activities and/or responses are initiated and are fully prompted and supported by others. This group would include children who are at a level of encounter and awareness and moving towards responsiveness.
TRANSITIONAL GROUP This group of children show interest in their environment, but still need prompting with adult initiation of activities and communication. This group would be at a level of response and engagement.
INTERACTIVE GROUP Activities for this group of children are aimed at those beginning to control their environment, develop curiosity and initiate activities and communication. This group would be at a level of participation, involvement and moving towards attainment.

PLANNING AND DEVELOPING GROUP SUPPORT

Awareness Group	Transitional Group	Interactive Group
1 carer : 1 child	1 carer : 2 – 3 children	1 carer : more than 4 children
• Full physical support needed • Working towards responsiveness	• Initial support → retract support • Co- active exploration • working towards engagement	• Mostly verbal support • Working towards attainment/accomplishment of a skill
• Sensory-based activities	• Self-care and routines	• Self-care • Basic home/work skills • Integrated activities

These groups should be flexible. Children may be in one of these groups or classrooms where every child in this group is working towards either responsiveness (Awareness Group), towards engagement (Transitional Group) or working towards attainment (Interactive Group). On the other hand there might be children from all three groups clustered together in one classroom. Differentiation can then take place by subdividing groups, use of workstations and adapting activities for different learning areas according to their ability to participate. (See Annexure 6: Integrating the LP into the Daily Programmes).

5. STRUCTURING THE DAILY PROGRAMME

Children with SPID have the potential to develop different skills and learn. They **have the right to quality opportunities to develop to their full potential**. The idea of a holistic and integrated approach is to use opportunities for learning within the child's natural daily environment, by means of a well-structured and functional Daily Programme. This applies to children wherever the LP is being delivered, whether it be at home in a special care centre or other delivery sites. The daily programme outlines the sequence of activities, the time and time allocation for the activities and the learning aims for the activities.

When planning a well-structured and functional Daily Programme, **time constraints** should be taken into account. **Learning experiences should be functional** and fit into the child's activities for daily living (ADLs) and self-care. In order to master some of these skills, the child might be dependent on constant support, motivation and praise within the given opportunities. A basic Daily Programme should ensure that learning opportunities are created during the course of each day to develop communication and language, life skills and visual perception and cognitive skills. The Daily Programme needs to be well-structured, starting at home from when the child awakes or at the learning site from the point of being dropped off until going home again.

EXAMPLE of a BASIC DAILY PROGRAMME
Routine Activities for Daily Living (ADLs) and self-care activities (Washing hands; breakfast; toileting, brushing teeth)
Morning Ring (Communication and Learning)
Gross motor activities - outside/indoors
Snack time (ADL) and routine activities
Discussion time (theme/topic)
Table activities (Fine motor; perceptual skills; integrated creative arts)
Structured free play (outside/indoors)
Music and Story time
Lunch and routine ADLs and self-care
Going home

6. INTEGRATING RESOURCES, SPECIALISED EQUIPMENT AND LEARNING TEACHING AND SUPPORT MATERIAL (LTSM)

To support the implementation of the LP resources, specialised equipment and LTSM need to be delivered in an integrated manner. Training and support will be made available from Departments of Basic Education (DBE), Social Development (DSD) and Health (DOH) as well as from NGOs in the procurement, use, integration and monitoring of resources, specialised equipment and LTSM.

NGOs involved with caring and support of children with SPID for numerous years, have developed resources and training. (Inter alia, Pathways - Pretoria, WCFID, Cape Mental Health, Hambisela, to name only a few (see Annexure 7: Resource list of NGOs).

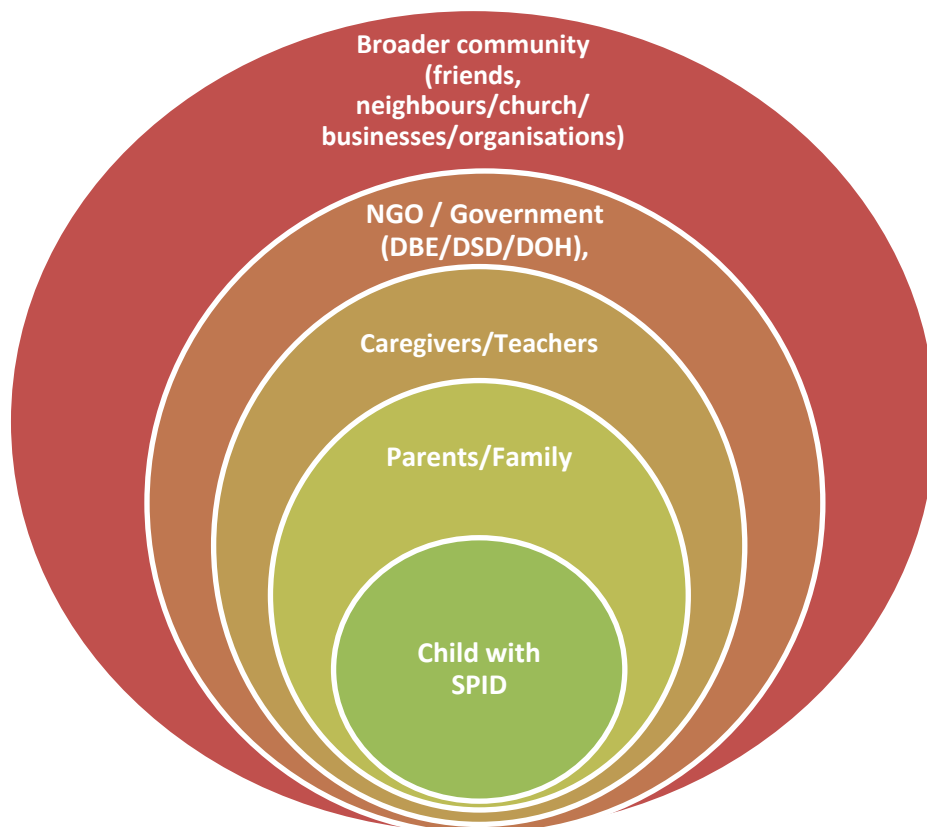
7. LIFELONG LEARNING

Education of children with SPID does not imply 'formal' schooling, but rather refers to a process of lifelong learning. Children with SPID are entitled to quality education and must therefore be given the opportunity to develop to their full potential and progress according to their own level of functioning and ability. With the necessary support, children with SPID can develop improved levels of functioning and independence, although this may take longer to accomplish. The development of skills are not restricted to age or function, but should rather be seen as developing and learning throughout life. While the child may only reach a certain level of cognitive functioning (vertical development) it is also important to look at the horizontal level of developing where further enrichment and stimulation can contribute to reaching optimal potential. The skills that the child has acquired may be carried out with greater competence and with less support as the child grows older. People with profound intellectual disability will require lifelong care and support with personal care tasks, communication, and accessing and participating in community facilities, services and activities. Lifelong learning is therefore dependent upon the broader community's involvement in having measures in place to accommodate these children with SPID, as well as their families.

The challenge lies in linking the LP for children with SPID with all programmes of care and education provided for them and their families by the lead DBE, DSD and DOH - to support seamless transitions. This would also include integrating children and young adults, who are out of school and those exiting from the education system, within their community.

8. PARTNERSHIPS FOR LIFELONG LEARNING

IT TAKES A COMMUNITY TO RAISE A CHILD



When working with children with SPID, collaboration between all role players involved with the child is very important. Who should all play a role? Referring to the Care, Support, Teaching and Learning (CSTL) Model/Programme of the Department of Basic Education (DBE), it shows that it is the responsibility of a broader community to educate a child. The child is part of the community and should not be excluded by the community.

The child with SPID will have a lifelong dependency on the community's support and involvement. Therefore the community should put measures in place to address the support needed. This requires collaboration and integration between all involved during different stages of the child's life (see diagram above). This collaboration between all partners can ensure that children with SPID receive the most meaningful development and learning experiences. This could include integration into the community after exiting an education system. Any support should always be person centred, age appropriate, holistic and integrated within the child's natural everyday environment. The LP is designed to follow the child with SPID at any stage of his/her life.

SECTION 3

THE DESIGN OF THE LEARNING PROGRAMME

THE 3 SUBJECT COMPONENTS WITHIN THE LEARNING PROGRAMME:

- COMMUNICATION AND LANGUAGE;
- VISUAL PERCEPTUAL AND COGNITIVE SKILLS;
- LIFE SKILLS

1. OVERVIEW OF THE 3 SUBJECT COMPONENTS AND CONTENT

Communication and Language	Visual Perceptual and Cognitive Skills	Life skills
Support communication and language development using whatever mode the child with SPID is capable of.	Develop visual perceptual skills and cognition in order to appropriately function within his/her environment according to his/her optimal level of independence.	Support for physical, emotional and social well-being. Develop optimal levels of independence with regards to activities of daily living (ADLs)
In order for communication/ language to develop/ improve the following PRE-LANGUAGE skills need to develop: • Joint attention • Eye contact • Cause and effect • Turn taking • Imitation LISTENING SKILLS AND UNDERSTANDING • Respond to sound • Respond to and follow instructions EXPRESSIVE LANGUAGE • Verbal: Using sounds and words to communicate needs.	VISUAL PERCEPTUAL READINESS: • Alertness & response • Visual attention • Eye movements: - Focus and tracking BODY AWARENESS: • Body concept • Body image • Body schema LATERALITY MIDLINE CROSSING	SENSORY: • Hear • See Information senses: Primary for learning. • Taste and smell • Touch Social senses: Primary for social skills and relationships. • Proprioception • Vestibular Regulation senses: Movement is primary for attention and regulation. GROSS MOTOR Developmental milestones: 0-6 years chronological age • Head and neck control • Trunk control and sitting • Mobility

Communication and Language	Visual Perceptual and Cognitive Skills	Life skills
Non-verbal: Using gestures, picture and symbols to communicate needs.	CONCEPTUAL SKILLS: • Size • Colour • Shape • Left & Right • Numbers Learning sequence: 1) Awareness 2) Match 3) Identify 4) Sort 5) Name 6) Patterns & Sequence VISUAL PERCEPTUALSKILLS: • Visual and Object Constancy • Visual Figure Ground • Spatial Skills - Position in Space - Visual Spatial Relations • Patterns and sequencing • Time sequencing	• Balance and agility • Upper limb control • Functional activities Physical skills: Chronological age 7-14 years • Locomotor/non locomotor (moving from one place to another) • Perceptual motor • Rhythm • Co-ordination • Balance • Spatial orientation • Laterality • Sports and games Physical well-being: Chronological age 15-18 years • Independence in daily living • Recreational activities • Sports and games SELF-CARE AND ACTIVITIES FOR DAILY LIVING (ADL) • Dressing • Feeding • Toileting • Washing hands/face • Brushing teeth • Blowing nose/wipe mouth • Orientation to time and daily routines SENSE OF SELF AND SEXUALITY

Communication and Language	Visual Perceptual and Cognitive Skills	Life skills
		• Body and gender awareness (also see Visual Cognitive Skills) • Personal hygiene (also see Self-Care Skills) • Nature of relationships • Privacy rights SOCIAL SKILLS AND RELATIONSHIPS • Relating to familiar people • Relating to friends/peers • Relationships EMOTIONS • Understanding and expressing emotions appropriately

It needs to be emphasised that even though the 3 subject components are represented separately in the LP, development of skills, learning and applying of knowledge should take place in a holistic and integrated way as it relates directly to how children develop and learn.

2. STRUCTURING THE LEARNING PROGRAMME

In the Learning Programme each of the three subject components are addressed separately (see SECTION 4: OUTLINE). Each subject component is structured in two sections, namely content and examples of activities. The content of each subject component is sequential beginning with simple skills and moving to more advanced skills.

The examples of activities used in the LP have been adapted to fit the specific and diverse needs of children with SPID, taking into account that children with SPID can chronologically be older, but function within the 0–4 age group. An example of a generic activity is provided. This is followed by the adaptation of these activities, as well as what can be expected of children grouped according to functioning ability in 3 specific groups, namely Awareness group, Transitional group and Interactive group. (See illustration below).

Example of an activity to support and enhance development of skills

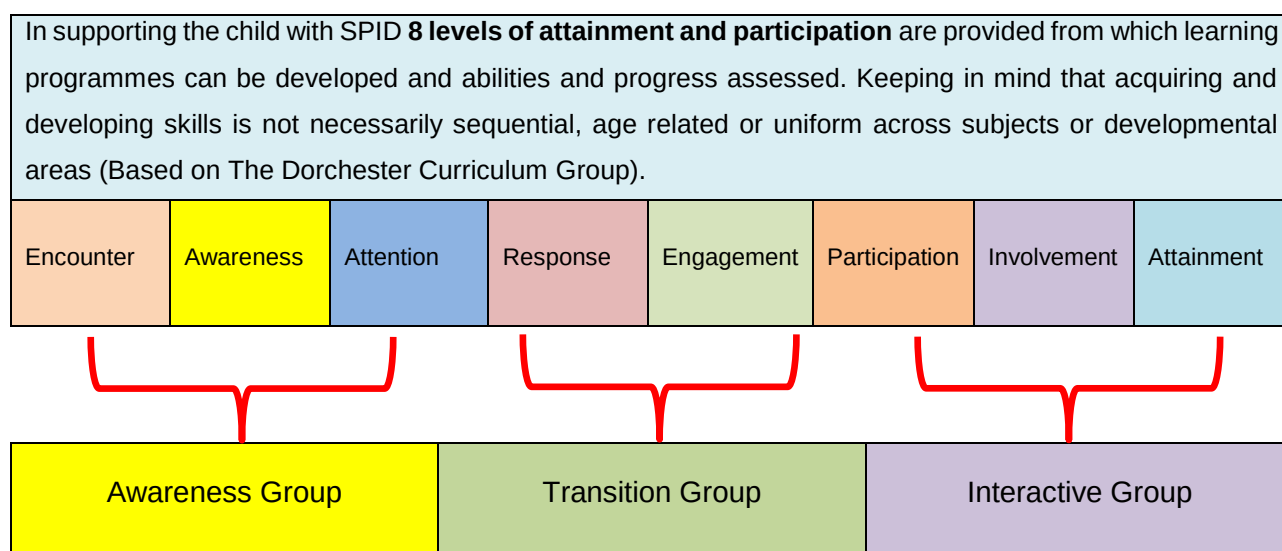
Each subject component is structured in two sections	
Content	Activity
	Adapted for Awareness group:
	Adapted for Transitional group:
	Adapted for Interactive group:

Additional activities and integration ideas for the Daily Programme is also provided.

Within the CAPS, children are grouped according to age and grades from Grade R–12. The LP makes provision for 3 age groups according to chronological ages. Within the 3 age groups provision is made for the 3 functioning groups.

Level/Phase?	Chronological age	3 Functioning Groups		
Junior	0-6 years	Awareness group	Interactive group	Transition group
Intermediate	7-14 years	Awareness group	Interactive group	Transition group
Senior/vocational	15-18 years	Awareness group	Interactive group	Transition group

The following diagram guides the child's allocation to a functional group:



This functional grouping leads to streamlined support. Within these functional groups further differentiation according to each child's ISP can and must take place.

Checklist of progressive stages (adapted from The Dorchester Curriculum Group, UK)

Range of performance	Explanation of the child's behaviour or performance
ENCOUNTER	• Is present during an experience or activity, but might be passive or even resistant. • May show simple reflex responses such as startling or crying at sudden movements or noises. • Any participation has to be fully prompted by other person. • May withhold attention or tolerate a shared activity.
AWARENESS	• Appears to show awareness to something happening (activity or experience), • Might also have a brief period of interrupting/ stopping a self-absorbed activity or movement. • Or show excitement during certain activities. • It happens by chance with fleeting focus, but child looks away again after a moment.
ATTENTION	• Begins to pay attention and respond to what is happening, but not always consistently, for example by showing signs of surprise, enjoyment, frustration or dissatisfaction.
RESPONSE	• Consistent response to familiar people, events and objects. • May have different reactions when introduced to new activities/ experiences, such as withholding attention.
ENGAGEMENT	• Shows more consistent attention to, and can tell the difference between, specific events in their surroundings, for example, communicate interest and enthusiasm through body language - smiles of recognition; focused looking or listening; body movement by turning to locate objects, events or people, following moving objects and events through movements of their eyes.
PARTICIPATION	• Engages in sharing, taking turns and the anticipation of familiar sequences of events, for example, by smiling, vocalising or showing other signs of excitement. • These responses may be supported by staff or other children.
INVOLVEMENT	• Active participation, attempts to reach out, join in, do things or comment in some way on the activity itself or on the actions or responses of the others, for example, by making needs known through hand and arm movements, seeking eye contact with others, or by speaking, signing or gesturing.
ATTAINMENT	• Have gained, practised or generalise skills, knowledge, concepts or understanding related to the child's experience of the activity, context, for example, if you turn on a switch of a fan, you will become cool.

3. THE DAILY PROGRAMME

It is important to remember that the content of the LP is a flexible tool and should be used as a guideline in the planning and implementation of programmes that need to be integrated within the DAILY PROGRAMME (see diagram below).

The integration should be implemented in such a way that all children's needs are met, while the necessary support is provided to enable each child to develop to his/her full potential. (See Annexure 5 of integrating the Learning Programme into the Daily Programme).

INTERGRATION OF THE 3 LEARNING AREAS WITHIN THE DAILY PROGRAMME			
Daily Programme	Communication and Language	Visual Perceptual and Cognitive Skills	Life Skills
Arrival Packing out bag;	• talks about home/ride • listens and responds	• perceptual skills • memory	• routines – self-care
Morning ring Greeting; weather; news; birthdays; days of the week songs; visual sequence – daily routine; Bible/moral story;	• stories • songs • listens and responds • participate (actions/words) • talks about object, picture	• perceptual skills – • matching • counting, colours • rhymes • sequencing – time (weather; news; birthdays; days of the week; visual daily routine chart) • memory	• movement; • songs and music; • routines • emotional skills • social skills
ADL – Breakfast, toileting, washing hands, brushing teeth	• stories • songs • listen and responds • participate in routine and discussion (actions/words) • talks about object, picture	• perceptual skills • matching • counting • colours • rhymes • sequencing of actions/routines	• self-care routines • gross and fine motor • emotional skills • social skills • basic work skills
Gross motor/ Physical education Balls/swings/ obstacle course	• listens and respond • participate (actions	• perceptual skills • body awareness • laterality skills • spatial skills • matching • counting • colours	• emotional skills • social skills • gross and fine motor skills • sensory skills

Daily Programme	Communication and Language	Visual Perceptual and Cognitive Skills	Life Skills
Discussion time (Seated on the carpet)	• stories; • songs; • listens and respond • participate (actions/words) • talks about object, pictures, topic	• perceptual skills • figure ground • spatial skills • matching • counting • colours • rhymes	• emotional skills • social skills • gross and fine motor skills • sensory skills
Table activity 1 (fine motor/art/play dough/puzzles)	• listens and respond • participate (actions/words) • talks about object, pictures, activity	• perceptual skills • matching • counting • colours • rhymes • sequencing	• emotional skills • social skills • gross and fine motor skills • sensory skills
Snack time	See ADL	See ADL	See ADL
Table activity 2 (optional according to functionality of group/ length of day)			Could include domestic/ work skills
Structured indoor/outdoor play construction/sand play/water play/pretend play/games	• listens and respond • participate (actions/words) • talks about activity	• perceptual skills • figure ground • body awareness • matching • counting • colours • rhymes	• emotional skills • social skills, e.g. turn taking • gross and fine motor skills • sensory skills
Music and movement Social ring/dancing/musical instruments/singing	• stories; • songs; • listens and respond • participate (actions/words) • talks about object, pictures, topic	• perceptual skills • matching • counting, colours • rhymes • attention • concentration • sequencing of actions/dance	• emotional skills • social skills • gross and fine motor skills • sensory skills

Daily Programme	Communication and Language	Visual Perceptual and Cognitive Skills	Life Skills
Storytime	• stories; • songs; • listens and respond • participate (actions/words) • talks about object, pictures, story	• perceptual skills • matching • counting • colours • rhymes • sequencing • attention • concentration	• emotional skills • social skills • gross and fine motor skills • sensory skills
Packing up and going home	• listens and respond • participate (actions/words) • talks	• perceptual skills •	•

4. TOPICS

Within the Daily Programme, the use of topics is suggested as a means to integrate the content from the 3 subject components where possible and appropriate. Topics should address the child's natural environment, beginning with the child self, immediate family, friends and surroundings (e.g. myself – people around me- my environment). Topics and sub-topics should be presented according to the child's needs, level of functioning and be age appropriate. Ready-to-be-used topic guidelines and integrated activities are provided in the manual. (See Annexure 7: Example: Topic: Me)

AGE GROUP	TOPIC	SUB-TOPIC
0-6 years	Me	• Self-Identification; - respond when called by name - identify self in mirror - identify self in photo • Gender
	My body	• Explore own body • Self-care skills and activities • Privacy
	My senses	• What I see, hear, touch, taste and smell
	My family	• Recognise own family members • Knows family members
	Carers, friends and others	• Recognise • Knows
	Food	• Basic mealtimes

AGE GROUP	TOPIC	SUB-TOPIC
7-14 years	Me	- Personal details (name; age; - What makes me special (name/ language/ gender)
	My body	- Identify body parts - Name body parts - Function of different body parts - Caring for my body, I am special - I have feelings (see social and emotional) - Keeping my body healthy - Self-care
	My senses	What I see, hear, touch, taste and smell
	People around me	- My family - My carers - My friends
	Clothes	- With my body - With weather
	My home	- My family - Clothing - Rooms in the house - Safety - My pets
	Food	- Mealtimes - Basic everyday foods - Fruit - Vegetables
	Animals	- Pets - Farm animals
	Seasons	- Integrated throughout year - Clothes
	People in my community	- Doctor/ hospital - Dentist - Fireman
	Transport	- Safety - Types of transport
	Safety	- In general – body/environment - Stranger danger and abuse - Road safety
	Healthy environment	- Clean environment - Recycling

AGE GROUP	TOPIC	SUB-TOPIC
15-18 years	Me	• Hygiene • Social acceptable behaviour in society • Sexual awareness • Abuse no more
	My home and family	• Home vocational skills/ helping at home
	People in my community	• Carers • Friends • Doctor; dentist • Shop • Community workers; etc.
	Food	• Every day food/meals • Fruit • Vegetables • Healthy food
	Transport	• Safety • Types of transport
	Animals	• Pets • Farm animals • Wild animals • Sea Animals
	Healthy environment	• Clean environment • Fire • Recycling
	Safety	• Safety in the home/workplace • Fire (fire drills) • In general – body/ at home/ environment • Stranger danger and abuse • Road safety
	Shopping and money	• Helping with shopping

Topic resources:

- Pre-school Themes: www.preschoolthemes.co.za;
- www.sparklebox.com

SECTION 4

THE 3 SUBJECT COMPONENTS AND CONTENT OUTLINE

1. COMMUNICATION AND LANGUAGE

1.1 WHAT IS COMMUNICATION AND LANGUAGE?

Communication is a critical life skill that enables all human beings to make their needs known and establish relationships with other human beings. The manner, amount or sophistication of the communication may vary between individuals. Communication is an activity that involves sending, receiving, processing and comprehending concepts and messages that are presented includes verbal, non-verbal and graphic. People use speech, vocalizations, facial expressions, body language, conventional gestures (e.g. pointing), objects and different means to express themselves. Language can be divided into expression (production) and reception (comprehension).

Children with SPID who may not be able to use speech to communicate effectively may use other modes which are dependent on their physical, sensory and cognitive abilities. A multiple means of communication benefits all individuals, irrespective of the severity of their disability. Some of these means can be without support, such as manual signs, facial expressions or gestures. Others need to be supported such as the use of pictures (picture communication -PCS), objects or a variety of augmentative and alternative communication (AAC) methods.

1.2 THE SPECIFIC AIMS OF COMMUNICATION AND LANGUAGE:

- Continuous development of symbolic communication in speech or gesture.
- Understanding of simple instructions or gestures.
- Expression of own desires and emotions largely through nonverbal, non-symbolic communication.
- Enjoyment of relationships with well-known family members, caretakers and familiar others through the use of communication.
- Initiating and responding to social interactions through gestural and emotional cues.

To be able to communicate and interact effectively a person needs to be able to respond, to listen and to understand communication.

The most effective way of improving the communication skills of children with SPID is:

- teaching within their natural environments and routines;
- providing multiple opportunities to practice communication skills; and
- ensuring responsive partners and systematic instruction.

Teaching within their natural environment and routine

Listening and communicating are crucial to all learning and in order for these skills to be developed effectively, it is important that continuous opportunities for developing and learning should be provided according to each child's specific needs, level of understanding and development and interests. The best way to ensure the development of communication would be within the child's natural daily environment and routine in which he/ she is actively involved. The development of language and communication skills is based on integration with Life-skills and Visual Perceptual and Cognitive skills by means of play-based learning. This occurs within a well-structured and functional Daily Programme. As communication interaction opportunities develop naturally, the teacher / caregiver intervenes as needed supporting and enhancing the child's communication skills.

- **Providing multiple opportunities to practice communication skills:**

Children with SPID often need numerous opportunities to practice skills. In most typical activities within the daily programme, many opportunities naturally exist for communication interaction to take place. Repetition to practice skills can be created additionally. A relatively easy way to motivate children to engage in communicative interactions is by the offering of choices. Instead of giving a child a particular item (food, drink, toy) allowing the child to make a choice creates an opportunity for the child to communicate. The choice can be given using speech, the items itself, representative pictures or signs. Different methods of input should enhance the child's understanding of choice-making and offer the child a variety of means of expressing the choice (e.g. signing, looking, pointing to a picture, grabbing an item). Even if the carer/ teacher is sure of what the child's choice would be, the child must be given the opportunity to communicate his choice, to reinforce communication and not necessarily the choice. This gives the child more independence and he learns that he is in control of the interaction. Making choices throughout the day can help developing self-determination. It is important to recognise and use these opportunities. Choices could include what clothes to wear, food to eat, activities to engage in, items to play or work with, who to play with, places to go, items to purchase, etc. Choices should at first be presented one at a time, with the child's response to each option as an indication of preference.

Creating the need to interact can also enhance communication opportunities, such as offering the child items or activities that he is known to dislike, allows the child the opportunity to reject. Not giving the child enough of a favourite item (i.e. food, drink, and toy) may encourage the child to indicate wanting more. Encouraging others to greet the child provides opportunities to respond.

- **Ensuring responsive partners**

When working with children with SPID, communication attempts made by them often go undetected by those around them. Carers/ teachers/ parents (as the communication partners) should be particularly sensitive and focus on being a responsive partner, because it is so critical in the development of communication skills for children with SPID. One should always be aware and on the lookout for any means of communication efforts made by the child, even if it seems unconventional, and then respond in a positive and appropriate way to these behaviours. It is important for all role-players to know and understand the specific behaviours and its communicative intent. It is important to interact frequently even though it might seem that a child is unresponsive. Communication requires at least 2 people, namely a sender and a receiver of a message. Response to attention is the basis of becoming aware of surroundings, people and would lead to interaction. Adult initiated activities such as these are crucial towards laying the foundation for further development.

- **Systematic instruction**

When teaching children with SPID, the use of systematic and direct instruction is highly recommended. This consists of a well-designed plan of teaching that involves targeting and evaluating what they can learn given meaningful opportunities to practice their skills. Such instruction would involve specific procedures of identifying, encouraging (trying to make the child react/ say something) and reinforcing targeted/ specific behaviours, within typical age-appropriate/ functional environments.

Focus should be on informal and spontaneous learning in various settings during the day. In a balanced, flexible language rich daily programme, important principles underpinning early learning are reinforced, namely, that children learn best through movement (kinaesthetically) and then through interacting with concrete materials (three dimensional learning), while being actively involved in the activities.

1.3 COMMUNICATION AND LANGUAGE: CONTENT AND ADAPTED ACTIVITIES

PRE LANGUAGE SKILLS

Content (0–6 years)	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Attention Eye contact Cause and effect Turn taking Imitation	GENERIC ACTIVITY: Aim To establish the following pre language skills: • Attention • Eye contact • Cause and effect • Turn taking • Imitation Materials • Noisy, press button toy - toy phone or real cell phone, Method <i>One carer facilitates the activity with an individual child.</i>

COMMUNICATION AND LANGUAGE : PRE LANGUAGE SKILLS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ACTIVITY: (Continued) The activity should be structured as follows: • Get the child's attention by having him/her looking at the carer. • Make eye contact with child. • Introduce the toy to the child in an energetic way e.g. exaggerated facial expression and tone of voice. • Encourage the child to look at and touch the toy. • The carer models the action of pressing the button to make a noise. • Verbalise what is being done and what is happening. • Take note of the child's facial expression. • Allow the child an opportunity to imitate the carer's action of pressing the button. • Reward any response immediately. • Reward should be verbal e.g. saying "good pressing!". • Introduce turn taking by requesting verbal and non-verbal communication. (Show and tell the child what to do). • Wait for the child to request the toy for their turn. NOTE: Carer should constantly watch and model eye contact throughout the activity

COMMUNICATION AND LANGUAGE: PRE LANGUAGE SKILLS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued)	ADAPTED FOR AWARENESS GROUP: Aim, materials and method same for above with the following adaptations in the method: • Eye contact with the child should be held for at least 5 seconds. • Hand-over-hand support is necessary for this group, thus the carer should place the toy in the child's hand and encourage exploration. • Verbalise what the child is doing, whilst hand-over-hand support is taking place. • Reward may be verbal (saying 'good pressing') or tactile (a firm hand on the child's arm to reassure him/her). • May need repeated reward. • Use constant hand-over-hand support. • Spontaneous and independent turns will not be initiated by a child in this group.
	ADAPTED FOR TRANSITIONAL GROUP: • Less support will be required and more opportunity for expansion of communication should be allowed. • Once eye contact is established, the carer should reward immediately by saying "Good looking". • Model the action to the child, namely by pressing the toys' button. • Initial hand-over-hand support will be necessary for most children in this group. • If the child attempts to imitate the carer, the carer must reward the child immediately by looking at the child, verbalising "good try" and touching the child's shoulder. • Initiate his or her own turn while the child watches. • A second turn should be facilitated on the part of the child by using hand-over-hand support by the carer. • The child may or may not initiate a second turn. • The child would require more modelling and demonstration than a child of the interactive group.

COMMUNICATION AND LANGUAGE: PRE LANGUAGE SKILLS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued)	ADAPTED FOR INTERACTIVE GROUP • Demonstrate which toy button is to be pressed, where after the child copies the action. • Hand-over-hand support would be required for the interactive group child whom experiences problems regarding their motor skills (such as hand function). For example, an athetoid cerebral palsied child with better cognitive skills than his/her motor skills, may require hand-over-hand support purely because of weaker limb muscles function. • Watch for the child's use of vocalization and eye contact when it is time for him to take his own turn. • Verbal cues, eye movements, body movements or speech may be demonstrated by the child when initiated his own turn. • Reward system will be the same as for the awareness and interactive groups. • The carer and child can each take more turns. • There may be more communication related to the toy object. For example, if this is a toy phone or real cell phone, the child may say "Hello" or "Bye" or "Hello mamma" if he is in a routine of speaking to his mother/parent over the phone.
ADDITIONAL ACTIVITIES AND IDEAS FOR INTEGRATION IN THE DAILY PROGRAMME	
EYE CONTACT: • Encourage eye contact during other activities/routines in the Daily Programme, such as: - during the morning ring – greeting song/social interactions; - during feeding routine; and - when a child wants something not close by/wants to ask a question. • Play peek-a-boo TURN TAKING: • Giving and taking objects. • Imitate sounds/movements that the child makes (whether he/she makes it intentionally or not).	

COMMUNICATON AND LANGUAGE: LISTENING AND UNDERSTANDING OF LANGUAGE

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
- Startles when there is a sudden noise - Recognizes mother's voice - Responds to sound, but cannot yet localize where it is coming from - Turns head to sound - Recognizes familiar sounds and voices - Can discriminate between friendly and unfriendly voices - Can localize sound 1 meter from ear - Begins to understand some simple verbal requests - Understands "No" - Responds to their name	GENERIC ACTIVITY: - Sing a song on body parts, e.g. adapted "hokey pokey"-song/ "hands, knees and toes"-song. - Sing slowly to give children the opportunity to sing along/copy actions – some children might have delayed responses. - Showing/touching body parts as song progresses. - Song can be used as an introduction to a discussion or a story about body parts. (see Annexure 7: Example: Topic: Me – my body)
	ADAPTED FOR AWARENESS GROUP - Constant hand-over-hand support and verbalising of experiences, e.g. touching different body parts or copying actions. - Child may smile and show different facial expressions to indicate awareness. - May make eye contact to indicate awareness. - The child may begin to show visual attention, but it would not be consistent.
	ADAPTED FOR TRANSITIONAL GROUP: - Initial hand-over-hand facilitation and verbalising of experiences. - According to child's abilities, start to retract hand-over-hand support and only provide verbal/ gesture support. - May show understanding of these body parts by pointing with own hand. (only applicable to children ready to transition to interactive group).

COMMUNICATON AND LANGUAGE: LISTENING AND UNDERSTANDING OF LANGUAGE	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued) • Responds to simple verbal commands, e.g. “Put your toy down.” • Understands at least 10 words without the object being present • Understands 15 – 50 words • Listens to rhymes and songs for 2 – 3 minutes • Able to learn simple songs and rhymes	ADAPTED FOR INTERACTIVE GROUP Children of this group are expected to show understanding of the following language skills: • Selection of specific picture symbols when asked “Show me.....”. • May also take turns in a conversation to answer simple questions about each body part. • Shows understanding of body parts during the body part song.
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Startles when there is a sudden noise • Recognizes mother’s voice • Responds to sound, but cannot yet localize where it is coming from • Turns head to sound • Recognizes familiar sounds and voices.	GENERIC ACTIVITY: • Discussion based on body awareness, also addressing tactile/physical stimulation. • Make laminated cardboard “feet” and place these in a trail (pathway) on the floor. • The children should follow the trail in a line. • If they are mobile and walking, they should walk independently or with assistance. • Children who are in the buggy should be pushed or encouraged to push themselves. • The children start moving as the music starts. • When the carer stops the music, the child’s movement stops. • The child will then be instructed to perform a specific movement with his/her body.

COMMUNICATON AND LANGUAGE: LISTENING AND UNDERSTANDING OF LANGUAGE	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Can discriminate between friendly and unfriendly voices • Can localize sound 1 meter from ear • Begins to understand some simple verbal requests • Understands “No” • Responds to their name • Responds to simple verbal commands, e.g. “Put your toy down.” • Understands at least 10 words without the object being present • Understands 15 – 50 words • Listens to rhymes and songs for 2 – 3 minutes • Able to learn simple songs and rhymes	ADAPTED FOR AWARENESS GROUP: • During activities of daily living, expose the child to communication and language similar as explained in 0-6 year age group. • Different textured “feet” are used and the child should be assisted to explore it with hand-over-hand support. • Footprints can be placed on the floor or on the footrest of a seating device and contact made with the child’s feet. • Talk to the child in short phrases, “Look this is a footprint. It looks like your foot. I am picking up your foot and placing it on the footprint. The footprint is made of cotton wool. It is soft”.
	ADAPTED FOR TRANSITIONAL GROUP: • An extra activity like musical footprints can be used for the transitional and interactive groups. • Children consistently show that they like or dislike the activity or a particular piece of music. • They might like a soft flowing musical piece, but pull away or use facial expressions to indicate their dislike of loud brassy sounds. • They start to remember how he/she previously (during the same session) responded to a specific song. • The child will show enjoyment each time that specific song is played. • Provide physical support to enable the child to move along the path when the music starts, and to stop when the music stops playing. • The child will have to be supported to touch the textured ‘feet’. • Support the child to focus on and see the ‘feet’ from up close. • Verbalise in simple language what the child feels or sees. “Look this is called a footprint. It looks just like your feet. These footprints are soft, because they are made of cotton wool”. • The child will allow the carer to initially give hand-over-hand support in exploring the ‘feet’ and later start to explore on their own for short periods.

COMMUNICATON AND LANGUAGE: LISTENING AND UNDERSTANDING OF LANGUAGE	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR TRANSITIONAL GROUP: (continued) • The child will listen to his/her own vocalisations • The child will eventually remember the ‘feet’ or a favourite song from one time slot in the day to another and respond appropriately for e.g. get excited when they hear a favourite song. ADAPTED FOR INTERACTIVE GROUP • An extra activity like musical footprints can be used for the transitional and interactive groups. • Children in this group listen and attend to familiar music and activities done routinely. • They will look around for the source if the song is played out of sight. • They recognise and choose their favourite song. • They listen in order to make a choice between two songs. • Some may respond appropriately to questions like “where is the cd” or “where is the cd player”? • A few might follow requests with two to three key words, signs or symbols – “take the cd to the cd player”. • Shows understanding of question by showing Head nod for YES and head shake for NO. • They remember the ‘feet’ and remember the actions of walking, stop and start. • The carer should use hand signs to introduce each separate step of the activity. For example, a hand sign is used for “stop, start, push and walk”. • They will shake or nod their heads to show understanding. • They use eye contact or gesture to show their understanding of whose turn it is. • They may however still experience frustration with regards to turn taking. • Children can be instructed to perform a movement with their body once the music stops, e.g. “Touch your head” or “Jump up and down”.

COMMUNICATON AND LANGUAGE: LISTENING AND UNDERSTANDING OF LANGUAGE	
Content 15–18 years	Vocational activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued) • Recognizes familiar sounds and voices • Can discriminate between friendly and unfriendly voices • Can localize sound 1 meter from ear • Begins to understand some simple verbal requests • Understands “No” • Responds to their name	GENERIC ACTIVITY: Making a fruit salad. • Use real fruit. • Use a visual schedule, consisting of photographs, to show step-by-step instructions of the activity. • Also refer to the visual schedule while following each step.
	ADAPTED FOR AWARENESS GROUP: • Assist the child with hand-over-hand support to explore the different fruits. • Using simple phrases, talk about what the child is feeling, seeing and smelling – “Look the apple is round and red” or “The pineapple smells sweet”. • Also assist the child to explore the objects like the cutting board or plastic bowl. • Assist the child with hand-over-hand support to make a noise by hitting a spoon on the plastic bowl. • Talk about the noise made. • Localization to sound may occur, but is mostly inconsistent. • Sounds which the adolescent may respond to can include the following activity-specific sounds: sound of water rushing from the tap, sound of grating of fruit, sound of cutting of fruit on a wooden board.
	ADAPTED FOR TRANSITIONAL GROUP: • Introduce the child to the fruit and utensils as described for awareness group. • Hand-over-hand support may be needed to support some individuals to touch the fruit or utensils. • The child will recognise and respond to familiar fruit or the repeated activity. • They may be able to imitate the hitting of the bowl with a spoon after repetitive showing and hand-over-hand support. • They concentrate for short periods of time and increasingly explore the fruit and utensils – with continuous verbal guidance

COMMUNICATON AND LANGUAGE: LISTENING AND UNDERSTANDING OF LANGUAGE	
Content 15–18 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued) • Responds to simple verbal commands, e.g. “Put your toy down.” • Understands at least 10 words without the object being present • Understands 15 – 50 words • Listens to rhymes and songs for 2 – 3 minutes Able to learn simple songs and rhymes	ADAPTED FOR TRANSITIONAL GROUP: (continued). • They observe the result of hitting the spoon on the bowl with interest and listen to their own vocalisations of delight. • They eventually will hit a bowl with a spoon if presented again later in that same day.
	ADAPTED FOR INTERACTIVE GROUP: • Supply the group with fruit already cut into pieces, let them peel a banana or citrus - or use hand-over-hand support and close supervision to cut a fruit into pieces. • Give instructions before and during the activity. • Use a visual schedule to show step-by-step instructions of the activity. • Also refer to the visual schedule while following each step. • Let the adolescent choose a few of their favourite fruits. • The adolescent is expected to understand and prompt others to join in the activity. • They anticipate the activity when they see the preparations or when other contextual clues indicate that. • If unable to speak, they may use head shake for “No” and head nod for “Yes” to show understanding of language. • They recognise potential solutions to problems, for example, they know that if they struggle to peel the banana they should bring it to a supervisor/ carer. • Some may answer questions about immediate and familiar events for example ‘where is the apple?’ ‘What are you doing?’ • They follow requests and instructions containing at least two key words, signs or symbols, for example, ‘Put the spoon in the dish’, ‘Give the apple to Johnny’. • A few may respond to others in group situations, for example, taking turns appropriately and pass a spoon from one individual to the next. • They follow requests and instructions with three key words, signs or symbols, for example, ‘Give me the little red apple’.

COMMUNICATON AND LANGUAGE: LISTENING AND UNDERSTANDING OF LANGUAGE
ADDITIONAL ACTIVITIES AND IDEAS FOR INTEGRATION IN THE DAILY PROGRAMME
• Draw child's attention to any sound (happening inside/outside)/objects that make sounds. • Play different kinds of music from CD's or musical instruments. • Hide squeaky toy and make child look for it. • During routine activities such as brushing of teeth, ask child to hand the toothbrush/pick up his/her cup among others (photo of child on cup to make identification easier). • Model the instructions before expecting them to be followed.

COMMUNICATION AND LANGUAGE: EXPRESSIVE LANGUAGE	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
- Vocalizes in different ways to express discomfort, hunger and pain. May attempt moving limbs. - Communicate using cooing sounds like “ah”, “uh” and “eh” (single syllabus) - Can laugh out loud - Makes longer “conversations” by combining sounds - Starts to use babbling and cooing sounds: p, b, m, k, g - Uses vocal play to change pitch of voice in order to obtain attention - Repeats consonant-vowel syllable combination, e.g. “ma-ma; da-da; pa-pa” - Can indicate “No” by shaking their head - Uses 1 – 2 word sentences	GENERIC ACTIVITY: Multisensory storytelling is a beneficial communication activity for this age group. The story book which is used should be appropriate for the age group of 0 to 6 years. Goals for this activity should include: - Listening to telling of the story and animal sounds. - Attempt at imitating keywords: e.g. Farm animal sounds and names, verbs such as push, go, come and feed. - Feeling the textures in the book e.g. fur or feathers of animals which has been stuck onto animal pictures. - Smell of scenery as applicable to theme of story e.g. smell of boiled eggs or straw/ hay. - Taste experience as applicable to theme of story e.g. taste of boiled egg/ cheese. PLEASE NOTE: Do not expose children with feeding problems to food items which might cause choking.
	ADAPTED FOR AWARENESS GROUP: - With hand-over-hand support, assist the child to feel the textures in the book e.g. fur/ feathers of animals which has been stuck onto animal pictures in book. - Also expose them to the different farm animal toys, e.g. water-play with rubber ducks. - Talk to the child about what they feel, hear and smell, e.g. “I am placing your hands in the water. The water is nice and warm. Look at the yellow duck. The duck makes quack-quack”. - Look out for possible change in the child’s facial expression, e.g. smiling or frowning when feeling the textures in the book or hearing the animal sounds.
	ADAPTED FOR TRANSITIONAL GROUP: - Perform the activity the same as for the awareness group. - Use three dimensional animal toys such as a cow, sheep, chicken, horse and pig. .

COMMUNICATION AND LANGUAGE: EXPRESSIVE LANGUAGE	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued) • Can say 4 – 6 words with consistency • Uses gestures and pointing to indicate certain needs • Verbalises 6 – 20 recognizable words • Uses 4 word sentences • Uses functional sentences & to explain actions, e.g. “I am hungry.” / “The baby eats”	ADAPTED FOR TRANSITIONAL GROUP: (continued) • Once the group is more familiar with the story, the carer can start exposing the group to photos of farm animals. • Give the child hand-over-hand support to touch and explore the toy animals and the tactile pictures (with fur/ feathers etc.) in the story book. • Begin to expose the child to situations of choice making between two animals, e.g. “Which animal goes mooooooo? Is it this one (show horse) or this one (show cow)?” • Place the cow in the child’s hand and verbalise “Well done, yes the cow goes moooooo!” • Encourage the child to copy the animal sound by repeating “mooooo”, whilst making eye contact with the child and ensuring he can see how you are shaping your mouth to form the sound.
	ADAPTED FOR INTERACTIVE GROUP: • Only a very few will be able to listen, attend to and follow stories for short stretches of time – as most children will display irregular attention skills. • A simple story should be told with plenty of gestures, hand signs, repetition of key words (such as animal names and sounds) and showing pictures. • Children with better communication skills can be encouraged to repeat words, gestures or phrases from the story during story telling. • Some children may be able to altogether verbalise between 10 – 50 words within their vocabulary. • Some children will be able combine single words or signs or symbols to communicate for example, ‘duck gone’ ‘duck swim’. They might ask simple questions to obtain information, for example, ‘Where’s pig?’ • Expose the child to situations of choice making between two animals, e.g “Which animal goes mooooooo? Is it this one (show horse) or this one (show cow)?” • Wait for the child to respond. • Correct the child if the response was incorrect, but still praise his/ her effort.

COMMUNICATION AND LANGUAGE: EXPRESSIVE LANGUAGE	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP: (continued) • If the child does not respond, place the cow in the child's hand and verbalise "Well done, yes the cow goes mooooooo! Can you also make the sound of a cow, go mooooooo!?" • Remember to ensure the child is able to see how you shape your mouth when making the animal sounds and words in the story.
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Vocalizes in different ways to express discomfort, hunger and pain. May attempt moving limbs. • Makes longer "conversations" by combining sounds • Uses vocal play to change pitch of voice in order to obtain attention • Repeats consonant-vowel syllable combination, e.g. "ma-ma; da-da; pa-pa" • Can indicate "No" by shaking their head • Uses 1 – 2 word sentences • Can say 4 – 6 words with consistency	GENERIC ACTIVITY: Choice-making of clothing during dressing activity.
	ADAPTED FOR AWARENESS GROUP: • Adolescent of this group will encounter, become aware and attend to the fact that they are being dressed. • Expose him/her to touch the different textures of materials and buttons whilst talking to him/her about what is felt, seen or smelled. • Use simple phrases and emphasize 'soft' or 'fluffy'. • Make eye contact with the adolescent and say the name of the clothing item while bringing the items into his/her field of vision or close to his/her nose. • This will help to establish awareness and attention to the real object. • Name the body part that you are touching during the process of dressing. • When the activity is done, tell him/her "all done". • He/she learns to show interest and respond more consistently to the familiar carer, clothes and the activity of dressing and undressing.

COMMUNICATION AND LANGUAGE: EXPRESSIVE LANGUAGE	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued) • Uses gestures and pointing to indicate certain needs • Verbalises 6 – 20 recognizable words • Uses 4 word sentences • Uses functional sentences & to explain actions, e.g. “I am hungry.” / “The baby eats”	ADAPTED FOR AWARENESS GROUP: (continued) • He/she may only respond by making vocalizations, as they are not yet capable of conveying feelings by means of words or basic gestures. • Vocalizations are mainly used to express discomfort, and not in response to educational stimulation.
	ADAPTED FOR TRANSITIONAL GROUP: • Adolescent of this group will be able to respond when you talk about clothes, body parts and the steps of dressing. • They start to engage in a choice-making situation between two pieces of clothing. • Make sure he/she looks at the pieces of clothing. • Say the name thereof and demonstrate (model) choice-making. • He/she should see the modelling demonstration 2-3 times. • Ask him/her to show his/her own choice. • Facilitate the right choice with hand-over-hand support, reinforcing the name of his/her choice. • Repeat the activity and allow time for response at his/her own pace. • If the pants are picked, reward this choice immediately with spoken or tactile praise (deep pressure on arm). • Say ‘pants’ and use hand-over-hand support as required to take the piece of clothing. • Choice-making may be attempted by means of eye-gaze, pointing or vocalisations (e.g. “da”). • The emphasis should be placed on non-verbal communication (like copying gestures, pointing or using eye-gaze) in this group, as word formation only occurs in the interactive group. • This group remembers learned responses during the same activity and therefor repeating it 2-3 times is advised. • They start to recognise familiar people and familiar events like dressing and vocalise or gesture in response. • As they begin to communicate more intentionally, concentrate for short periods and explore materials with less support, they begin to remember some of the routine of dressing.

COMMUNICATION AND LANGUAGE: EXPRESSIVE LANGUAGE	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR TRANSITIONAL GROUP: (continued) • Coloured photographs which are identical in appearance to the real desired clothing item can be introduced as he/she progresses towards the interactive group. • The photographs can be used alongside the garments, but matching is not expected in this group. • The photograph should be placed at a distance of 30 cm from the eyes. • Also consider the size of the photographs. • Consult with occupational therapist for visual adaptations. • Say the name of the clothing item while encouraging the child through dressing by hand-over-hand support. • Encourage vocalisation and reward participation in dressing activity with spoken praise and tactile cues (deep pressure on arm). ADAPTED FOR INTERACTIVE GROUP: • The interactive group should be able to show selection and choice-making of the real object (clothing item) when presented with two different options. • Some will develop the ability to choose a specific photograph as a representation of the real object. • Clothing item or photograph may be selected through using eye-gaze, pointing, nodding/ shaking their head or vocalization (using single words or short sentences, e.g. “Yes”, “No”, “That one”). • These different actions may be used together as a communication attempt. • Reward choices immediately. • Rewards may be given in the form of spoken praise (“Well done”, or “Yes, that one is the shirt”) or tactile cues (firm hand pressure on arm).

COMMUNICATION AND LANGUAGE: EXPRESSIVE LANGUAGE	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued)	ADAPTED FOR INTERACTIVE GROUP: (continued) - The following steps can be followed: - Establish understanding of real object - Establish understanding of photograph and later picture of real object. - Establish understanding of photograph or picture as a symbol of real object. - This is established by matching the relevant photograph picture and/or real object. - Reward matching with praise - This could be spoken praise and tactile cues (firm hand pressure on arm). - Give the real clothing item to the child as a dressing activity continues. - Say the name of the clothing item while encouraging the child through supported dressing. - Reward participation in dressing activity with spoken praise and tactile cues (firm pressure on arm). - This group can apply potential solutions systematically to problems by bringing a piece of clothing to an adult in order to request getting dressed. - Some will repeat, copy and imitate between 10 and 50 single words, signs or phrases or use a repertoire of objects or symbols. - They use single words, signs and symbols for familiar objects, for example shoes, socks, pants and to communicate about what they like and dislike. - A few will be able to combine two key ideas or concepts, for example, “take off” “put on”. - They make attempts to repair misunderstanding without changing the words used, for example, by repeating a word with a different intonation or facial expression. - Only a very few will initiate and maintain short conversations consisting of 3 word phrases using their preferred medium of communication. - They ask simple questions to obtain information, for example, ‘Where’s shoe?’

COMMUNICATION AND LANGUAGE: EXPRESSIVE LANGUAGE	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP: (continued) • They can use prepositions, such as ‘put arm in’ or ‘put shoe on’, and pronouns, such as ‘my shoe’ or ‘it is pretty’, correctly.
Content 15–18 years	Vocational activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Vocalizes in different ways to express discomfort, hunger and pain. May attempt moving limbs. • Makes longer “conversations” by combining sounds • Uses vocal play to change pitch of voice in order to obtain attention • Repeats consonant-vowel syllable combination, e.g. “ma-ma; da-da; pa-pa” • Can indicate “No” by shaking their head • Uses 1 – 2 word sentences • Can say 4 – 6 words with consistency • Uses gestures and pointing to indicate certain needs	GENERIC ACTIVITY: • Cooking or baking activity. • Start with a basic recipe e.g. making a sandwich. • An adolescent with blindness should be given more cues of sounds, smell, taste and touch. • An adolescent with deafness should be given more visual and touch cues. • Also use hand signs and gesture to communicate with him/her. • Although this is a domestic/vocational activity, the goals for this group should be realistic. • They cannot be expected to be fully independent in the execution of the activity.
	ADAPTED FOR AWARENESS GROUP: • Expose the adolescent to different ingredients and utensils used by means of hand-over-hand support. • Using simple phrases, talk about what he/she is touching, seeing and smelling, e.g. “Mmmm the bread smells nice and fresh”. • Assist him/her with hand- over-hand support to spread the margarine and cut the sandwich – if the physical condition allows him/her to perform the action. • Look for signs of alertness from him/her, e.g. expression of discomfort by crying/ facial expression change or fleeting/ inconsistent eye-contact.

COMMUNICATION AND LANGUAGE: EXPRESSIVE LANGUAGE	
Content (15–18 years)	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued) • Verbalises 6 – 20 recognizable words • Uses 4 word sentences • Uses functional sentences & to explain actions, e.g. “I am hungry.” / “The baby eats”	ADAPTED FOR TRANSITIONAL GROUP: • Introduce the bread, spreads and utensils as described for awareness group. • Hand-over-hand support may be needed to support some individuals to touch the bread or utensils. • The adolescent will recognise and respond to familiar spreads or the repeated activity by vocalising or gesturing in a particular way. • Through reaching out towards the carer or the sandwich, he/she will indicate his/her choice. • They remember how to assist in spreading (during the same session) after repetitive showing and hand-over-hand support. • They concentrate for short periods and increasingly explore the sandwich, ingredients and utensils with verbal guidance from the carer. • They observe the result of cutting the sandwich with interest and vocalise their delight. • They imitate the action and will eventually attempt to cut the sandwich if presented a similar situation again later in that same day. • Some adolescents in this group will seek your attention through eye contact, gesture or action. • They will request a sandwich through pointing and remember how to spread a sandwich from one day to another, if done daily and routinely. • Stimulation for expressive language will include the following goals: • Exposure to choice-making situations (see activity for age group 7-14 years). • Experience of different smells and tastes and linking it to choice-making situations as per two different smells or two different tastes. • Acknowledge and encourage pointing, hand signs and vocalisation (only sounds, no words yet). • Consistent use of a specific hand or body movement to express needs and wants e.g. moving own buggy towards the door to show desire to not participate, or attempt at hand sign for bread or water.

COMMUNICATION AND LANGUAGE: EXPRESSIVE LANGUAGE	
Content (15–18 years)	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR TRANSITIONAL GROUP: (continued) • Watch for facial expression as expression of likes and dislikes.
	ADAPTED FOR INTERACTIVE GROUP • Supply the group with bread slices and different spreads. • Demonstrate step-by-step how the sandwich is to be made, where after the adolescent makes his/her own sandwich by means of verbal guidance from the carer (and/or possible visual schedules in the form of photographs of the steps of the activity in sequence). • Use close supervision (and a blunt knife) when the group is cutting the sandwich into pieces. • Let the adolescents choose their own favourite spreads. • The adolescent can be expected to greet known people and prompt others to join in the activity. • They anticipate the activity when they see the preparations. • They are able to choose between spreads. • If unable to speak, they may use head shake for No and head nod for Yes. • They recognise potential solutions to problems, for example, they know that if they struggle to spread they should bring the bread to a supervisor. • They show consistent use of an object symbol to express needs and wants e.g. holding cup to express need for water. • Some repeat, copy and imitate about 50 words or signs, including the names of for e.g. “bread, jam and knife”. • A few combine two key ideas or concepts, e.g. ‘more butter’. They make attempts to repair misunderstandings without changing the words used, for example, by repeating a word with a different intonation or facial expression. • Very few will initiate and maintain short conversations using their preferred medium of communication. • They ask simple questions to obtain information, for example, ‘Where’s bread?’ • They can use prepositions, ‘on plate’, and pronouns, such as ‘my plate’.

COMMUNICATION AND LANGUAGE: EXPRESSIVE LANGUAGE	
Content (15–18 years)	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP (continued) - The interactive group will be expected to show the following skills for expressive communication: - Use of photograph to express needs, requests, questions, greetings and responses to questions - To use photograph symbols in conjunction with hand signs and spoken single words or vocalized syllables - To show turn taking in a conversation with the adult carer or other group members - To show eye contact with communication partner (adult or adolescent) during conversation - To express politeness markers: namely: Hello, bye, please, thank you
ADDITIONAL ACTIVITIES AND IDEAS FOR INTEGRATION IN THE DALY PROGRAMME	
- Play social games with the child that requires gesturing/verbalising. For example – greeting song with eye contact, hand shake and other gestures. - Play pretend games such as feeding/dressing dolls; pushing a toy car/truck and pretending to pick up people/animals – talk about what is happening and encourage the child to participate in play and talking/gesturing. - Talk to the child during: - Toileting/nappy changing - Dressing - Feeding - Put words and sounds to the child's actions and have him/her repeat it.	

COMMUNICATION AND LANGUAGE: EXPRESSIVE LANGUAGE

ADDITIONAL ACTIVITIES AND IDEAS FOR INTEGRATION IN THE DAILY PROGRAMME(continued)

- Introduce new words in context of the child's natural environment and create numerous opportunities during the day for the child to be exposed to, make associations and repeat words. (Use objects, photos and pictures to support communication and language development).
- Show excitement when child attempts to use words.
- Ask questions and extend the child's vocabulary by constantly naming and describing objects and actions.
- Use descriptive words for the things the child sees and does.
- Encourage the child to communicate and acknowledge and praise spontaneous attempts.
- Listen to and answer the child's questions.
- Encourage conversations during activities such as playtime, snack time, outdoors, etc.
- Picture books/scrap books – the children can help make these books. For example, different topics such as body parts; family; food; toys; animals; bathroom; utensils for washing; etc. Give opportunities for the child to page through books, discuss and ask questions.
- Give the child a chance to initiate a conversation.
- Expand vocabulary and use of sentences within routines, activities and themes/topics within the Daily Programme making use of objects, photos and pictures.

2. VISUAL PERCEPTUAL AND COGNITIVE SKILLS

2.1 WHAT IS VISUAL PERCEPTUAL AND COGNITIVE SKILLS?

The interpretation of a visual stimulus is a mental process involving cognition. Interpreting the stimulus allows us to give meaning to the visual stimulus, which aids us in understanding the environment. The attainment of these visual perceptual and cognitive skills occurs via the process of perceptual learning.

We know that children (babies) first become aware of their surroundings (people, objects, environment), through their senses (seeing, hearing, touching, tasting, moving, smelling) also known as perceptual awareness (perceiving of surroundings). From being aware they then start to respond to, interact with, and later on to actively becoming involved in the exploring of their surroundings, still using/ relying on their perceptual skills. Through this active involvement in routines and activities in their natural environment the basis for all development - physical, social and emotional, language and **cognitive development (ability to learn and think)** - is being build. These different aspect/areas of development does not happen separately, but are integrated (combined).

Cognitive development therefore depends on:

- the information the child receives through the senses;
- how the child makes sense of the information received (ability to learn and think);
- the activities the child is engaged in;
- the knowledge gained;
- the new skills developed and learned; and
- the language development.

Cognitive development will take place when children are given opportunities to explore, practise, talk and enjoy learning about their environment. It is while learning about their environment that their **brains are developing:**

- concentration and attention;
- storing of information in the memory;
- the use of language to organise thinking; and
- spatial and conceptual understanding (concepts such as space, shape, colours, numbers, symbols).

2.2 SPECIFIC AIMS OF VISUAL PERCEPTUAL AND COGNITIVE SKILLS:

- Development of perceptual skills, such as matching and spatial awareness, needed to participate in self-care, play and vocational activities.
- Use of objects in a goal-directed fashion in order to function as independently as possible within their immediate environment.

Children with profound intellectual disabilities might not be able to master sufficient skills necessary to be able to grasp and fully understand academic/mathematical concepts. They are mostly functioning on a sensorimotor and perceptual level, which forms the basis for pre-academic skills. This however does not imply that the child should be excluded or not exposed to these concepts. In the child's natural environment he/she is exposed to numerous spatial, conceptual and mathematical "happenings" throughout the day. This should be acknowledged and incorporated during the normal daily routine, with the emphasis on using the aspects of the **DAILY PROGRAMME** to promote the acquisition of skills such as body awareness, matching and sorting, counting, shapes, colours, size, space, time and sequencing. Attention to last mentioned skills should occur in an integrated, fun and spontaneous context throughout the daily programme.

Formal instruction is not needed, but children will benefit from adults talking to them about what they are experiencing (practical/actual experiences). The focus is not on the development of mathematical skills, but rather creating opportunities to be exposed to and experience these skills in integrated and incidental ways. For example, adult-guided learning opportunities can be offered during early morning ring when children are greeted and a roll-call is taken provides an opportunities for self-identification and basic perceptual skills like matching a photo to a group member's face. Other rings such as perceptual-motor rings, movement and music rings, creative arts, the weather chart, calendar and birthday rings all give opportunities for exploring (See Annexure 6 for an example on integrated development in the **Daily Programme**).

DEVELOPMENT AREAS AND CONTENT OUTLINE OF VISUAL PERCEPTUAL AND COGNITIVE SKILLS:

For a child to be able to develop visual perceptual and cognitive skills he/she needs to be ready to engage, must have developed memory, object permanence and have understanding of cause and effect. Some children with PID may never fully develop sensory awareness, which would thus inhibit readiness for acquiring visual perceptual and cognitive skills.

The following content is found within visual perceptual and cognitive skills:

- Visual perceptual readiness;
- Body awareness;
- Laterality;
- Midline crossing;
- Conceptual skills: size, colour, shape, number; and
- Visual perceptual skills.

2.3 VISUAL PERCEPTUAL AND COGNITIVE SKILLS: CONTENT AND ADAPTED ACTIVITIES

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Visual Perceptual Readiness • Brief attention to stimuli • Focus eyes on objects • Follow large objects for short period • Transferring their focus from one object to another • Consistent following of moving objects 1. Within visual field 2. Up and down as well as sideways 3. Diagonal 4. Track and locate out of visual field	GENERIC ACTIVITY: • Individuals who are still mastering perceptual readiness, are in need of regular one-to-one sensory stimulation. • It is difficult to optimally involve the child in a group setting (e.g. morning ring), as the carer constantly needs to attempt gaining their brief attention. • It is however still important to make them part of the group/class, and specifically make use of the assistant to focus on involving the child. <u>Specific activity:</u> Playing with a rattle/shaker to enhance visual perceptual readiness. <i>NB: The carer must keep in mind that the child will most likely have a slower response time, and should thus give enough time for the child to attempt to focus and track the object.</i> ADAPTED FOR AWARENESS GROUP: • Participation of the child needs to be fully encouraged/supported. • Make sure that the environment is not too distracting. T • The child needs to focus on the rattle and the carer. • Make sure the rattle/shaker is large and brightly coloured to gain the child's interest/attention. • Start with shaking the rattle/shaker. • The multi-sensory (visual and auditory) experience assists with gaining their interest and attention.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Visual Perceptual Readiness (continued)	ADAPTED FOR AWARENESS GROUP: (continued) • The child's vision is best when the rattle/shaker is about 30cm away. • Only focus on gaining the child's focus and brief attention on the rattle/shaker. • Promote following of the rattle/shaker within their visual field.
	ADAPTED FOR TRANSITIONAL GROUP: • Move the rattle/shaker in different directions (vertical and horizontal), but in a bigger visual field. • The rattle/shaker can be moved slower/faster depending on the ability of the child to follow it with his/her eyes/head. • Use hand-over-hand support to enable the child to shake the rattle, and withdraw your physical support if the child shows signs that he/she might want to shake the rattle by him/herself. • Hide the rattle behind your back, and observe for signs that the child is searching for the rattle, e.g. attempting to look behind your back by tilting his/her head or becoming upset.
	ADAPTED FOR INTERACTIVE GROUP: • Child must be able to follow the rattle by only using eye movements and not head movements. • Children in buggies/wheelchairs may be allowed to follow using their heads along with eye-movements in order to overcome their physical barriers. • Move the rattle/shaker in different visual fields (vertical, horizontal & diagonal). Make use of two shakers and shake one e.g. left top and the child must locate and focus on the object. • Shake the rattle at the bottom right and ask the child to locate and focus on the rattle/object. • In this activity a variety of objects can be used (not only a rattle or sound making objects) for the child to focus on and follow.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Visual Perceptual Readiness: • See 0-6 age group on applicable content. • If a child has difficulty with attention, focus and tracking that is not improving please make appropriate adaptations to the activity. E.g. work within the child's available visual field when presenting activities. <i>Consult professional help for further guidance.</i>	GENERIC ACTIVITY: • Same activity and principles apply as above 0–6 yr. • The carer might opt to use an alternative object to gain the child's visual and auditory attention – as opposed to a rattle – e.g. a colourful shaker/ bells.
Content 15–18 years	Vocational activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• See 0-6 age group on applicable content. • If an individual still has not mastered the skills, make appropriate adaptations to the activities. E.g. work within the child's available visual field when presenting activities.	GENERIC ACTIVITY: • If the adolescent is still not fully ready to be exposed to perceptual concepts, it is not possible to include them in vocational activities – and should the focus rather be shifted to social groups and activities. • Same activity and principles apply as above 0–6 yr. • The carer might opt to use an alternative object to gain the child's visual and auditory attention – as opposed to a rattle – e.g. a colourful shaker/ bells.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Body Awareness Sufficient body awareness is important for the functioning of other skills, such as motor skills and perceptual/ conceptual skills. <u>Body awareness consists of 3 areas:</u> -Body concept (individual's knowledge regarding their body parts – including location, size, amount, function and self-identification) - Body image (individual's emotional experience of their bodies) - Body schema (awareness of the relationship between various parts of the body to each other)	GENERIC ACTIVITY: Obstacle course Addressing attention, body schema, listening, visual skills (depth perception), proprioception (the unconscious perception of movement and spatial orientation arising from stimuli within the body itself), movement and basic spatial concepts (on, under).
	ADAPTED FOR AWARENESS GROUP: • Support the child by means of hand-over-hand support throughout the obstacle course. • Verbalise where the child's body is in relation to the obstacle course, e.g. "Your feet are standing on the chair; you are crawling underneath the table". • A non-mobile child may be pushed by means of a seating device throughout a adapted obstacle course, e.g. "around a chair, on a straight line", however the child must also constantly be given the opportunity to experience his/her body parts in different positions in space during self-care routines, e.g. "I am lifting your legs to put on your pants" or "I am tilting your chin so you can take a sip of water".
	ADAPTED FOR TRANSITIONAL GROUP: • Initial hand-over-hand support is provided to start the obstacle course, once the child has more self-confidence the carer can increase verbal guidance and decrease physical support. • Encourage the group to verbalise the positions the individual is taking in, e.g. "Sipho is crawling over the wedge".

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Laterality: The child develops awareness that his body has 2 sides. It is not expected of a child whom experiences profound intellectual disability to accurately identify their left/ right sides, but only develop awareness regarding their 2 sides Midline crossing: An individual's ability to efficiently move and use his/her hands, feet and eyes of the one side of the body in the space of the other side of the body – thus crossing the imaginary line that divides the body into a left and right side.	ADAPTED FOR INTERACTIVE GROUP: • Child should be able to accurately complete the obstacle course without verbal or physical guidance – the carer only provides an initial demonstration of completing the obstacle course. • Children in this group may also attempt to verbally guide others in the transitional/awareness group to complete the obstacle course. Midline crossing: <i>Is a specialized area, and if midline crossing problems are suspected - consult a Physiotherapist/ Occupational Therapist for guidance.</i>

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Body Awareness: • Body concept (individual's knowledge regarding their body parts – including location, size, amount, function and self-identification) • Body image (individual's emotional experience of their bodies) • Body schema (awareness of the relationship between various parts of the body to each other) Laterality: The child develops awareness that his body has 2 sides. It is not expected of a child whom experiences profound intellectual disability to accurately identify their left/ right sides, but only develop awareness regarding their 2 sides	GENERIC ACTIVITY: Simple Simon says. Addressing body concept, body schema, attention, receptive language and movement senses.
	ADAPTED FOR AWARENESS GROUP: • Children in this group have limited body awareness. • Enable the child to explore his/her body by means of hand-over-hand support, e.g. taking the child's hand and placing it on their stomach whilst slowly verbalising "This is your hand, and I helping you to touch your stomach". • Self-care routines provide an optimal opportunity for the carer to enhance the child's body awareness through continually verbalising the child's body parts, functions and actions. • Exposing the child's body parts to different textures is a wonderful way to improve sensory awareness, e.g. rubbing his/her legs with different textured towels, a warm cloth or sponges during diaper changing.
	ADAPTED FOR TRANSITIONAL GROUP: • Children in this group have emerging body awareness. • Most of the children will not be able to independently identify major body parts during Simple Simon says instructions. • The carer should thus first demonstrate where the specific body part is, and encourage the child to copy her actions, for example the carer verbalises "Simple Simon says place your hands on your head (carer places her hands on her head), come children place your hands on your heads". • Provide the child with enough time to respond. • Hand-over-hand support may be needed to support some individuals to touch their specific body parts.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP: - Most children in this group will be able to identify major body parts (e.g. head, stomach, arms and legs), and some might even be able to identify more complex parts (e.g. thumb, knees and shoulders). - The carer might still need to show where her body parts are, and then allow the child the opportunity to copy her whilst she names the body part.
Content and functional 15-18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Body Awareness: Body concept (individual's knowledge regarding their body parts – including location, size, amount, function and self-identification) Body image (individual's emotional experience of their bodies) Body schema (awareness of the relationship between various parts of the body to each other)	GENERIC ACTIVITY: Self-identification during morning ring (<i>in preparation for formal roll-call in a vocational setting</i>). Addressing self-identification, attention, matching, sorting, hearing, verbal and non-verbal communication.
	ADAPTED FOR AWARENESS GROUP: - Verbalise the child's name during morning ring, and position a photograph within the child's visual field – in attempt to gain a momentarily gaze from the child in the direction of the photo. - Physically touch the child and repeat his/her name while maintaining physical contact.
	ADAPTED FOR TRANSITIONAL GROUP: - Show the group with their individual photographs, and encourage the children to identify themselves by looking at the photo. - It might still be necessary to say child's name before self-identification takes place. - Focus on the child's non-verbal communication signs for possible reaction, for example: quick nodding of the head, possible clapping of the hands, placing their hand in the air or on their chest to identify themselves.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Laterality: The child develops awareness that his body has 2 sides. It is not expected of a child whom experiences profound intellectual disability to accurately identify their left/ right sides, but only develop awareness regarding their 2 sides.	ADAPTED FOR INTERACTIVE GROUP: • Show a photograph or verbally mention a child's name, where after the child must identify him/herself by means of non-verbal gestures or single words/sentences. • The child is also able to identify other individuals in the group and mention the people who are absent for the day.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Basic perceptual concepts (e.g. colours and shapes) develop in the following sequence for normal childhood development: 1. Develop awareness of concept. 2. Matching then sorting skills. 3. Identify concept on a 3-Dimensional level then a 2-Dimensional level. 4. Name the concept on 3-D then later on a 2-D level. Please note: Individuals whom experience intellectual disability might divert from the normal development sequence. Conceptual Skills: Please note that most conceptual skills will only make sense to a child if he/she has sufficient body awareness.	GENERIC ACTIVITY: (during social ring): Rolling a ball (any colour) between children seated in a social circle. Addressing attention and awareness regarding size, colour and shape as well as social skills (including eye contact, self-identification and turn-taking skills). ADAPTED FOR AWARENESS GROUP: • Call the child's name or touch the child to draw his attention and prepare him for engagement in the activity. E.g. blue ball and a red ball. • Remember you are only making the child aware and introducing the concept to the child. • Show the two objects to the child. • Talk about the colour of the ball. • Ask the child which colour ball he wants to play with, where after the carer suggests a colour (as the child will not physically/verbally respond to questions). • Provide hand-over-hand support to enable the child to explore the ball. • Talk about the shape, size and colour properties of the ball (round, red, soft, bouncy). • Facilitate rolling of the ball (the social ring can include other children from the interactive group to roll the ball back to the carer and child in the awareness group <i>OR</i> the circle can consist of only awareness group children whom are supported by assistants). ADAPTED FOR TRANSITIONAL GROUP: • Children are positioned in a circle, where after the carer mentions a child's name and then gently rolls a big ball to the child.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Size concept: Big and small, thick and thin, long and short. Colour concept: • Becomes aware of colours of objects • Matching and sorting objects according to colour • Identifying colours by pointing it out when asked • Naming colours Start with the primary colours (blue, red and yellow) and only focus on ONE colour for an extended period – only move on to the next colour when the child is able to consistently recognize the initial colour. •	ADAPTED FOR TRANSITIONAL GROUP: (continued) • The carer verbalises “Look I am rolling a biiiiiig ball”. • Carer/ assistant then supports the child who received the ball to indicate (by pointing or using a gesture) which friend he/she wants to roll the ball to. • The carer calls the friend’s name, and then provides hand-over-hand or verbal guidance so that the ball is rolled from the one child to the one whose name was called out. • After a few turns, the carer introduces a smaller soccer ball. • Before repeating above social ring activity, the carer shows the group the difference regarding the big and small ball and lets the children copy her gestures of big and small (physical support may be needed to enable the children to copy the gestures of big and small). • The carer may ask some of the children (who are almost ready to transition to the interactive group) to show her the big/ small ball when placed next to each other. ADAPTED FOR INTERACTIVE GROUP: • Children are positioned in a circle, where after the carer mentions a child’s name and then gently rolls a big ball (colour e.g. red) to the child. • The carer verbalises “Look I am rolling a biiiiiig ball. The ball is red”. • Carer/ assistant then requests the child who received the ball to indicate (by pointing or using the friend’s name) which friend he/she wants to roll the ball to. • The child is then instructed to gently roll the ball to the mentioned friend. • After a few turns, the carer introduces a smaller soccer ball (different colour, e.g. blue).

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Shape concept: - Manipulate three-dimensional shapes - Matching and sorting of objects according to shape Identifying shapes by pointing it out when asked Naming shapes Start with the basic shapes (circle, square and triangle) and only focus on ONE shape at a time for an extended period – only move on to the next shape when the child is able to consistently recognize the initial colour. Number concept: - Respond to and join in with familiar number rhymes, stories, songs and games,	ADAPTED FOR INTERACTIVE GROUP: (continued) - Before repeating above social ring activity, the carer shows the group the difference regarding the 2 balls (namely: big/ small and red/blue) and lets the children copy her gestures of big and small. - The carer may ask some of the children to identify which ball is red, or alternatively remind them which is the red ball where after the carer asks the child to search for other red objects in the room (matching).
	ADDITIONAL ACTIVITIES AND IDEAS FOR INTEGRATION IN THE DAILY PROGRAMME: - Create opportunities throughout the Daily Programme to make children aware of and expose them to basic concepts (counting, colours, shapes,) in an incidental, integrated and playful way, and never as formal activities. - Children learn best when exposed to and experiencing for themselves within their natural environment. - During the Daily Programme in routines and activities for example during toileting/morning ring discusses the colour of the clothes all are wearing. For example, “John has a red shirt on, who else is wearing something red?” “What else is the class is also red / let’s all find something red and put on the table in the red box.” Matching, sorting, identifying and naming of: Objects - jackets, shoes, socks, lunch boxes – as part of packing out/away during morning/playtime/packing up time - during topic discussions, (i.e. clothes, toys, animals, food, etc.) Colours - building blocks; bean bags; etc. according to colours during construction free play - pegs; beads - during table activities to develop fine motor skills

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Number Concept (continued) - Enjoys finger play and rhythmic counting games. - Join in rote/ automatically counting up to five without thinking - expanding up to 10 Understanding of one-to-one correspondence in a range of contexts for example: matching objects such as cups to saucers, straws to drink from cartons; dividing/ sharing of sandwiches/ sweets / slices of fruit Count reliably to three, - Demonstrate an understanding of the concept of ‘more’ - Demonstrate an understanding of ‘less’ - Respond appropriately to key vocabulary and questions, for example, ‘How many?’ - Counting of objects 1 – 5 (e.g. to complete basic patterns/	Colours (continued) - Food stuff such as fruit or sweets - during mealtimes. - Clothes, toys, objects, animals, etc. – during topic discussion time Shapes - Putting shapes into a shape sorter, using 3D objects to build and manipulate. - Picking out all the round shapes in the classroom, finding shapes with straight edges, fitting shapes into matching holes. - Stamping shapes in sand and describing them, using a set of flat shapes to make pictures or patterns, naming some of the shapes used, identifying specific shapes from pictures, simple models or patterns. Counting - Counting of everyday objects, toys, cups, plates, clothes, cars; blocks etc. - Songs and rhymes; e.g. birthday songs, days of the week, - Read stories and look at picture books /pictures with objects with numbers – counting objects - During self-care activities -counting body parts <u>Actions</u>:- " show me and count" - point to body part, object or picture <i>name/ count</i> body part, object or picture <i>matching</i> body part, object or picture - Sorting, “posting”/throwing objects in containers (even when packing up) Counting reliably to three - make sets of up to three objects and use numbers to three in familiar activities and games for example, touching one, two, three items as an adult counts, counting toys or pictures, counting out sets of three, e.g. knife, fork and spoon Understanding more or less Practical activities during snack time for example, indicating that more cups, counters, and food items are required.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Number Concept (continued) • sequencing for vocational activities). • Recognizing numbers 1 – 5 (for vocational possibilities) <i>Please note that number concept is not applicable to individuals whom experience profound intellectual disability, but only apply to individuals with moderate to severe disability..</i>	

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Conceptual Skills: Same content and principles apply as for 0–6 year group. 1. Size concept 2. Colour concept 3. Shape concept 4. Number concept There could be attempted to work on children identifying the left/ right sides of the body - only for individuals who have been identified for possible sheltered employment in the future, as being able to identifying left and right will be beneficial when addressing their vocational skills.	GENERIC ACTIVITY: Sorting recycled bottle lids (big and small) Addressing size concept, matching/ sorting and attention.
	ADAPTED FOR AWARENESS GROUP: • Expose the child to the big bottle cap, by positioning it within his/her visual field. • Place the bottle cap in the child's hand. • Verbalise the characteristics of the bottle cap, e.g. it is round and green. • Introduce the small bottle cap and follows the same steps, explaining that the second bottle cap is smaller than the first.
	ADAPTED FOR TRANSITIONAL GROUP: • Introduce the first (bigger) bottle cap to the child/group, and encourage the child to explore the bottle cap by means of hand-over-hand support. • Introduce the second bottle cap (smaller), and position both bottle caps within the child's visual field. • Repeatedly verbalise the words “big” and “small” while showing the big and small caps to the children. • Expand the activity to other big and small objects within the class environment. • Instruct some of the children to hand the big/small cap to the carer when placed in front of them.
	ADAPTED FOR INTERACTIVE GROUP: • Position the big and small caps in front of the child/group and explain the difference with regards to their size. • The child is then instructed to place all the big caps in one container, whilst placing the smaller caps in another container.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Conceptual Skills: Same content and principles apply as for age group 7–14 yr. If an individual has not mastered some of the perceptual skills, please adapt activities/expectations so that the adolescent is still able to participate. The activities may be divided into different parts/steps, so that the adolescent is only expected to partake in the part that he/ she is able to do – focus on their strengths.	GENERIC ACTIVITY: Washing and packing away dishes Addressing sensory awareness, visual figure ground and matching/ sorting skills.
	ADAPTED FOR AWARENESS GROUP: • Support the adolescent to touch/explore and look at the different dishes, whilst verbalising what he/she is touching or looking at. • Place the adolescent's hands in the water, for the sensory experience thereof, and mention how the temperature of the water feels. • Allow them to touch the dishes in the water. • Dry their hands with a dishcloth and talk about the texture of the cloth and which part of the arm/ hands/ fingers are being dried.
	ADAPTED FOR TRANSITIONAL GROUP: • Provide hand-over-hand support to open the taps in order to fill the sink, after which the soap is put into the water. • Support the child to use a sponge to wash a plate, where after physical support is withdrawn. • Observe if the adolescent copes with only verbal guidance. • The same steps with regards to physical and verbal support apply to the drying of the dishes. • Only use easy washable/dryable dishes for this group, e.g. plates, bowl, forks and spoons. • Avoid sharp knives and thin glasses which could break/crack and lead to injury
	ADAPTED FOR INTERACTIVE GROUP: • Provide verbal support regarding the opening of the hot and cold water (mention the danger of using hot water) and using a sufficient amount of soap.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Conceptual Skills: (continued)	ADAPTED FOR INTERACTIVE GROUP: (continued) • A tablespoon or small measuring cup can be supplied to be filled with soap before mixing it into the water. • Again provide verbal guidance during the washing and drying of the dishes. • Adolescent must attempt to group all the similar dishes together, e.g. all the plates, mugs and spoons separately. • If he/she no longer requires verbal guidance (or experiences hearing problems), a visual schedule can be introduced. • The visual schedule must include photos of the activity, placed in the specific steps/sequence of the task. • An occupational therapist and/or speech therapist can be consulted to support the carer with designing a visual schedule.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Visual Perceptual skills: Visual/object constancy The ability to recognize objects even as their size, shape, colour or orientation change – e.g. one chair may be big and made of wood, whilst another chair may be small and made of plastic – both are however chairs. Visual Figure Ground The ability to locate an object within a busy visual field, without getting confused by the background or surrounding images – e.g. finding a ball in a toy box. Individuals with attention problems often struggle with this visual perceptual skill.	GENERIC ACTIVITY: Individual gathers a chair and places on the carpet (or in the corner) for morning ring participation. Addressing position-in-space and spatial relations (up, down, towards).
	ADAPTED FOR AWARENESS GROUP: • Provide hand-over-hand support to enable the child to accurately grasp and move his/her chair to the appropriate area. • Children in buggies/wheelchairs may be pushed to the morning ring area, whilst the carer verbalises what spatial actions are being performed, for ex “I am turning you around and pushing you into the corner/ onto the carpet”.
	ADAPTED FOR TRANSITIONAL GROUP: • At first, physically direct the child to the chair, • then remove physical support whilst verbalising or gesturing where he/she is to place their chair • and then encouraged to sit on the chair..
	ADAPTED FOR INTERACTIVE GROUP: • The child must be able to independently gather his/her chair and place it in the correct position on the carpet. • Thereafter he/she sits in preparation for the morning ring to start.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 0–6 years	ADDITIONAL ACTIVITIES AND IDEAS FOR INTEGRATION IN THE DAILY PROGRAMME
Spatial Skills • Position-in-Space - enables the child to orientate his body in space, e.g. sit <i>on</i> the chair.. • Visual spatial relations Child is able to orientate/ place objects in relation to each other or the environment, e.g. place the one block on top of the other. (This includes the ability to copy) Patterns and sequencing - e.g. copy a pattern of different coloured beads to make a necklace. The ability to recognize basic patterns and sequences may assist some children to learn and understand other sequences like the days of the month, routines and visual schedules. Time sequencing Show awareness of time, through some familiarity with names of the days of the week and significant	Spatial Skills: • Explore the position of objects with visual, kinaesthetic (movement, touch, making us of the body and muscles) and listening input. • Practise on a daily basis is needed to reinforce learning. • Use in functional, practical context e.g. responding to a request: Put - cup IN basin; doll ON chair; lift arms UP; crawl UNDER table etc. • Use visual and verbal cues to assist/improve vocabulary. • Stacking or joining objects or using construction materials. • Placing objects in and out of containers, placing objects inside and outside a hoop, fitting as many objects as possible into a box. • Moving self 'forwards' and 'backwards' on request and moving an object forward and backwards during activities. • Music ring – copying drumbeats - copying a simple pattern of repeated movements (dancing) or clapping. Patterns and sequencing <i>Please note that sequencing is not applicable to individuals whom experience profound intellectual disability, but only apply to individuals with moderate to severe intellectual disability.</i> Sequencing of Time: During the morning ring specific attention should be given spend on creating awareness of time through: • Ordering events in each day on a visual daily timetable, • Naming days of the week, discussing, singing songs, doing activities to strengthen understanding and use. • Weather, names of the months (songs) - photos of birthday friends on month.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS

ADDITIONAL ACTIVITIES AND IDEAS

Time sequencing (continued)

times in the day, such as **meal times, bed times**, outside play, home time

- Begin to sometimes use reference to time (Weather / days of week/ birthdays - daily calendar/ events)
- Start to understand concept of time and use in daily conversation

- Incidental introduction and reference to time during the day – time of the day (morning, afternoon, evening), time on a watch (hours in relation to favourite tv-programmes, bedtimes, school time, etc.)
- Incidental reminders; referring to time; events related to time during the day.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Visual Perceptual skills: Visual/ object constancy The ability to recognize objects even as their size, shape, colour or orientation change – e.g. one chair may be big and made of wood, whilst another chair may be small and made of plastic – both are however chairs. Visual Figure Ground The ability to locate an object within a busy visual field, without getting confused by the background or surrounding images – e.g. finding a ball in a toy box. Individuals with attention problems often struggle with this visual perceptual skill.	GENERIC ACTIVITY: Jumping inside and out of a hoola-hoop or circle. Addressing position-in-space, depth perception and spatial relations (in, out, next to, over, in front, behind).
	ADAPTED FOR AWARENESS GROUP: • Provide hand-over-hand support to guide the child to jump into and out of the circle. • Child may only have to step into or out; crawl into/out if he/she has physical difficulty jumping. • Children seated in a buggy/wheelchair may be pushed into/out of the circle, whilst the carer verbalises what actions are being performed with the child.
	ADAPTED FOR TRANSITIONAL GROUP: • Initially provide hand-over-hand support to guide the child into the circle. • Extract physical support and verbally (or using gestures) guide the child to jump out of the circle. • Child may also be instructed to lift the hoola-hoop above his/her head, and then let go to observe how the hoola-hoop falls to the ground.
	ADAPTED FOR INTERACTIVE GROUP: • Verbally instruct the child to jump into and out of the circle. • Child may be instructed to jump/step/crawl in front or behind and over the circle. • Children who are not able to walk/ jump, but whom have floor mobility – can make their way into and out of the hoola-hoop using their own preferred method of floor mobility. • The carer can challenge specific children and place a few hoola-hoops in a straight row. • After the instruction is given, start at the one end and jump their way in and out of the hoola-hoops until they reach the hoola-hoop on the opposite end.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 7–14 years	ADDITIONAL ACTIVITIES AND IDEAS FOR INTEGRATION IN THE DAILY PROGRAMME
Spatial Skills • Position-in-Space (enables the child to orientate his body in space, e.g. sit <i>on</i> the chair) • Visual spatial relations (child is able to orientate/ place objects in relation to each other or the environment, e.g. place the one block on top of the other). This includes the ability to copy. Patterns and sequencing, e.g. copy a pattern of different coloured beads to make a necklace. The ability to recognize basic patterns and sequences may assist some children to learn and understand other sequences like the days of the month, routines and visual schedules.	Spatial Skills: • Explore the position of objects with visual, kinaesthetic (movement, touch, making us of the body and muscles) and listening input. • Practise on a daily basis is needed to reinforce learning. • Use in functional, practical context e.g. responding to a request: Put - cup IN basin; doll ON chair; lift arms UP; crawl UNDER table etc. • Use visual and verbal cues to assist/improve vocabulary. • Stacking or joining objects or using construction materials. • Placing objects in and out of containers, placing objects inside and outside a hoop, fitting as many objects as possible into a box. • Moving self 'forwards' and 'backwards' on request and moving an object forward and backwards during activities. • Music ring – copying drumbeats • copying a simple pattern of repeated movements (dancing) or clapping Patterns and sequencing: <i>Please note that sequencing is not applicable to individuals whom experience profound intellectual disability, but only apply to individuals with moderate to severe intellectual disability</i>

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 7–14 years	ADDITIONAL ACTIVITIES AND IDEAS FOR INTEGRATION IN THE DAILY PROGRAMME
Time sequencing Show awareness of time , through some familiarity with names of the days of the week and significant times in the day, such as meal times, bed times , outside play, home time • Begin to sometimes use reference to time (Weather / days of week/ birthdays - daily calendar/ events) • Start to understand concept of time and use in daily conversation	Sequencing of Time: During the morning ring specific attention should be given spend on creating awareness of time through: • Ordering events in each day on a visual daily timetable. • Naming days of the week, discussing, singing songs, doing activities to strengthen understanding and use. • Weather, names of the months (songs) - photos of birthday friends on month. • Incidental introduction and reference to time during the day – time of the day (morning, afternoon, evening), time on a watch (hours in relation to favourite tv–programmes, bedtimes, school time, etc.) • Incidental reminders; referring to time; events related to time during the day.
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Visual Perceptual skills: • Visual/ object constancy The ability to recognize objects even as their size, shape, colour or orientation change – e.g. one chair may be big and made of wood, whilst another chair may be small and made of plastic – both are however chairs.	GENERIC ACTIVITY: Setting the table for lunch. Addressing position-in-space, depth perception, and spatial relations (on, next to, left). ADAPTED FOR AWARENESS GROUP: Hand-over-hand support is supplied to enable the child to place a bowl and/ or spoon on the table (does not necessarily have to set the table for the whole group of individuals). The carer slowly verbalises step-by-step what actions she is supporting the adolescent to perform. If the individual is not able to access the table, due to being positioned in a seating device, the carer can support the adolescent to set his own tray table with his/ her eating utensils.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Visual Figure Ground The ability to locate an object within a busy visual field, without getting confused by the background or surrounding images – e.g. finding a ball in a toy box. Individuals with attention problems often struggle with this visual perceptual skill. • Spatial Skills ○ Position-in-Space (enables the child to orientate his body in space, e.g. sit <i>on</i> the chair) ○ Visual spatial relations (child is able to orientate/ place objects in relation to each other or the environment, e.g. place the one block on top of the other). This includes the ability to copy Patterns and sequencing, e.g. copy a pattern of different coloured beads to make a necklace.	ADAPTED FOR TRANSITIONAL GROUP: Adolescent must be able to place a spoon with every bowl on the table (does not necessarily have to set the table for the whole group of individuals if attention is decreasing). The carer may provide initial hand-over-hand support, after which she continues with verbal guidance and gesturing throughout the setting of the table. Let the adolescent first place the bowls for every group member, followed by the placing of spoons next to every bowl.
	ADAPTED FOR INTERACTIVE GROUP: Individuals must be able to place a spoon on the left side of every bowl (carer can demonstrate initially which is the left side, by setting the first spoon and bowl). Individual must be able to recognize if somebody at the table is missing an eating utensil. The carer may need to provide verbal guidance throughout the setting of the table.
	ADDITIONAL ACTIVITIES AND IDEAS FOR INTEGRATION IN THE DAILY PROGRAMME Spatial Skills: • Explore the position of objects with visual, kinaesthetic (movement, touch, making use of the body and muscles) and listening input. • Practise on a daily basis is needed to reinforce learning. • Use in functional, practical context e.g. responding to a request: Put - cup IN basin; doll ON chair; lift arms UP; crawl UNDER table etc. • Use visual and verbal cues to assist/improve vocabulary. • Stacking or joining objects or using construction materials. • Placing objects in and out of containers, placing objects inside and outside a hoop, fitting as many objects as possible into a box.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 15 – 18 years	ADDITIONAL ACTIVITIES AND IDEAS
○ Visual spatial relations (continued) The ability to recognize basic patterns and sequences may assist some children to learn and understand other sequences like the days of the month, routines and visual schedules. Patterns and sequencing, e.g. copy a pattern of different coloured beads to make a necklace. The ability to recognize basic patterns and sequences may assist some children to learn and understand other sequences like the days of the month, routines and visual schedules. Sequencing of time Show awareness of time, through some familiarity with names of the days of the week and significant	Spatial Skills (continued) • Moving self 'forwards' and 'backwards' on request and moving an object forward and backwards during activities. • Music ring – copying drumbeats • copying a simple pattern of repeated movements (dancing) or clapping Patterns and sequencing <i>Please note that sequencing is not applicable to individuals whom experience profound intellectual disability, but only apply to individuals with moderate to severe intellectual disability.</i> Sequencing of Time: During the morning ring specific attention should be given spend on creating awareness of time through: • Ordering events in each day on a visual daily timetable. • Naming days of the week, discussing, singing songs, doing activities to strengthen understanding and use. • Weather, names of the months (songs) - photos of birthday friends on month.

VISUAL PERCEPTUAL AND COGNITIVE SKILLS	
Content 15 – 18 years	ADDITIONAL ACTIVITIES AND IDEAS FOR INTEGRATION IN THE DAILY PROGRAMME
Time sequencing (continued) times in the day, such as meal times, bed times, outside play, home time • Begin to sometimes use reference to time (Weather / days of week/ birthdays - daily calendar/ events) Start to understand concept of time and use in daily conversation	• Incidental introduction and reference to time during the day – time of the day (morning, afternoon, evening), time on a watch (hours in relation to favourite tv-programmes, bedtimes, school time, etc.) • Incidental reminders; referring to time; events related to time during the day.

3. LIFE SKILLS

3.1 WHAT IS LIFE SKILLS?

Life Skills is central to the holistic development of children with SPID, and includes the following skills: Social, emotional, personal/ self-care, sensory, perceptual and physical. Life Skills is a cross cutting subject that should support and strengthen the development, learning and applying of knowledge in other core subjects, namely Communication and Language as well as Visual Perceptual and Cognitive Skills. Life Skills becomes the backbone for supporting, teaching and learning.

3.2 SPECIFIC AIMS OF LIFE SKILLS

Supporting the child with SPID to understand himself and to develop and learn physically, emotionally and socially with others through:

- Increasing independence in all aspects of daily physical care, health and safety - with increasing participation;
- Assisting with some daily work tasks at home and the learning centre, like carrying dishes to the table;
- Performing simple actions with objects as the basis of participation in some self-care and vocational activities - with high levels of ongoing support;
- Recreational activities that involve, for example: enjoyment in listening to music, watching movies, going out for walks or participating in water activities - all with the support of others;
- Reducing maladaptive behaviour to the minimum;
- Appropriate sexuality and relationship education; and
- Establishing appropriate relationships with community members, family, caregivers and peers.

WORKING WITH CHILDREN WITH SPID

Most children with SPID need high levels of support in caring for either a longer period of time or even on a permanent basis. This would mean that a substantial part of their day could be spent on caring activities such as feeding, toileting and dressing. In order for children with SPID to achieve maximum independence and attain certain life skills, it is necessary that the individual with SPID is exposed to supported and guided opportunities throughout the daily programme – both at the learning centre and in their home environment. The learning centre personnel and immediate family

members should create a team effort with regards to the development of individuals with SPID's life skills.

It has to be kept in mind that a high number of children with SPID also experience co-morbidities, with the presence of one or more additional disabilities or syndromes. The additional disabilities could have an influence on their physical, sensory and functional ability, for example Cerebral Palsy (CP), blindness and deafness. A multi-disciplinary team approach is needed to determine the level of support and would in most instances include therapeutic intervention. This needs to be taken into consideration when planning for educational programmes and learning opportunities.

When working with persons who needs support in self-care or is fully dependent on others for caring, it is important to have knowledge about policies regarding the management of these individuals and correct procedures, with regards to health and safety. Some important factors are the wearing of protective and appropriate gloves during feeding, nappy changing, supported toileting, brushing of teeth etc. Correct waste management is very important. See policies from DOH, DSD, DBE and NGO where applicable.

LIFE SKILLS CONSISTS OF:

SENSORY SKILLS

These refer to the use of vision, hearing, movement, touch, smell and taste. This conscious recognition and interpretation of sensory stimuli/ information from the external world serves as a basis for understanding, learning or motivating a particular action or reaction. These are actions or behaviours a person uses to locate, identify, respond, select, interpret, associate, organise and remember sensory events based on discriminating experiences. Sensory integration refers to how people use the information provided by all the senses, experienced by the body and from the external environment.

MOTOR SKILLS

Motor skills are motions carried out when the brain, nervous system and muscles of the body work together. Motor skills include two groups of muscles: gross or large muscles and fine or smaller muscles.

- **Gross motor skills** refer to the using of the large muscles of the body - which are concerned with posture and movements of the arms, legs, feet or entire body. This would include head control, rolling, creeping, sitting, crawling, standing, walking and running.

The focus of physical activities is to:

- acquire and develop physical skills including mobility and voluntary control of movement;
 - develop physical stability and postural endurance;
 - participate in recreational activities, including sports and games;
 - attainment of physical skills and maintenance of postures for optimal independence in activities of daily living (ADL's); and
 - promote optimal level of physical health and well-being through optimal positioning, comfortable handling and individualized programs to address child-specific needs of especially the child with severe and profound physical disabilities (for instance prevention of contractures).
-
- **Fine Motor skills** refer to small movements, using the small muscles of the hands, fingers, toes and wrists. These muscles are involved in eye-hand coordination and tasks which require the individual to feel, grasp and manipulate objects, for example when picking up and manipulating small items such as spoon or fastening buttons. Eye-hand coordination is the simultaneous complex coordination of the actions of the eyes and the hands performing precise manipulative movements. Development of intentional fine motor control and movement ranges from early developing behavioural patterns to finely tuned, highly complex functional activities such as threading large beads, building with blocks, paging through a book, holding and cutting with scissors along a line, holding and using drawing and writing utensils sufficiently.

SELF-CARE SKILLS (PERSONAL)

These are skills needed to independently take part in basic self-care activities such as bathing, feeding, brushing teeth and dressing. These skills are also known as activities for daily living (ADL). As the child becomes more independent in these basic self-care skills, he/she should be more exposed to helping with domestic activities such as taking his/her plate to the kitchen, making a sandwich or setting a table for mealtimes. These activities are fundamental to living in a social world and are aimed at functioning as independently as possible within their immediate environment – or in the broader community, such as a sheltered employment setting.

SENSE OF SELF AND SEXUALITY

Sexuality incorporates characteristics of who a person is as a human being, as a man or a woman. It includes biological, psychological, social and cultural components. Children with SPID should be seen as maturing individuals who need information and skills that can help support them to develop from child-like behaviours to more age-appropriate behaviours.

Learning about sexuality happens naturally every day as people live and interact with the world around them. This starts from caring, touching, cuddling and gentle caressing through which the child knows that he is loved and valued. Through this the foundation of how a child learns to show affection and touch in his/her life is developed. They also learn about sexuality by observing others around them and being exposed to attitudes and behaviour, such as understanding gender roles, by watching men and women in their lives. Sexuality education should be integrated within everyday ADLs (toileting, dressing, etc.) and social and emotional skills development.

Discerning the appropriate focus of sexuality education for children and youth with more severe levels of cognitive impairment, namely the high support needs of those with SPID disability, is difficult due to their significantly-reduced understanding and autonomy.

A person with profound intellectual disability cannot understand verbal requests, has very limited communication and self-care skills and could be incontinent.

People with disabilities may have:

- difficulty learning;
- limited genital and other tactile sensations;
- communication problems; and
- uncertainty about their sexual function and fertility status

Issues that may hinder development of healthy body image and self-concept include:

- use of braces, crutches, wheelchair;
- bladder and bowel management routines;
- physical differences from peers (atrophy);
- diminished gender role expectations from society; and
- mistrust of own body.

One approach is to engage caregivers to acknowledge and understand the rights and needs of this group in relation to their sexual health.

The issues related to 'recognising the dignity' and sexual health rights of these individuals are complex and need urgent engagement in relation to policy and practice in learning centres. Supporting children in their sexual development and learning, needs to be discussed with parents and should be seen as a team effort. Important factors concerning sexuality are:

- Ensuring that children with SPID are at all times being kept safe and under adult supervision, limiting opportunities for possible exploitation and sexual abuse;
- Use appropriate language for body parts and bodily functions. Children using appropriate language are more likely to report abuse if it occurs;
- Allowable Sexual Expression: Children should not be hugged, caressed, massaged, kissed or embraced by peers and carers. Exceptions include need for physical calming. If a carer is

inappropriately touched by a child, firmly let the child know that the touch is inappropriate, Distinguish between appropriate and non-appropriate touching;

- Respecting normal sexual behaviour needs (masturbation) by establishing appropriate private times; and
- Respecting their privacy and treating them with dignity and respect especially if they need support in self-care.

Sexuality Education for children with disabilities requires some degree of individualization. An ISP must be used as an instrument for adapting sexuality curriculum. If human sexuality education is written into the ISP, it is more likely to be designed and delivered around the unique needs of the student.

It is strongly advised that all staff should be trained in programmes on supporting and managing of sexuality development of children with SPID. Training programmes and training are available from various organisations, for example Western Cape Forum for Intellectual disability (WCFID) and Cape Mental Health (CMH). (See Resource list in the guidelines for more information).

SOCIAL SKILLS AND RELATIONSHIPS

We live in an interactive society where a person needs to be able to communicate and interact with others. Societal rules guide how to relate to others, how to express emotions, closeness and affection in a socially acceptable manner. In order for a child to become a functional member of society, the acquiring of social skills are needed.

EMOTIONAL SKILLS

Emotional skills refer to the ability to show and control feelings and emotions that are socially acceptable and age appropriate. A child's overall development is affected by his/her relationships and feelings. A child's sense of self and the ability to form relationships with others are shaped in the interactions with others on a day-to-day basis. The emotional well-being of a child supports the way in which he/she explores and makes sense of his environment. Both the social and emotional life of a child is rooted in family and culture specific standards and needs to be respected.

Thinking, appreciation and evaluation skills develop when they take part in challenging activities. Gross motor movements along with music, songs and rhymes provide children the opportunity to express themselves and be creative. As they work individually and as part of a team, this builds their self-confidence and self-esteem.

3.3 LIFE SKILLS : CONTENT AND ADAPTED ACTIVITIES

LIFE SKILLS: SENSORY

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Hearing - Respond to sound - Localize sound - Recognize sound <i>Please note: some individuals may be sensitive to certain sounds or loud auditory stimuli.</i> See (visual): - Focus on an object - Eye contact - Follow of object with eyes: - close/far distance	GENERIC ACTIVITY: Sand play Addressing: touch (tactile), movement, basic concepts (empty/full; shapes) and spatial relations.
	ADAPTED FOR AWARENESS GROUP: - Place the child's hands inside the sand and encourages playful movements with the hands and fingers in the sand. - Verbalise what the child's hands are experiencing, e.g. "the sand is soft and cold". - Support the child with rubbing the sand between his/her hands and/or feet.
	ADAPTED FOR TRANSITIONAL GROUP: - Encourage the child to play with his/her hands in sand. - Hand-over-hand support and verbal guidance may be provided to guide the child to use a small shovel to dig and transfer sand into a bucket. - Demonstrates the difference between two buckets to the child, one being <i>empty</i> and one being <i>full</i> of sand.

LIFE SKILLS: SENSORY	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
See (visual) (continued) - different planes (vertical/ horizontal/diagonal and circular) • Eye-hand coordination (addressed under Fine Motor area) Taste and Smell • Sweet • Sour • Salty • Spicy • Bitter • Dry <i>Please note: Certain children may experience oral-sensory hypersensitivity. Consult a Speech Therapist or Occupational therapist if oral hypersensitivity is expected.</i>	ADAPTED FOR INTERACTIVE GROUP: • Draw a basic shape in the sand while the child is looking (e.g. a straight line, circle or square) • Encourage the child to copy the same shape in the sand, using his/her fingers.

LIFE SKILLS: SENSORY	
Content 0-6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Touching/Tactile: Plays an important role in body awareness, emotions, visual perception and motor skills • Light touch • Deep pressure • Pain • Temperature <i>Please note: This is a specialized area as certain individuals may be hypersensitive to certain textures and tactile input. Consult an Occupational therapist if hypersensitivity is suspected.</i> Proprioception • Any movement of muscles and joints. • The sense of movement through space, e.g. one is able to close ones eyes and still be	

LIFE SKILLS: SENSORY

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Proprioception (continued) • aware of the position of each body part as well as movement of the limbs through space. Vestibular/ movement: Important for regulation of muscle tone, posture, balance, spatial experience, eye movements and emotional stability. Includes the following movement patterns: • Forwards/ backwards • Sideways • Up and down • Rolling • Swinging • Spinning <i>Please note: this is a very specialized area, as certain individuals may be hypersensitive to certain movement patterns.</i>	

LIFE SKILLS: SENSORY	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
The same content and principles apply as for above age group 0-6 year.	GENERIC ACTIVITY: Morning ring song Addressing: attention, memory, eye-contact, hearing, listening, movement and vision.
	ADAPTED FOR AWARENESS GROUP: • Make eye contact with the child whilst carer is singing the morning ring song. • Physical support of the child to make the song's actions with his/her body may be necessary – if the child does not indicate resistance.
	ADAPTED FOR TRANSITIONAL GROUP: • Child might remember certain parts or actions of the song (usually performing actions in their own unique way). • Less physical and verbal support can be given. • The song should be short and actions simple, as concentration is limited. • Sing and perform the actions slowly, to give the group enough response time.
	ADAPTED FOR INTERACTIVE GROUP: • Song can be extended in length and actions may be more complex. • Child may be encouraged to copy a simple beat (pattern). • Carer can provide certain contextual clues, during familiar activities, to indicate the start of a song – e.g. waving a toothbrush in the air before the tooth brushing routine, to indicate that the group is going to sing the tooth brushing song.

LIFE SKILLS: SENSORY	
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
The same content and principles apply as for above 0–6 year group.	GENERIC ACTIVITY: Raking/picking up leaves in the garden Addressing tactile, movement, hearing, seeing and smell. (Can be linked to the topic of seasons and specifically done during fall/winter)
	ADAPTED FOR AWARENESS GROUP: • Explain to the children what the activity will entail, namely going outside to the garden to pick up leaves. • Physically support the adolescent to grasp and/or touch a leaf. • Position the leaf in front of the adolescent and verbalises the characteristics of the leaf “Look at the leaf, it is brown and fell off the tree”. • Attempt to position the leaf near the adolescent’s nose and encourage him/her to smell the leaf. • The same activity can be executed using flowers or herbs (basil/mint/coriander).
	ADAPTED FOR TRANSITIONAL GROUP: • Explain that they are going outside to the garden to pick up some leaves. • Leaves can be used at a later stage for a craft-activity during the day/week, e.g. making cards or bookmarks. • Show and talk about what a leaf looks like and that it fell off the trees/bushes. • Provide hand-over-hand support to adolescents who struggle to identify/pick up the leaves. • Demonstrate how she explores the leaf by using her tactile (touch), visual and smell senses, whilst she verbally encourages the group to copy her.

LIFE SKILLS: SENSORY	
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP: • Demonstrate how a rake is used to gather fallen leaves into a pile, • Followed by transferring the leaves with a shovel or their hands into a refuse bag. • Adolescents are expected to independently participate the activity, although with supervision • and verbal guidance is necessary. • A visual schedule (including photos of the activity steps in the specific sequential order) can be supplied if the adolescent experiences a hearing impairment – or to allow for the carer to provide less verbal support as to enhance the adolescent's independence during the task.

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS

The gross motor development of children with SPID can range from being able to walk and move (mobile) to being non-mobile needing physical assistance and assisted technology. Normal (neuro-typical) gross motor development describes the stages, like rolling and crawling, a child moves through in their first few years of life until he/she can walk, run, jump and combine movements in a huge variety of possible combinations. (See annexure 4 which includes a broad outline of normal motor development – David Werner)

Atypical development refers to development that does not follow the stages of normal development, usually due to abnormalities of the brain and nervous system. Some children with SPID might broadly seem to attain developmental milestones, but they would be grossly delayed and/or certain elements might not be achieved as expected in the typical developing child.

Resultantly, children with SPID present with huge variations in physical capabilities. Some children may only experience limitations with fluency or rhythmic combinations of movement which requires co-ordination, motor planning and using the two sides of the body together to make different motor actions. Others show limitations in balance which affects all functional activities, for e.g. stair climbing and/or endurance when walking long distances. A few might be able to walk using a hand-held mobility device or can gain some level of independence with the help of a manual or powered wheelchair. Many have mastered various methods of moving on the floor and uses rolling, creeping, crawling or walking on their knees as their primary method of mobility. Of those unable to mobilize independently, some children can sit on their own or needs limited support to sit. Children with severe physical disabilities need supported seating and positioning devices, splints and other assistive devices for optimal individual functioning. Children with severe limitations in head and trunk control require extensive assisted technology and physical assistance. (See Annexure 2: Picture based overview of gross motor function).

Thus, optimal acquisition of motor skills is the main objective of activities in the early developmental years. See Annexure 2: Picture based overview of gross motor function and skills to develop).

The content of gross motor activities would therefor need to be determined by or adapted according to age, motor capacity and the ability to participate. The layout of the LP for gross motor is therefore different than for the other subject areas. Groups are formed according to the amount of support required to participate, whether due to physical disability or ability to participate (awareness, transitional or interactive). The huge variations in the acquisition of physical skills is most marked in the first years of life. Therefor the age group 0-6 year old is further divided into

different age categories to accommodate for this fact. A keen eye also needs to be developed to identify barriers to participation and initiative is required to address these barriers. Possible barriers will be discussed and expanded on in the manual. Only then can appropriate and optimal participation to physical activities be realised for each child. In these instances individualised support to enhance physical skills and physical well-being is needed and should be added to the child's ISP (managed by district support team)

In the chronological age group of seven years and older the content and aim of activities may be to consolidate the skills acquired. The implication of atypical motor development is such that some individuals might still acquire a new skill. Age appropriate activities should be employed for improving or maintaining optimal functional skills in the adolescent.

The content of gross motor activities would therefore need to be determined by or adapted according to age, motor capacity and the ability to participate. To ensure appropriate and optimal participation, barriers to participation need to be identified and initiative is required to address these barriers. In these instances individualised support to enhance physical skills and physical well-being is needed and should be added to the child's ISP (managed by district support team)

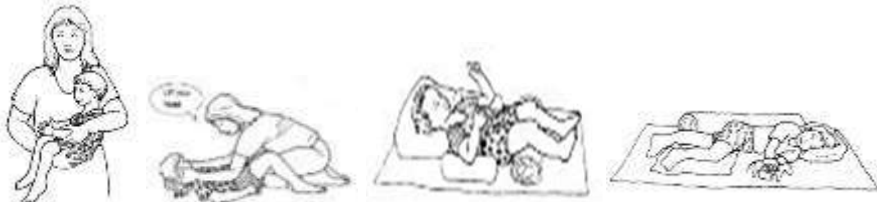
The adolescent with SPID and multiple physical disabilities still require physical activities and/or physical intervention in order to:

- maintain the ability to acquire and maintain positions required for ADL's
- maintain optimal levels of cardiovascular health, muscular-skeletal health and general physical well-being
- maintain optimal levels of voluntary control of movement necessary for choice-making and optimal independence, despite the constant support required

In caring for the adolescent, moving into adulthood, the level of physical strain on the care worker usually increases. It is of utmost importance that staff and carers are trained in kinetic handling and ergonomic principles.

The physical environment can pose a risk to the child's physical well-being and regular risk assessments by learning centre during all activities of the daily programme should be performed. Training all stake holders in this regard would be a prerequisite.

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS

Developmental Milestones 0-2 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Head and neck control • Trunk control and sitting • Moving between positions through crossing of the midline - Rolling - Lying to sitting - Sitting to standing • Mobility - Rolling - Creeping - Crawling - Walking • Upper limb control - Reach including crossing of the midline - Weight bearing	GENERIC ACTIVITY: This age category would benefit from individualised programmes aimed at improving functional abilities as this is when the child's development is crucial. This would require the assistance of a therapist to help work out a programme and to train the carer in implementation thereof.
	FULL SUPPORT: Physical disabilities limit voluntary control of movement. Infants are unable to maintain antigravity head and trunk postures in lying on the stomach and sitting. Infants require adult assistance to roll. For this age group, carrying, handling and positioning is crucial to • Prevent contractures • Promote comfortable handling • Position for ADL's • Position for play and communication  Focus of activities: Stimulate and support voluntary movements of the limbs

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS

Developmental Milestones	Activity to enhance and support development
0–2 years	(See Manual for integrated programmes within the Daily Programme)
(continued)	ASSISTANCE REQUIRED: Infants have head control but trunk support is required for floor sitting. Infants can roll onto back and may roll onto stomach. Some may creep forward on their stomachs. For this group, carrying, handling and positioning is still crucial to • Prevent contractures • Promote comfortable handling • Position for ADL's • Position for play and communication The aim would be to develop an optimal level of head and trunk control. Much of this can be achieved through positioning, carrying and handling, but individual attention is required. Focus of activities: Stimulate active voluntary movements  Individual strategies and techniques to facilitate rolling and creeping can be shown by a physiotherapist.

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS

Developmental Milestones 0-2 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued)	ASSISTANCE REQUIRED: (continued)
	MOVING TOWARDS INDEPENDENCE: Infants maintain floor sitting but may need to use their hands for support to maintain balance. Infants creep on their stomach or crawl on hands and knees. Infants may pull to stand and take steps holding on to furniture. Some might move in and out of sitting and floor sit with both hands free to manipulate objects. Some infants crawl on hands and knees, pull to stand and take steps holding on to furniture. These infants would walk between 18 months and 2 years of age without the need for any assistive mobility device.

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS

Developmental Milestones 0–2 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued)	MOVING TOWARDS INDEPENDENCE: (continued)  Focus of activities: • Participate in physical activities and move freely using their preferred method of mobility • Develop optimal postural endurance • Develop the ability to move between positions Individual strategies and techniques to facilitate optimal development of a variety of skills can be demonstrated by a physiotherapist.
Developmental Milestones 2–4 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Head and neck control • Trunk control and sitting • Moving between positions • Rolling • Lying to sitting • Sitting to standing	Morning/social ring: Maintain floor sitting (long sitting with legs apart) in small group and roll ball towards other children ADAPT FOR FULL TIME HAND-OVER-HAND SUPPORT: (Awareness group and according to physical assistance required) • Child is carried to the floor or pushed to the group activity in a buggy. • The child with no head control should preferably not be positioned on a carer's lap. • Assistive devices which gives full support due to difficulties in both trunk and head control, are used. • This could include buggies, side-lyers and possibly standing frames.

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS

Developmental Milestones	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
2–4 years • Mobility - Rolling - Creeping - Crawling - Walking • Upper limb control - Reach and manipulate - Weight bearing	ADAPT FOR FULL TIME HAND-OVER-HAND SUPPORT: (continued – morning ring/social ring) • Splints or other communication devices should be used as specified individually. • In the absence of assistive devices, great care should be taken to ensure constant and total support to maintain alignment of the trunk and especially the head and neck. • Children should not be pulled up or picked up by holding onto the hands and arms. • This applies to carrying, handling and while positioning. • <u>Option 1:</u> Carry child to floor and position in side-lying. Using full hand-over-hand support, facilitate rolling of the ball to a peer. • <u>Option 2:</u> Position two children in buggies adjacent or opposite each other. Each child is supported from behind by a carer providing full hand-over-hand support
Functional activities • Walks, stops and starts safely • Pushes and pulls large toys, boxes and light objects around the floor • Crawl up stairs or by holding onto a rail with assistance	ADAPT ACCORDING TO SUPPORT AND PHYSICAL ASSISTANCE REQUIRED: Mobile child (transitional): • The mobile child within the transitional group can be facilitated to walk to the floor and given the instruction to sit. • Use should be made of short phrased commands, reinforced with gestures and hand signs. • Time should be allowed so that the child can process the command. • Physical support should be rendered only when required to attain the sitting position. • The carer can position herself behind or beside the child and use hands on support to facilitate the rolling of the ball. • This action should be repeated a few times. • Gradually the amount of support should be reduced to hands on – hands off support where time is allowed for the child to attempt rolling the ball.

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS

Developmental Milestones	Activity and adaptations to enhance and support development
2–4 years	(See Manual for integrated programmes within the Daily Programme)
 M-sit or W-sit	Child with limitations to physical abilities: (can be in the Transitional or Interactive group) • Child is carried to and placed in sitting on the floor. • Alternatively a child can be encouraged to creep or roll towards the group. • If a child can walk short distances with the help of a walking device, this should be encouraged • The child with head control, is able to sit on the floor, but needs his/her hands to help with balance, can be positioned to sit in front of or on the lap of a carer. • The amount of physical support rendered should be kept to a minimum. • Stabilize the child by holding at the level of the lower back or at both thighs, in order to free up the hands for rolling the ball. • Only after the child has attempted or partially completed the rolling action, could the ball be redirected so that another child can take a turn. • The M-sit or W-sitting (see picture) should be discouraged at all times. • If, due to contractures and deformities, this is the only position a child can independently attain on the floor, however, a child should be allowed to participate in this position. • Once a child has undergone surgery, the child should not be allowed to revert to this position. MOBILE INTERACTIVE CHILD, MOVING TOWARDS INDEPENDENCE: • The child can either walk or crawl towards the activity. • Some children might still show signs of difficulty maintaining balance in sitting. • No physical support should be required to maintain floor sitting, but encouragement to maintain this position for increasing periods might be required. • No physical support should be required to roll the ball towards a peer.

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS	
Developmental Milestones 2–4 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	MOBILE INTERACTIVE CHILD, MOVING TOWARDS INDEPENDENCE: (continued) • The interactive child making use of alternative methods of mobility, should be allowed time and space to use their walking device or wheelchair. • They should be given time to complete the transition to and from the floor at their own pace and might require minimal physical assistance due to their physical disability. • They should be able to roll or pass the ball independently to a peer.
ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS	
Developmental Milestones 4–6 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Head and neck control • Trunk control and sitting – including chair sitting • Moving between positions - Rolling - Lying to sitting – including chair sitting - Sitting to standing – including chair sitting	GENERIC ACTIVITY: (The areas and objects used must relate to the theme of the week) Outdoor obstacle course: • Make use of tree logs. • Climb onto and over big obstacles like tree logs/trunk or over small branches. • Explore the environment. • Replace with stairs or chairs when indoors or in the event that tree trunks are unavailable. • Children climb onto high and low chairs.

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS

Developmental Milestones 4–6 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Mobility - Rolling - Creeping - Crawling - Walking - Running • Balance and agility - Walking backwards - Jumping - Stand on toes • Upper limb control - Reach, manipulate and co-ordinate; - Weight bearing	ADAPT FOR FULL TIME HAND-OVER-HAND SUPPORT: (Awareness group and according to physical assistance required) • Child is pushed in the buggy towards the tree log and transferred into a fully supported sitting position on the log. • Great care should be taken to ensure constant and total support to maintain alignment of the trunk and especially the head and neck. • The child should not be transferred out of the buggy if the amount of head lag cannot be compensated for. • The transfer is done keeping in mind principles of back care and safe handling techniques. • It should not be done if it poses a risk to the well-being of the carer. • Children should not be pulled up or picked up by holding onto the limbs. • This applies to both carrying, handling and positioning. • <u>Option 1:</u> Transfer child onto tree log and, using full hand-over-hand support, facilitate exploration of the texture and feel of the tree trunk. Talk to the child about the experience. • <u>Option 2:</u> Position 2 children in buggies adjacent or opposite each other in close proximity to the tree trunk. Each child is supported from behind by a carer providing full hand-over-hand support to facilitate exploration of the texture and feel of the tree trunk. Talk to the child about the experience. • The child who stays in the buggy can be supplied with a small branch and assisted to explore the texture and feel, giving hand-over-hand support where necessary. • The child can be pushed around the tree trunk. • A path can be built using branches on each side and the buggy can be pushed following the route.

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS	
Developmental Milestones 4–6 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Functional activities • Walks, stops and starts safely • Carry large objects • Climb stairs independently with or without holding onto a rail	ADAPT ACCORDING TO SUPPORT AND PHYSICAL ASSISTANCE REQUIRED: Mobile child (transitional): • The child can be facilitated to walk to the tree trunk and given the instruction to climb onto and sit on it. • Use should be made of short phrased commands, reinforced with gestures and hand signs or photographs as explained in the subject area of communication. • Time should be allowed so that the child can process the command. • Physical support should be rendered and decreased gradually for the child to complete the action. • The mobile child within the transitional group can be facilitated to climb over the obstacle after bringing their attention to someone else completing the action. • Use should be made of short phrased commands, reinforced with gestures and hand signs or photographs as explained in the subject area of communication. • Time should be allowed so that the child can process the command. • Physical support should be rendered and decreased gradually for the child to complete the action. • The carer can position herself behind or beside the child and use hands on support to facilitate breaking of sticks. • This action should be repeated a few times and the snapping sound emphasised and exaggerated by the carer. • Gradually the amount of support should be reduced to hands on – hands off support, where the child is allowed to attempt the movement by himself. • One on one attention of the care worker is still needed to enable the transition from engagement to supported participation.

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS	
Developmental Milestones 4–6 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPT ACCORDING TO SUPPORT AND PHYSICAL ASSISTANCE REQUIRED: Child with limitations to physical abilities: (can be in the Transitional or Interactive group) • Child is pushed in the wheelchair to and transferred onto the tree trunk. • If a child can walk short distances with the help of a walking device over uneven terrain, this should be encouraged. • If however, due to contractures and deformities, the only way a child can mobilize independently is by means of floor mobility, a child should be allowed to participate in this manner. • Supported participation to facilitate the action of climbing will apply as needed by the individual. • The child with head control, who are able to sit on the tree trunk, but needs his/her hands to help them balance, can be positioned in sitting, in front of a care worker also sitting on the tree trunk. • The amount of physical support rendered should be kept to a minimum. • Stabilize the child by holding at the level of the lower back or at both thighs, in order to free up the hands for rolling the ball. • Only after the child has attempted or partially completed the action of breaking the stick, could the child be physically assisted to complete the action. • The child can be facilitated to climb/crawl over the obstacle. • Physical support should be rendered and decreased gradually for the child to complete the action.

ACQUIRING AND DEVELOPING GROSS MOTOR SKILLS	
Developmental Milestones 4–6 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	MOBILE INTERACTIVE CHILD, MOVING TOWARDS INDEPENDENCE: • The child should be encouraged to run towards the activity. • If stairs are available on the premises, follow the route which includes stairclimbing. • Some children might still show signs of difficulty maintaining balance and will not be able to run. • They can be encouraged to walk fast. • They will also need to hold onto the railing. • No physical support should be required to maintain the sitting position on the tree trunk, but encouragement to maintain this position for increasing periods might be required in order to develop endurance. • The child can be encouraged to reach in different directions. • Limited physical support should be needed. • The child making use of alternative methods of mobility, should be allowed time and space to use his/her walking device or wheelchair, if he/she is able to or learning to negotiate uneven surfaces. • Time should be given to complete the transition to and from the wheelchair at his/her own pace, where possible and might need physical assistance due to physical disability

GROSS MOTOR SKILLS	
Content 7–14 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
- Locomotor - Walking backwards and forwards - Walk and take big and small steps - Walk and run in different directions, including stop and start on instruction - Marching, hopping, sliding, jumping and rolling - Non-locomotor, using body parts: - Twisting, bending, stretching and curling the body into various shapes - Non-locomotor, using senses: - Proprioception, touch, smell, vision, hearing, movement - Non-locomotion, using music: - Rhythm - Co-ordination - Rhythm and co-ordination using music	GENERIC ACTIVITY: Hop scotch: - Place hoops in a pattern 1-2-1. - The child hops into the first hoop landing on one leg. - The child hops into the next two hoops (next to each other) and land on both legs, apart and each leg in a separate hoop. - The child hops into the last hoop, landing on one leg. - The child jumps out of the hoop and lands on a piece of bubble wrap
	ADAPTED FOR AWARENESS GROUP: - Acquiring and developing physical skills begins with children becoming aware of themselves and their bodies, tolerating and accepting passive movement, and knowing the different positions and movements they can make. - The child can be pushed in a buggy around the hoops or facilitated to walk around or over the hoop. - He/she can be exposed to the texture, feel and noise of popping bubble wrap bubbles. - Full time hand-over-hand support is required at all times. - The aim of the activity is to learn to react to sensory stimuli and move appropriately. - Expose children to apparatus used in the activity. - Use materials and adapt resources that children can access through sight, touch, sound or smell, for example, different-sized, textured, noisy and fluorescent hoops. - Extreme care should be taken in the event that a child has epilepsy and an occupational therapist should be consulted of when and what would be appropriate. - Ensure consistent full time hand-over-hand support in order to develop interest in movement. - Improving performance begins with children being aware of what movements they can make.

GROSS MOTOR SKILLS	
Content 7–14 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
- Balance Laterality – see definition under subject area visual cognitive skills - Activities using both sides of the body and/or turn to both sides. Sports and games	ADAPTED FOR TRANSITIONAL GROUP: (hopscotch) - Acquiring and developing physical skills can help children to explore and engage in basic movement patterns and actions they can make, for example, experiencing, the skills that involve stepping from one hoop to into another. - The aim of the activity is to indicate preference of actions. - Use should be made of short phrased commands, reinforced with gestures and hand signs or photographs as explained in the subject area of communication. - Physical activities should be made more accessible by helping children to participate at first hand. - They should be physically guided to copy the carer's demonstration of stepping in and out of one hoop and then allowed time to attempt or partially complete the repeated action. - Allow children time to respond. - The same action of climbing in and out of a hoop should be expanded and reinforced by using different equipment like boxes. - Repetition of the same action in different settings (indoor/outdoor) while children are physically assisted to co-operate and engage in a variety activities. - Assist the child to explore movements and a range of different equipment. - Assist the child to repeatedly jump up and down on the piece of bubble wrap. - Exaggerate the sound made due to jumping on this specific surface. - Improving performance thus begins by showing them what they can achieve through movements and they observe the results of their actions.

GROSS MOTOR SKILLS	
Content 7–14 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP: (hopscotch) • Acquiring and developing physical skills can help the child to initiate and anticipate movements and actions. • Once mastering the abovementioned skills, a child can play through use and understanding of cause and effect. • They can for example, predict that their own action of jumping onto the bubble wrap will produce a certain effect. • Physical activities should help a child to remember the actions of jumping in and out of hoops. • The child also learns to remember his/her own responses in the specific situation and during the same motor activity. Therefor he/she will start jumping in and out of hoops (join in familiar actions) prior to a demonstration of the required motor routine. • The aim is to develop coordination and control of their bodies and increase the range of gross motor skills and movements. • Children progress by increasing the range of skills they use in activities and improving the control of their movements. • First a child should be able to jump in and out of one hoop independently or with minimal guidance. • Then they learn to combine these actions and repeatedly jump in and out of more than one hoop, at regular intervals. • Sequencing can follow by jumping one leg – two legs – one leg. • For some the goal is linking these fluently and to advance by moving spontaneously, for instance moving easily through all the hoops in hop scotch. • The task can be made more challenging, for instance instruct the child to move fast or slow. • A few will learn to change their performance and improvise in a range of different activities which involves jumping in patterns. This child could for instance choose to re-order the hoops according to his own variation or use other equipment to replace the hoops (draw circles in the sand). • With support, a very few would be able to use a bean bag as an added element to the game.

GROSS MOTOR SKILLS	
Content 7–14 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP: (hopscotch) (continued) • The support needed is to give children space and supervised freedom to do things for themselves and allowing time to respond and explore. • A range of equipment is needed. • In general, they should develop knowledge of how to use equipment safely and for a purpose. ADAPT ACCORDING TO CAPACITY: • Provide alternative activities, where necessary, to overcome difficulties with for instance manipulating tools, equipment or materials. • Push them, in their buggy, around the hoops and over a path laid out with bubble wrap. • Put a piece of the bubble wrap onto the lap tray of a buggy so that the child can hit the bubbles with his/her hand instead of jumping on it. • Because of the physical effort required, give children with physical limitations to movement time to be correctly positioned for the activity. • Ensure a balance of consistency and challenge, according to individual needs. • Adapt the activity to allow for alternative form of mobility, crawling, kneel walking, walking with assistive devices into and out of circles drawn with chalk on the surface. • Linking of arm movements and developing a movement sequence can substitute the jumping action in these children. • Use of full time, full body and hand-over-hand support to facilitate optimal involvement in the activity for children without voluntary control of movement. • These need to be done without compromising the health and wellbeing of the carer.

GROSS MOTOR SKILLS	
Content 7–14 years	Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPT ACCORDING TO CAPACITY: (continued) • Make effective use of physiotherapy, occupational therapy, speech therapy and mobility programmes, designed to meet individual needs and promote access to a range of physical activities.
Content and functional 15–18 years	Vocational Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Independence in daily living • Stability • Voluntary control of movement • Mobility • Physical fitness Recreational activities • Social activities including dancing and music Sports and games	GENERIC RECREATIONAL ACTIVITY: Musical chairs
	ADAPTED FOR AWARENESS GROUP: • Maintaining physical skills begins with children staying aware of themselves and their bodies, tolerating and accepting passive movement, and knowing the different positions and movements they can make. • Activities help maintain their ability to react to sensory stimuli and music and move appropriately. • A child can be pushed in a buggy while exposing him/her to sounds and music. • This is done on rhythm, forwards and backwards, with the carer standing in front of the child to enable eye contact and social interaction. • Musical instruments or noise making apparatus can be added and the child can, with hand-over-hand support, shake the instrument staying to the beat as a result of the carer's facilitation of the movement. • The activity can be varied by using materials and resources that children can access through sight, touch, sound or smell, for example, different-sized, textured, noisy and fluorescent balls. • Extreme care should be taken in the event that a child has epilepsy and an occupational therapist should be consulted of when and what it is appropriate or not.

GROSS MOTOR SKILLS	
Content and functional 15–18 years	Vocational Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR AWARENESS GROUP: (continue) • The aim of the activity for this group is to learn to react to sensory stimuli, being aware of what movements they can make and move appropriately. • Expose children to apparatus used in the activity. • Use materials and adapt resources that children can access through sight, touch, sound or smell, for example, different-sized, textured, noisy and fluorescent hoops.
	ADAPTED FOR TRANSITIONAL GROUP: • Consolidating and maintaining physical skills can help children to sustain the ability to explore and engage in basic movements and actions they can make in response to music or sound used in the activity variation as presented in the awareness group. • They should experience the skills that involve moving from one place to another or experience supported facilitation of a movement in response to music • Allow opportunity to indicate preference of actions, music or song with body language and sounds. • Be attuned to the child when he does indicate what is to his liking. • Activities should be made more accessible by helping children to give attention to and engage in the activity at first hand. • They should be physically guided to copy the carer's demonstration of shaking an instrument or noise making apparatus and then allowing time to attempt or partially complete the repeated action. • Allow children time to respond. • Actions could also be copied and repeated using mirrors to watch different parts of their bodies move during the musical activity

GROSS MOTOR SKILLS	
Content and functional 15–18 years	Vocational Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED AND APPROPRIATE FOR TRANSITIONAL GROUP: • Use a range of noise making apparatus, where the first steps are for children to co-operate and engage in action songs and rhymes. • Repetition of the same action during different songs or musical pieces while children are physically assisted to co-operate and engage in a variety of ways to move along to the beat and rhythm of the sounds. • Thus shared exploration of music, movements and a range of different noise making apparatus is facilitated. • Improving performance begins by showing them what they can achieve through movements and they observe the results of their actions.
	ADAPTED AND APPROPRIATE FOR INTERACTIVE GROUP: • Consolidating and maintaining physical skills can help children to get involved in basic movements and actions they can make in response to music or sound used. • Children can stand on one end of the room with a chair placed across the room. • A sound or the starting of a song can indicate that they can start to walk or move in the direction of the chair. • When the music abruptly comes to an end, the child should be expected to stop; starting again when the sound or musical piece commences again. • They should experience the skills that involve moving from one place to another in response to music. • The choice of the music should be age appropriate and fitting the taste of their peer group. • Allow opportunity for and be attuned to the child's preference of music or sounds and provide opportunities for children to be involved in choosing or making a noise making apparatus.

GROSS MOTOR SKILLS	
Content and functional 15–18 years	Vocational Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP: (continued) • In this way, turn taking is emphasised as each child gets to choose his favourite apparatus or instrument for different sessions during the week. • Activities should be made more accessible by allowing children to get involved in the activity of their preference. • They could be instructed to walk around the chair and go sit once the music stops. • Steps can be big or small. • They can walk fast or slow according to a slow paced or fast paced piece of music. • As a progression, two children could compete for the same chair. The physical skills previously acquired helps the child to initiate and anticipate movements and actions. • They need co-ordination and control of their bodies to be able to compete successfully. • Onlookers can dance to the music, combine and link them fluently. • They are encouraged to sequence and use their own spontaneous movement patterns and improvise in dancing to a range of different songs. ADAPT ACCORDING TO CAPACITY: • Adapt activity to allow for alternative form of mobility, e.g. crawling, kneel walking, walking with assistive devices etc. • Encourage use of arm movements in demonstrating big/small and fast/slow. • Linking of arm movements and developing a movement sequence can substitute dancing in these children. • Adapt activities or apparatus, where necessary, to overcome difficulties with for instance manipulating instruments. • Because of the physical effort required, give the child time to be correctly positioned for the activity. • Ensure that there is a balance of consistency and challenge, according to individual needs. • Use of full time, full body and hand-over-hand support to facilitate optimal involvement in the activity for children without voluntary control of movement.

GROSS MOTOR SKILLS	
Content and functional 15–18 years	Vocational Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPT ACCORDING TO CAPACITY: (continued) • These needs to be done without compromising the health and wellbeing of the carer. • Making effective use of physiotherapy, occupational therapy, speech therapy and mobility programmes, designed to meet individual needs and promote access to a range of physical activities. PHYSICAL INDEPENDENCE IN DAILY LIVING - Stability - Voluntary control of movement - Mobility - Physical fitness • To optimally develop and maintain physical skills needed for an optimal level of independence (in order to minimize amount and nature of support required) during ADL's, requires an individual functional assessment by professionals. • Physiotherapists and Occupational therapists analyse tasks and determine which specific physical skill is required to optimise independence. • That way children can maintain optimal physical abilities, physical health and well-being. • Through repetitively practicing a specific skill several parts during the day, this can be possible. • This practice should not to be isolated during an individualised therapeutic session. • If a person uses a wheelchair and is learning to transfer between chairs, for instance, the same technique should be used every time this functional activity is required during the course of the day. • Time should be allowed so that the optimal technique is used instead of the one that is the quickest for the carer (for example doing a passive transfer where the carer picks up the child who does not physically contributing towards the action.)

GROSS MOTOR SKILLS	
Content and functional 15–18 years	Vocational Activity and adaptations to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	PHYSICAL INDEPENDENCE IN DAILY LIVING (continued) • Last mentioned also takes away the sense of independence that the child has developed and has a direct impact on the emotional health of the child. • Knowledge and understanding of fitness and health begins with tolerating discomfort, maintaining good physical posture and correct positioning, and a willingness and desire to move and exercise. (including sports and games) • Physical activity aimed at maintaining levels of physical fitness and optimal posture can help children to become aware of the range of exercise and movement, necessary for their own health and personal wellbeing. • This will have a direct impact on for example, eating and sleeping. • The aim is to establish positive attitudes to exercise and regularly take part in physical activity. • It can facilitate an understanding of the importance of hygiene when the child wash and gets changed, washing. • Staff should be aware of, and carry out, risk assessments at regular intervals and make sure that children are engaged in appropriate activities and wear appropriate clothing. • In order to optimally address this aspect of physical health, assessments by professionals is required. • Goals should be set and form part of the ISP. • Training of carers should be done as to the specifics of the action/s required. • Professionals should take into consideration the tremendous burden of care on the carer. • The aim is the transfer of a skill which is integrated into the daily program. • In a more rehabilitative setting, time can be allocated towards individual therapeutic intervention to compliment the integrated skills practice. • Whether during group or individual intervention, the child should play an optimal part, even if only through decision making, and in this way contribute towards him/her own quality of life.

LIFE SKILLS: FINE MOTOR

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Fine Motor • Reach • Voluntary grasps: <u>Palmar grasp</u> - a normal reflex in a the newborn in which the baby's fingers spontaneously curl around any object placed within them and do not spontaneously let go; <u>Pincer grasp</u> – enables picking up of small items using the thumb and index finger. <u>Spherical grasp/gross motor grasp</u> – in which the whole hand is in contact with an object and curved with thumb opposition. (when holding a broom handle/ ice cream cone) • Releasing of object. • In-hand manipulation skills.	GENERIC ACTIVITY: Posting animals during a morning ring story. Addressing: • grasping with whole hand; • release and posting (eye hand coordination); and • receptive and expressive language. ADAPTED FOR AWARENESS GROUP: • Expose the child to the plastic animal and encourage grasping and/or reaching thereof. • Provide hand-over-hand support to enable the learner to explore the animal. • Talk to the child regarding the characteristics of the animal, e.g. “Look it is a pig. The pig is pink and fat. A pig makes “oink oink”. ADAPTED FOR TRANSITIONAL GROUP: • Talk to the child regarding the characteristics of all the animals one at a time, e.g. “Look it is a cow. The cow is black and white. A cow makes “moooooo mooooo”. • Encourage the children to copy the animal sounds. • Support the child to post the plastic animal, by means of hand-over-hand guidance. • Withdraw physical guidance and verbally support the child to post a second animal independently, if possible.

LIFE SKILLS: FINE MOTOR	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Bilateral hand-use. Stabilizing one object while the dominant hand is working or actively using both hands to manipulate objects. • Eye-hand coordination Coordinated control of eye movement with hand movement, e.g. being able to look at a hole in a tin and accurately posting a bead through the hole.	ADAPTED FOR INTERACTIVE GROUP: • The child should be able to independently identify the animals and accurately grasp, reach and post in the specific container. • 2 Dimensional animal cards may be considered for children with more sophisticated manipulation skills (thumb-index finger grasp) and basic 2 Dimensional visual-perceptual skills. • The carer can make animal cards by cutting out photos of common animals from a magazine and pasting it onto pieces of carton. • The group may help with finding the photos in the magazines
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Fine Motor • Reach	GENERIC ACTIVITY: Tearing paper Addressing reaching, fine motor grasps and bilateral-hand use.

LIFE SKILLS: FINE MOTOR	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Voluntary grasps Palmar grasp, Pincer grasp, Spherical grasp (see 0-6 years) • Releasing of object. • In-hand manipulation skills. • Bilateral hand-use. Stabilizing one object while the dominant hand is working or actively using both hands to manipulate objects. • Eye-hand coordination Coordinated control of eye movement with hand movement, e.g. being able to look at a hole in a tin and accurately posting a bead through the hole.	ADAPTED FOR AWARENESS GROUP: • Visually expose the learner to a piece of newspaper or paper from a magazine. • Use hand-over-hand support to help the child to crumple the piece of paper into a ball. • Verbalise what you are doing with the paper. • If possible, place the ball of paper in the child's hand and observe if they attempt to grasp their fingers around the ball. • Children whom are in a seating device can be supported to swipe the paper ball off their tray tables using their forearms or hands, whilst verbalising "Look we are swiping the ball off your table. The ball has fallen on the ground".
	ADAPTED FOR TRANSITIONAL GROUP: • The carer could have previously supported the group to paint a piece of newspaper. • Use the painted sheets of paper and demonstrate how to tear the paper into long strips. • Provide hand-over-hand support to enable the individual to tear a few strips. • Withdraw physical support and observe if the group copes with verbal guidance only – if not, continue further physical support. • The long strips can then be further teared into small pieces, or rolled into balls, on another day – so that the group can use it for decorating craft projects (like a Mother's day card). • The activity is thus divided into different steps for different days, as the groups' concentration is mostly limited.

LIFE SKILLS: FINE MOTOR	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP: • The carer could have previously supported the group to paint a piece of newspaper. • Use the painted sheets of paper and demonstrate how to tear the paper into long strips. • Verbally guide the children to tear their own strips of paper. • It may be necessary to repeat the tearing demonstration. • If time allows, the group can be instructed to roll the pieces of paper into tiny balls. • The carer can then provide each child with a plastic tweezer (<i>or alternatively fold a plastic straw in the middle to form a handmade tweezer</i>), and demonstrate how a tweezer is used to transfer the paper balls into a bowl/ small box that is situated in front of the learner. • Some children may find it difficult to manipulate the tweezer at first. • If hand function is severely limited, the tearing activity can be replaced by another step of the craft activity, e.g. the carer physically supports the learner to paint big sheets of paper with a sponge, which the other learners' can use for tearing purposes. • The group can use the paper balls, at a later stage, for decorating craft projects (like a Mother's day card). • The activity is thus divided into different steps for different days.

LIFE SKILLS: FINE MOTOR	
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Fine Motor • Reach • Voluntary grasps: Palmar grasp, Pincer grasp, Spherical grasp (<i>see 0-6 years</i>) • Releasing of object. • In-hand manipulation skills. • Bilateral hand-use. Stabilizing one object while the dominant hand is working or actively using both hands to manipulate objects. • Eye-hand coordination Coordinated control of eye movement with hand movement, e.g. being able to look at a hole in a tin and accurately posting a bead through the hole.	GENERIC ACTIVITY: Making a necklace. • Addressing pincer grip, • manipulation skills, • eye-hand coordination and • perceptual skills (matching, basic counting and sequencing)
	ADAPTED FOR AWARENESS GROUP: • Carer exposes the individual to the different colours and textures of the different large beads and string. • Hand-over-hand support is provided, and carer explains what she/ he is facilitating whilst positioning the objects one at a time within the individual's visual field and developing tactile awareness.
	ADAPTED FOR TRANSITIONAL GROUP: (continued necklace activity) • Carer demonstrates the threading of beads. • Initial hand-over-hand support may be needed, where after the individual might attempt to independently thread a few large beads, as concentration in this group is limited. • Following a specific pattern with regards to the threading is not possible in this group.

LIFE SKILLS: FINE MOTOR	
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Fine motor (continued)	ADAPTED FOR INTERACTIVE GROUP: • Individuals are able to independently thread a necklace, although it needs to be taken into account that the speed of threading may still not be similar to that of the carer (child may thread at a slower pace). • The carer demonstrates the threading process slowly and step-by-step, whilst the group members copy her. • Individuals who are able to follow patterns/sequences may be given a completed example of the necklace – where after they copy the pattern. • Patterns may differ according to the beads' colours, shapes and sizes.
ADDITIONAL ACTIVITIES AND INTEGRATION IDEAS FOR THE DAILY PROGRAMME	
Activities for daily living (ADL)/self-care • Have mobile above nappy changing area – motivate reaching and grasping • Use objects with a variety of textures and sizes e.g. face cloth, toothbrush, etc. for child to grip • Let child hold a spoon while being fed by carer • Grips and grasps of different utensils (adapt utensils if necessary) – create opportunities for child to reach out, grab, hold and handle utensils and objects • Assist in picking up food with hand • Provide food that can be eaten using fingers • Allow children to feed themselves • Dressing – support child with buttons/ zips/ shoelaces • Stirring activities such as sugar in tea; making jelly of instant pudding • Spreading own bread with plastic knife	

LIFE SKILLS: FINE MOTOR

ADDITIONAL ACTIVITIES AND INTEGRATION IDEAS FOR THE DAILY PROGRAMME

Activities:

- Make use of colourful, bright toys that can make a sound to get child's attention
- Make use of toys that easily fit into child's hand
- Make use of toys that create an effect when moved, cause and effect toys
- Provide safe objects for holding, pushing or squeezing
- Play stacking games using empty containers or blocks
- Provide different material for the child to be experiment on paper – finger paint, sponge paint, thick paint brush; thick crayons; thick markers
- Provide numerous opportunities for the child to experiment with different objects to practise various fine motor skills for example:
- Tearing and crumpling paper to develops the small muscles of the hand
- Picking and placing coins, cotton balls, or small items
- Play dough
- Beading, tweezers, finger puppets, action songs with fingers (twinkle little star)

LIFE SKILLS: FINE MOTOR

ADDITIONAL ACTIVITIES AND INTEGRATION IDEAS FOR THE DAILY PROGRAMME

Eye-hand coordination

- Provide a variety of materials such as large beads, shape sorters, puzzles, sidewalk chalk and finger puppets
- Do finger plays (songs and rhymes – Itsy bitsy spider)
- Sit with the child at the table or on the carpet and prompt the child to participate.
- Support the child with hand-over-hand actions where needed with a variety of activities such as:
 - Placing objects into a container
 - Picking up small balls
 - Adapted bead work- threading activities
 - Encourage finger feeding for the correct grasping of the food
 - Drawing/ painting utensils – using big arm actions on A3 size paper
 - Pegboard
 - Adapted bowling
 - Playing with building blocks
 - Filling containers with sand/ water
 - During routines activities (ADLs) enhance development by giving the child opportunities to:
 - Scoop food from container into plates
 - Wiping tables; washing and drying eating utensils
 - Remove the toothpaste cap
 - Brush teeth by himself
 - Open and close taps
 - Zipping big zips on clothing up or down
 - Packing up time – picking up toys/ learning material/ paper

LIFE SKILLS: SELF CARE

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
TOILETING - Emerging awareness regarding toilet-training - Recognizing when they need the toilet. - Formal toilet-training, including the following skills: using the toilet, undressing/ dressing, wiping and washing of hands.	GENERIC ACTIVITY: - Participating in a toileting story. (as part of the toilet routine and/or self-care topic) - Use a picture for each step for toileting. - Where applicable use sounds gestures and encourage children to copy. - Tell the story slowly and give the children opportunity to participate in the story telling. - Ask questions - Talk about toileting steps Story: Sipho wants to poo Sipho Wants to poo Go to the toilet Sipho! Sipho closes the door Sipho pulls down his pants Sipho sits on the toilet 1,,,,,2,,,,,3,,,, Sipho is done! Sipho wipes his bum Sipho pulls up his pants Flush the toilet Sipho!!! Wash your hands Sipho! Sipho can go and play!

LIFE SKILLS: SELF CARE

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR AWARENESS GROUP: • The child is still on the nappy and fully dependant on adult in this activity. • Participation is still passive or resistant. • The importance is to have a structured and set toilet/nappy routine. • Make use of the same language (appropriate to the child's culture and reference). • Picture and signs to announce when it is toileting time so that the child can be prepared on what is going to happen. • Talk to the child during toileting on what's happening and why. E.g. "Your diaper is wet, because you made a wee. I am going to change your diaper now. Look, here is the new, clean diaper. I am lifting your legs to take off your pants etc." ADAPTED FOR TRANSITIONAL GROUP: • Child is still on the nappy but urination might happen when taken to toilet on regular routine times. • Smearing and exploration of the faeces can be experienced at this stage, due to increased awareness and exploration. • The child is able to concentrate for short periods of time, so is able to listen to parts of the story – and some might be able to point to certain basic objects on the pictures regarding the story, e.g. "Thandiwe can you show me where the toilet is in the picture?" • The toileting story can be adapted according to specific groups, e.g. a story regarding nappy changing for individuals not yet using the toilet (or children in buggies who are most likely to be dependent on diapers for their entire lives). • The carer might be required to re-focus some individuals' attention back to the story, should they start losing concentration, by means of calling their names or touching them on their shoulders.

LIFE SKILLS: SELF-CARE	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP: (continued toileting) • Child is well adapted to the toileting routine. • Child is no longer on nappies, unless they have a medical condition hindering toileting, but can still occasionally soil their pants. • Child is able to identify all major objects/ equipment and actions during the toileting story, e.g. “Sipho is pulling down his pants; Sipho is wiping his bum; Sipho uses the toilet paper to wipe his bum”. • Matching of the toileting story pictures can be expected of some learner’s, e.g. matching 2 pictures of Sipho washing his hands.
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
TOILETING • Child will no longer use a potty but the toilet. • Ensure privacy and dignity.	GENERIC ACTIVITY: Participating in a toilet-training social story. • If potty training has not occurred when the child was younger, start with toilet training if the individual has a sense of toileting awareness. • It is important to remember the child’s age and to incorporate privacy during toileting to ensure their dignity. (See guidelines on toilet training in manual).
	ADAPTED FOR AWARENESS GROUP: • Same activity and principles applies as above for 0-6 years. • Important to stick to a routine. • Some individuals in the awareness and transitional groups may however never reach independence with regards to toileting.

LIFE SKILLS: SELF-CARE	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR TRANSITIONAL GROUP: (continued toileting) • Same activity and principles applies as above for 0-6years. • Important to stick to a routine. • Some individuals in the awareness and transitional groups may however never reach independence with regards to toileting
	ADAPTED FOR INTERACTIVE GROUP Same activities and principles apply as above for 0 – 6yearr.
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
TOILETING • Child will no longer use a potty but the toilet • Ensure privacy and dignity	GENERIC ACTIVITY: Cleaning of the bathroom environment.
	ADAPTED FOR AWARENESS GROUP Same activity and principles apply as for previous sections 0–6 years and 7–14years.
	ADAPTED FOR TRANSITIONAL GROUP: • Initial hand-over-hand support is provided to apply soap on a cloth/sponge and then facilitating the scrubbing action of the basin and taps. • Physical support can be decreased and verbal guidance introduced as the adolescent becomes familiar with the action. • Carer explains in simple terms as to why it is necessary to keep the area clean.

LIFE SKILLS: SELF-CARE	
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP (continued bathroom activity) • Carer applies soap on adolescent's sponge/cloth and demonstrates scrubbing the basin and taps. • Verbal guidance may be provided whilst the individual attempts to scrub the area independently. • Carer explains in simple terms as to why it is necessary to keep the area clean. • Verbal guidance is provided to enable the learner to successfully rinse the area, without wasting water. • Some individuals may be able to independently follow a visual schedule existing of photos in the correct steps of the washing task.
LIFE SKILLS: SELF-CARE	
• The following self-care skills have been added, but does not include the 3 different age groups or adapted activities for the 3 functioning groups. It can be adapted according to the examples given in Toileting as seen above. • As children become independent with self-care skills, focus can be put more on hygiene and home/working skills. • Self-care skills should be incorporated in other topics such as Me, My body, staying safe, etc. (see Topics as discussed in section 2).	
Content:	ACTIVITIES AND INTEGRATION IDEAS FOR THE LP
FEEDING • Fully depend on the caregiver, no voluntary oral movement in feeding • Participates in the feeding activity	• Awareness of feeding routine (tactile, sensory stimulation) - textures of food • Sing a song to make everyone alert of feeding time • Create a calm optimal environment for feeding • Social interaction with the child during feeding • Talk about what he/she is eating. • Encourage social interaction between the children

LIFE SKILLS: SELF-CARE	
CONTENT	ACTIVITIES AND INTEGRATION IDEAS FOR THE LP
FEEDING (continued) - Relies on the caregiver to spoon feed, feeding without aspirating - Finger feeds on his/her own - Uses utensil in feeding with assistance (spooning of food) - Uses utensils independently in feeding. - Stabilizes the plate and uses utensil (spoon, fork) - Drinks with maximal assistance from cup - Drinks from cup with partial /initial help - Drinks independently from cup with little spillage - Drinks thicker consistency with straw - Drinks independently	- Hand-over-hand support if needed, but retract to encourage self-feed. - Give the child opportunities to grip and grasp different utensils (adapt utensils if necessary) - Encourage finger feeding - Encourage self-feeding with spoon and later fork - Encourage cutting food with knife - Encourage eating food with knife and food INTEGRATION WITH OTHER TOPICS/SUBJECT COMPONENTS - Fantasy play to model behaviour during feeding time - Encourage table manners - Laying table – (matching, sorting, counting) - Cleaning up after eating (home skills, correct steps for washing up, matching, sorting, counting) - Food – daily meals, vegetables, fruit, healthy food

LIFE SKILLS: SELF-CARE	
CONTENT	ACTIVITIES AND INTEGRATION IDEAS FOR THE LP
DRESSING - No active participation in activity Undressing Participates by means of movement of limbs to aid undressing Tries to undress self, but needs lots of support - Takes off hat - Takes off socks - Takes off shoes (with shoelaces) - Takes off shoes (without shoelaces) - Takes off jacket (after it's undone) - Pulls t-shirt over his/her head and takes it off Undresses self with some support - Pulls down unfastened pants - Takes off unfastened pants Undresses independently - Unfastens the pants and takes off pants Completely takes off underwear	- Awareness of being undressed (tactile stimulation touch, passive stretches, talk through the process) - Awareness of being undressed and participation in undressing – motor through activities. - Hand-over-hand support as needed - Modelling and imitation - Visual cues for undressing and dressing sequence - Use clothes that are loose and easy to take off - Practice with Velcro shoes, big zips on loose clothes, big buttons with vertical buttonholes. Child should practice while wearing the clothes INTEGRATION WITH OTHER TOPICS/SUBJECT COMPONENTS - Integrate with action song, story and discussions. - Other topics: My body; seasons

LIFE SKILLS: SELF-CARE	
CONTENT	ACTIVITIES AND INTEGRATION IDEAS FOR THE LP
-DRESSING (continued) No active participation in activity Participates by means of movement of limbs to aid dressing Tries to dress self, but needs a lot of support • Puts on hat • Puts t-shirt over head Dresses self with some support • Puts t-shirt overhead and putting arms in sleeves of t-shirt • Puts socks on • Puts on jacket • Pulls up pants • Puts on pants Dresses self with limited support • Fastens pants (zip or button) • Pulls zippers up and down (once started) • Zips up (including starting) • Does buttons Dresses self independently	

LIFE SKILLS: SELF-CARE	
CONTENT	ACTIVITIES AND INTEGRATION IDEAS FOR THE LP
Washing hands • Carer wipes child's hands with a cloth • Child assists with wiping own hands with cloth • Child wipes own hands with cloth • Washing hands in basin with assistance • Turns on the tap • Picks up soap/uses liquid soap • Rubs soap between both hands • Rubs hands together to form a lather • Rinses off all the soap • Turns off the taps • Dries hands with towel	• Hand-over-hand support as needed • Visual cues/sequence • Daily opportunity to practice; use backward chaining initially, change and increase as skill improves • Reward and praise Integrate with action song, story and discussions.

LIFE SKILLS: SELF-CARE	
CONTENT	ACTIVITIES AND INTEGRATION IDEAS FOR THE LP
BRUSHING TEETH • Carer brushes child's teeth • Child opens mouth • Child assists with brushing teeth • Opens the toothpaste • Puts one squeeze of toothpaste on the toothbrush • Closes toothpaste • Holds toothbrush in dominant hand • Places toothbrush in mouth • Brushes inside surfaces of teeth • Brushes outside surfaces of teeth • Brushes biting surfaces of teeth • Spits toothpaste into basin • Turns on the tap • Rinses mouth with water • Rinses toothbrush with water • Turns off the tap Dries mouth with towel	• Regular opportunity to practice • Hand-over-hand support as needed • Make use of backward chaining initially • Increase and change as he/she improves • Visual and verbal cues/sequence • Reward and praise Integrate with action song, story and discussions.

LIFE SKILLS: SELF-CARE

CONTENT	ACTIVITIES AND INTEGRATION IDEAS FOR THE LP
BLOWING NOSE • Takes tissue/cloth from the box • Lifts arm to wipe nose/ mouth • Puts the tissue over nose • Blows air through nose • Pinches both nostrils closed with tissue • Wipes area around nose • Folds the tissue • Puts tissue over nose • Blows air through nose • Pinches both nostrils closed with tissue • Wipes area around nose • Folds the tissue • Makes a final check in the mirror Throws the tissues away	• Hand-over-hand support as needed • Verbal and visual prompts • Praise • Mirror • Make child aware of his/her appearance by looking at the mirror when his/her nose is runny/after he/she sneezed. • Keep the tissues within reach near the mirror. • While looking in the mirror let child wipe his/her nose. • He/she might need hand-over-hand assistance at first. • Give him/her a new tissue and get him/her to blow his/her nose – it is helpful to have him/her imitate the action. • Prompt action if needed and praise his efforts.

LIFE SKILLS: VOCATIONAL SKILLS

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Prevocational skills Important skills that are needed to function in protective workshops, that is developed during 0-6 are: • Visual cognitive skills (basic concepts, matching, sorting, spatial skills, patterns and sequence) • Fine motor (grasps, reaching, release, object manipulation and bilateral hand-use) • Language and communication • Social skills (greeting, socializing, turn-taking etc.) • Self-care (personal hygiene and toileting) <i>Please refer to the specific content under visual cognitive skills for further detail and discussion on abovementioned skills.</i>	GENERIC ACTIVITY: • The foundation for these skills is established during 0–14 years. • Early development of domestic skills will be for example teaching a child to take his dish to the sink or to wipe his table clean after lunch etc. • Thus working on normal self-care tasks and classroom routines while the child is still young, will lead to improved readiness for domestic and vocational skills as an adolescent.

LIFE SKILLS: VOCATIONAL SKILLS	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Prevocational skills Important skills that are needed to function in protective workshops, that is developed during 0-6 are: • Visual cognitive skills (basic concepts, matching, sorting, spatial skills, patterns and sequence) • Fine motor (grasps, reaching, release, object manipulation and bilateral hand-use) • Language and communication • Social skills (greeting, socializing, turn-taking etc.) • Self-care (personal hygiene and toileting) <i>Please refer to the specific content under visual cognitive skills for further detail and discussion on abovementioned skills.</i>	• The foundation for these skills is established during 0–14 years. • Early development of domestic skills will be for example teaching a child to take his dish to the sink or to wipe his table clean after lunch etc. • Thus working on normal self-care tasks and classroom routines while the child is still young, will lead to improved readiness for domestic and vocational skills as an adolescent.

Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Domestic: refers to tasks that are performed in and around the house (household activities). • Cleaning • Meal preparation • Gardening • Laundry	GENERIC ACTIVITY: Laundry • matching, sorting and folding socks • Addressing tactile, smell, vision, visual figure ground, matching and sorting.
	ADAPTED FOR AWARENESS GROUP: • Visually expose the adolescent to the socks, talk about the colour of the socks and support them to feel and smell the clean socks. • Participation is fully supported.
	ADAPTED FOR TRANSITIONAL GROUP: • Provide the group with a basket filled with a few items of laundry. • Demonstrate to the group how to search through the laundry and take out a sock. • The carer then physically and/ or verbally supports the adolescent to search for more socks in the basket in order to separate the socks from the rest of the laundry. • As concentration in this group is limited, the individual may only be expected to search for a few socks, and not all of them. • Provide the group with a basket filled with a few items of laundry. • Demonstrate to the group how to search through the laundry and take out a sock. • The carer then verbally instructs the adolescent to search for all of the socks in the laundry, after which they must place all the matching socks in their separate pairs. • If time allows, the carer can demonstrate and verbally guide the group through folding of the socks.

LIFE SKILLS: VOCATIONAL SKILLS	
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
Preparation for protective workshop/ sheltered employment: refers to employment provided under special conditions because of severity of the individuals' disability and are unable to perform a work under ordinary/ public working conditions. Examples of protective workshop activities include: • Match • Sort • Assemble • Package	SPECIFIC ACTIVITY: • Sorting different coloured soda caps. • Addressing attention, matching and sorting.
	INTERACTIVE GROUP: • <i>(Individuals in this age group will have to be on an Interactive level and have the necessary motor skills to be prepared for sheltered employment, although some tasks can be adapted to accommodate less severe physical impairment.</i> • <i>An Occupational therapist should be consulted in this regard.</i> • <i>The awareness and transitional groups will be prepared for domestic tasks.)</i> • Use 4 boxes or buckets and stick one of each 4 different coloured soda caps on an each of the boxes/ buckets. • Place all the caps that need sorting in a container. • Demonstrate how to place the different coloured caps into each specific container. • Verbally guide the learner through the rest of the sorting task.

LIFE SKILLS: SENSE OF SELF AND SEXUALITY

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
- Develop awareness of themselves and their bodies refer to visual cognitive skills - Learn about the need for personal hygiene - Recognise and communicate their likes and dislikes.	GENERIC ACTIVITY: - Development of the body is a natural process. - When exploring and learning about his/her body as he/she is growing up, the child would also become aware of his/her private body parts and their functions, talk about this during toileting and washing activities. (Carers/parents/adults are responsible for protecting and safekeeping of children, as well as seeing that children are treated with respect and dignity at all times – privacy during toileting and nappy changing)
	ADAPTED FOR AWARENESS GROUP: - Self-care routines provide an optimal opportunity for the carer to enhance the child's body awareness through continually verbalising the child's part, functions and actions. - The carer use the correct names for body parts during washing and nappy changing."
	ADAPTED FOR TRANSITIONAL GROUP: - Children in this group have emerging body awareness. - Most of the children will not be able to independently identify major body parts. - Explore and make use of hand-over-hand support when using toilet paper or washing of private parts.
	ADAPTED FOR INTERACTIVE GROUP: - Most children in this group will be able to identify major body parts (e.g. head, stomach, arms, and legs), and some might even be able to identify more complex parts (e.g. thumb, knees and shoulders). - The carer might still need to show where the body parts are, and then allow the child the opportunity to copy her whilst she names the body part. - Sort clothes according to the body part they cover e.g. panties, etc. – refer to body awareness and topics of me and my body. (boy/girl; safety/privacy discussion – see annexure 7: Topic: My body)

DEVELOPMENTAL AREA: LIFE SKILLS: SENSE OF SELF AND SEXUALITY	
Content and functional 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
- Recognise that they are growing and changing - Take part in and maintain personal hygiene routines - Develop body and gender awareness - Encouraged to develop a sense of right and wrong and to learn basic rules for keeping safe. - Emerging sense of modesty - Identify private places	GENERIC ACTIVITY: - Development of the body is a natural process. - When exploring and learning about his/her body as he/she is growing up, the child would also become aware of his/her private body parts and their functions, talk about this during toileting and washing activities. - Talk about privacy when dressing/toileting/washing. - Make aware of boys/girls toilets (pictures) during discussions and toilet routine. (Carers/parents/adults are responsible for protecting and safekeeping of children, as well as seeing that children are treated with respect and dignity at all times – privacy during toileting and nappy changing).
	ADAPTED FOR AWARENESS GROUP: The carer use the correct names for body parts during washing and nappy changing – e.g. “let’s wipe your bum”.
	ADAPTED FOR TRANSITIONAL GROUP: - Explore and make use of hand-over-hand support when wiping of washing of private parts, while using the correct body parts. <i>(The carer must have knowledge in the correct handling of normal body functions such as erections and masturbation- additional training and policy of centre/school).</i>
	ADAPTED FOR INTERACTIVE GROUP - Most children in this group will be able to identify major body parts (e.g. head, stomach, arms, and legs), and some might even be able to identify more complex parts (e.g. thumb, knees and shoulders). - The carer might still need to show where her body parts are, and then allow the child the opportunity to copy her whilst she names the body part. - Sorting of clothes according to the body part they cover (panties/ bra – difference between boys and girls clothes). (See Annexure 7: Topics: My body)

LIFE SKILLS: SENSE OF SELF AND SEXUALITY

Content and functional 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Learn about the nature of friendships and relationships, including sexual relationships • Recognise the risks in some situations, making safe choices – know when they can and should give their permission and when to withhold their permission (To communicate 'no') • Communicate the need for help. • Recognise violation of privacy rights • Respect privacy of others	GENERIC ACTIVITY: Talking about the different roles of family and non-family members (special circle – see manual). Discussion about appropriate means showing affection/ different means of touch (who do we hug/ who do we greet by hand). Making use of photographs and pictures.
	ADAPTED FOR AWARENESS GROUP: • Making aware of other people in his/her immediate environment by means of taking child's face in your hands and turning his/her face towards the person
	ADAPTED FOR TRANSITIONAL GROUP: • Use photo of child and with hand-over-hand support place it with different photos of other people they know, don't know and know a little. Describe the relationship to the child.
	ADAPTED FOR INTERACTIVE GROUP: • Use photos of children and others and ask them to place photos and describe relationship; actions to be taken when feeling vulnerable.

LIFE SKILLS: SOCIAL SKILLS AND RELATIONSHIPS

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
RELATING TO FAMILIAR PEOPLE - Looks at face of parent or caregiver. - Watches (that is, follows with eyes) someone moving by crib or bed for 5 seconds or more. - Smiles or makes sounds when approached by a familiar person. - Makes or tries to make social contact (e.g. Smiles; make noises; etc.). - Reaches for a familiar person when the person holds his / her arms out to him or her. - Shows preference for certain people and objects (e.g. Smiles; reaches for or moves toward person or object; etc.). Shows affection to familiar persons (e.g.	Care should be aware of own facial expressions and try to show positive feelings towards child. GENERIC ACTIVITY: Recognising familiar people. (see morning ring – greeting/ greeting song and Topics: Me - self-identification)
	ADAPTED FOR AWARENESS GROUP: - Carer sings a song, takes the face of child in her hand and makes eye contact during the morning ring. - Carer verbalises the individual's name, and positions a photograph within the individual's visual field – in attempt to gain a momentarily gaze from the learner in the direction of the photo.
	ADAPTED FOR TRANSITIONAL GROUP: - As for the Awareness group. - Increased use of photographs during greeting/ greeting song in morning ring, of all the children as well as caregivers. - Carer sings song and points to the child and everybody identifies the specific child. - The carer might still have to verbalise the individual's name before identification takes place. - Carer should closely focus on the individuals' non-verbal communication signs for possible reactions.
	ADAPTED FOR INTERACTIVE GROUP: - Photographs of all carers and children are used. - Children recognise certain carers and associate them with specific events, e.g. helpers for feeding, toileting, drivers and centre manager. - Matching photo to carer.

LIFE SKILLS: SOCIAL SKILLS AND RELATIONSHIPS

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(Continued) (e.g. Touches; hugs; kisses; cuddles; etc.). • Imitates or tries to imitate parent's or caregiver's facial expressions (e.g. Smiles; frowns; etc.). • Moves about and looks for parent or caregiver or other familiar person nearby. • Uses actions to show happiness or concern for others (e.g. Hugs; pats arm; holds hands; etc.). • Displays stranger anxiety. • Separates more easily from the / mother caregiver RELATING TO FRIENDS/PEERS • Shows interest in children the same age, other than brothers or sisters (e.g. Watches them; smiles at them; etc.).	

LIFE SKILLS: SOCIAL SKILLS AND RELATIONSHIPS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(Relating to friends/peers Continued) Has best friend or shows preference for certain friends (of either sex) over others.	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
RELATIONSHIPS - Develop and experience a range of relationships - Recognise and understand different types of relationships	GENERIC ACTIVITY: - Talking about the different roles of family and non-family/ community members. - Making use of photographs and pictures. (See Topics – Me – my family, etc.)
	ADAPTED FOR AWARENESS GROUP Making aware of other people in his/her immediate environment by means of taking child's face in your hands and turning his/her face towards the person
	ADAPTED FOR TRANSITIONAL GROUP: - Place photo of child's parent/guardian amongst 2 random photos of unknown people and with hand-over-hand support enable child to select correct photo, discuss basic personal details of parent/guardian (name). - Place photo back amongst other 2 photos. - Request photo "where is mommy?", verbally directing child to select correct photo. "Look that is mommy!"
	ADAPTED FOR INTERACTIVE GROUP: Use photos of children and other known and unknown (i.e. community workers) and ask them to match sets of photos and describe relationship (i.e. doctor/nurse gives medicine; taxi driver takes us home/to shop).

LIFE SKILLS: SOCIAL SKILLS AND RELATIONSHIPS	
Content and functional 15– 8 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
RELATIONSHIPS - Learn about the nature of friendships and relationships, including sexual relationships. - Recognise the risks in some situations and making safe choices. - Deal with changing relationships	GENERIC ACTIVITY: - Talking about the different roles of family and non-family members. - Discussion about appropriate means showing affection/ different means of touch (who do we hug/ who do we greet by hand). - Making use of photographs and pictures.(see Topics; Me – relationships/ sexuality)
	ADAPTED FOR AWARENESS GROUP: Making aware of other people in his/her immediate environment by means of taking child's face in your hands and turning his/her face towards the person
	ADAPTED FOR TRANSITIONAL GROUP: - Place photo of child's friend/ fellow group member amongst 2 random photos of unknown people, and with hand-over-hand support enable child to select correct photo, - discuss basic personal details of friend (name). - Place photo back amongst other 2 photos. - Request photo "where is (name friend)?" verbally direct child to select correct photo. "Look that is!"
	ADAPTED FOR INTERACTIVE GROUP: - Use photos/pictures showing groups of people. - Matching pictures to visual signs (hospital, church, shop, house, school) that show different categories of friends, family, community members and strangers.

LIFE SKILLS: EMOTIONS

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
- Shows two or more emotions (e.g. laughs; cries; screams; etc.). - Is comforted by touch and people's faces and voices. - Seeks physical and emotional comfort by snuggling into trusted adults. - Shows joy, distress (frustration or pain), surprise, interest - Calms from being upset when held, spoken or sung to in soothing voice. - Shows specific emotional attachment to mother. - Protests separation from mother. - Shows anger, affection. - Expresses curiosity and exploration. - Displays dependent behaviour. - Growing ability to sooth self and may be dependent on a comfort object	GENERIC ACTIVITY: - Managing emotions. - Observe and examine faces by looking into mirrors, looking at pictures and making happy, sad and angry faces. - This can be reinforced by using a story about emotions.
	ADAPTED FOR AWARENESS GROUP: - Hold mirror in front of child's face and facilitate eye contact in mirror. - It is important that the carer is constantly aware of the child's emotional state, and acknowledge this to the child e.g. "I see you are feeling sad today. - What happened? Are you hungry/ feeling pain?"
	ADAPTED FOR TRANSITIONAL GROUP: - Hold mirror in front of child's face and name the emotion that child displays (happy or sad). - It is important that the carer is constantly aware of the child's emotional state, and acknowledge this to the child. - Verbalise emotions as expressed by child.
	ADAPTED FOR INTERACTIVE GROUP - Carer demonstrates facial expressions (happy, sad or angry). - Position mirror in front of child and encourage child to imitate and possibly name/ identify emotions. - Guide the child to name emotions he/ she is experiencing during the day (mainly focusing on happy, sad and angry as children with SPID find it difficult to recognise more abstract emotions).

LIFE SKILLS:EMOTIONS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued) • Cooperates in caregiving activities, such as dressing • Begins to understand “yes” and “no” boundaries Aware of other’s feelings, might look concerned at hearing crying/ excited at happy voice • Growing sense of will - throws tantrums. • Expresses violent emotions, e.g. anger. • Shows an array of emotional expressions. • Displays a sense of humour. • Responds to a few appropriate boundaries with encouragement and support	

LIFE SKILLS: EMOTIONS

Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued) • Begins to understand that some things are his/hers, others are shared and some belong to others • Can stop self from doing something he/she shouldn't do • Aware of own feelings, and know that some actions can hurt other's feelings. • Begins to accept the needs of others and can take turns and share resources, often with support from others. • Begins to tolerate delay when needs are not immediately met, and may understand wishes may not always be met. • Begins to adapt behaviour to different events, social situations and change in routine	

LIFE SKILLS: EMOTIONS	
Content 0–6 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued) • Begins to understand that own actions affect other people, for example, becomes upset or tries to comfort the person he/she upset. • Begins to be aware of boundaries set, and of behavioural expectations in settings.	
LIFE SKILLS: EMOTIONS	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
• Recognise that they are growing and changing. • Develop a positive self-image • Explore, express and communicate their needs, feelings and opinions	GENERIC ACTIVITY: • Managing emotions. • Observe and examine faces by looking into mirrors, looking at pictures and making happy, sad and angry faces. • This can be reinforced by using a story about emotions.
	ADAPTED FOR AWARENESS GROUP: • Hold mirror in front of child's face and facilitate eye contact in mirror. • It is important that the carer is constantly aware of the child's emotional state, and acknowledge this to the child e.g. "I see you are feeling sad today. • What happened? Are you hungry/ feeling pain?"

LIFE SKILLS: EMOTIONS	
Content 7–14 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
(continued) Take responsibility for themselves and their belongings.	ADAPTED FOR TRANSITIONAL GROUP: - Hold mirror in front of child's face and name the emotion that child displays (happy or sad). - It is important that the carer is constantly aware of the child's emotional state, and acknowledge this to the child. - Verbalise emotions as expressed by child.
	ADAPTED FOR INTERACTIVE GROUP: - Carer demonstrates facial expressions (happy, sad or angry). - Position mirror in front of child and encourage child to imitate and possibly name/identify emotions. - Guide the child to name emotions he/she is experiencing during the day (mainly focusing on happy, sad and angry as children with SPID find it difficult to recognise more abstract emotions).
Content 15–18 years	Vocational Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
- Be aware of their personal qualities, skills and achievements. - Make choices - Realise that all individuals are important in their own right. - Recognise differences and similarities in people.	GENERIC ACTIVITY: - Managing emotions. - Observe and examine faces by looking into mirrors, looking at pictures and making happy, sad and angry faces. - Telling of social stories and scenarios that might occur in home/sheltered workplace.
	ADAPTED FOR AWARENESS GROUP: - Managing emotions. - Observe and examine faces by looking into mirrors, looking at pictures and making happy, sad and angry faces. - This can be reinforced by using a story about emotions.
	ADAPTED FOR TRANSITIONAL GROUP: - Hold mirror in front of child's face and name the emotion that child displays (happy or sad). - It is important that the carer is constantly aware of the child's emotional state, and acknowledge this to the child. - Verbalise emotions as expressed by child.

LIFE SKILLS: EMOTIONS	
Content 15-18 years	Activity to enhance and support development (See Manual for integrated programmes within the Daily Programme)
	ADAPTED FOR INTERACTIVE GROUP • Carer asks children to identify happy, sad or angry faces in magazines. • Guide the child to name emotions he/ she is experiencing during the day (mainly focusing on happy, sad and angry as children with SPID find it difficult to recognise more abstract emotions). • Emphasis should be placed on the different emotions that can be experienced in the workplace or home environment (e.g. discussions about appropriate handling of conflict situations).

SECTION 4

ASSESSMENT

WHAT IS MEANT BY ASSESSMENT?

Assessment is a continuous, ongoing and planned process of identifying, gathering and interpreting information to determine or inform whether development and learning took place.

WHAT IS THE PURPOSE OF ASSESSMENT?

When working with children with SPID assessment will be used to determine:

- What the level of functioning or development is of the various skills (communication, gross and fine motor skills, social and emotional, self-care and cognitive (thinking, understanding and learning) - this is needed to form the baseline from which to plan activities/ programmes/ interventions;
- How to group children to ensure more effective support for development of skills, learning and applying of knowledge;
- At what level to start the planning and developing of programmes to ensure development of skills and learning can take place;
- Each child's strengths, weaknesses and interests to ensure that each child's needs can be met through the development of an individual support plan (ISP);
- Whether what was planned and implemented is correct or needs to be adapted and whether more or less support and adaptations are needed;
- Whether progress (development of skills, learning and applying of knowledge) is taking place in a group and/or individually; and
- Monitor that continuous development of skills and learning are taking place

METHODS AND APPROACHES OF ASSESSMENT

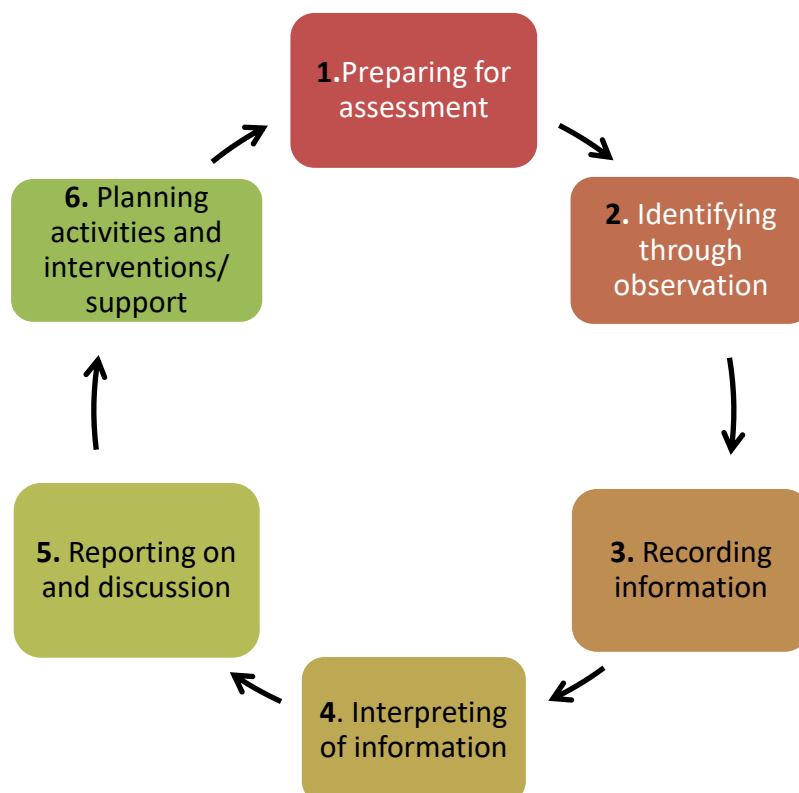
The LP for children with SPID consists of guidelines to assist in:

(Adapted from the NCF 0-4, 2015):

- **Observing** the developmental and learning needs of each child;
- **Planning** activities to meet each child's own developmental needs, strengths and learning interests;
- **Doing** the activities with the children (adapting according to needs);
- **Assessing** children's developmental and learning needs; and
- **Evaluating** the LP in terms of its ability to meet the needs of each child.

THE ASSESSMENT PROCESS

There are six steps in the Assessment Process:



WHAT, WHEN, WHERE AND HOW TO ASSESS

Preparing for assessment

Assessment should be informal and children should not be subjected to a 'test' situation. Assessment should be done in the child's natural environment during structured everyday routines and activities of the daily programme.

The best way to assess would be through ongoing observation. This means taking the time to observe and note down information on each child. A good way to do this would be with the use of an observation "lap book" that should be readily available. The information gathered can then be used to make an assessment of the child or situation as the year progresses, as a full picture of each child complete with challenges and strengths is gradually build. Anything interesting or of concern should be noted in the observation book, and followed up each day. These notes should also include planning of development and learning, and can also include future assessments

Each activity used for assessment should be carefully planned so that it integrates a variety of skills. This allows for challenges to be addressed and strengths to be maximised through planning and implementation of needed support. For instance, Life Skills gives children an opportunity to explore/ experience their world and begin to understand it. The purpose of assessment of Life Skills is to assess the development of concepts, skills and values.

Identifying through observation: generating and collecting evidence of achievement

The purpose of assessment is to support and encourage the child, and to assess the child's holistic development. We know that a child has developed concepts and skills by observing the child's participation and engagement in activities related to those concepts. Assessment should be based on the components in the Learning Programme: Communication and Language, Life Skills and Visual Perceptual and Cognitive skills.

Recording the information

- a) **Observations** should be written and recorded to be used to plan effectively for each child's needs and interests. This would also assist when discussing and planning with parents and other role players. Discussions and planning should always be based on evidence.
- b) **Resources for assessment:** Observation note books and checklists.

Baseline assessment:

A baseline assessment must be done for each child to determine his/her level of functioning by:

- Collecting background information from files, parents, early intervention providers, health professionals who have assessed the child; and
- Conducting-picture-based screening (David Werner (1987). Disabled village Children: A guide for community health workers and families – see Annexure 5)

Interpreting information

The gathered information should be evaluated. The parent should be involved in making decisions about the child's development needs and learning interests. The LP should be used to assist in the planning for development and learning. Through the interpreting and discussion of the information the need for further referrals to other professionals could also be determined.

Reporting on and discussing the findings

Reporting is the process of communication the child's development and learning status and needs to parents, schools, therapists (other role players).

Individual support planning

The individual support plan (ISP) provided must be in line with the SIAS Policy and include the needs of children with SPID. The planning process will involve the following:

- Conducting a needs analysis by reviewing and analysing the strengths and needs of the child;
- Interpreting the results of the baseline assessment;
- Determine the level of functioning;
- Planning the focus of interventions, including medical and therapeutic interventions and provision of assistive technology;
- Structuring of individual or group activity programmes;
- Assigning responsibilities to members of the transversal team; and
- Setting target dates for review and consultation with stakeholders.

HOW TO RECOGNISE LEARNING

For the child with SPID, learning may not necessarily occur along the same pathways as for other children, but it does occur nevertheless. Learning is a process of accumulating experiences. As with all children, to be able to develop skills learn and apply knowledge, they need to be given opportunities to experience and to be made part of activities in their natural environment. How to recognise whether learning has taken place is important to ensure that the child is receiving the best

support, or whether changes are needed to support any further development of skills and learning. As has been outlined in the LP (section 3), breaking up the learning process into smaller steps, supports learning. The same can be said for recognising learning.

By looking at different ways of behaviour and accomplishment, progression can be more specifically recorded. By observing the child in his natural environment on a daily basis his behaviours and responses will give a clear indication of progress individually, in a small or bigger group. More than one area of development as part of an integrated approach can also be observed.

(See Section 3: Design of the LP: Grouping and Checklist for progressive stages – Dorchester Curriculum Group (UK).

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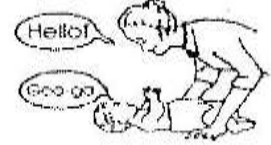
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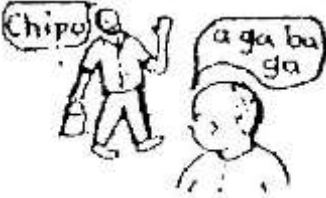
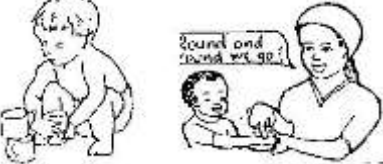
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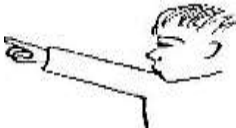
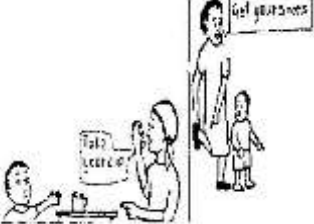
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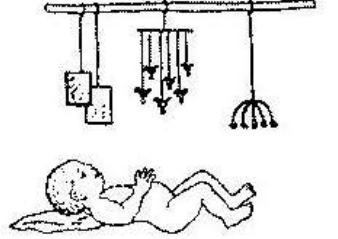
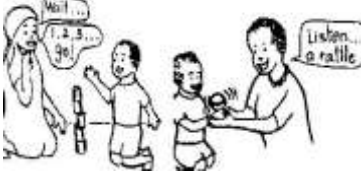
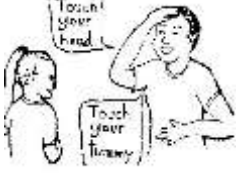
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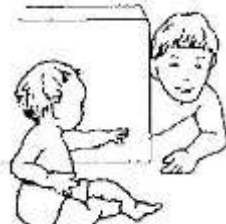
PICTURE-BASED OVERVIEW OF COMMUNICATION AND LANGUAGE

Unresponsive	Increased responsiveness	Listens and begin to understand/ respond	Communicates through words and gestures
Child is: - Unresponsive  - Cries 	Increased responsiveness Responds to environment (sound, visual stimuli, touch, taste and smell, movement): Vision - Follows objects/ people with eyes  - Makes eye contact  - Recognizes familiar people  Sound - Looks towards sound	Child: - Listens and begin to understand/ responds to simple commands  - Responds to yes/no  - Child likes games, social interaction and making things happen	Child: - Communicates through words and gestures  0 – 12 months  - Communicates through phrases and gestures  - Communicates through sentences (simple to complex)

Unresponsive	Increased responsiveness	Listens and begin to understand/ respond	Communicates through words and gestures
	 - Start to respond to name:  Touch: - Reach out towards people and objects  - Examines objects / takes object to mouth  Speech - Makes simple sounds	  - Imitates and take turns  - Repeats sounds and gestures 	 12 months – 3 years 

Unresponsive	Increased responsiveness	Listens and begin to understand/ respond	Communicates through words and gestures
	 1 – 2 months Begins to babble  4 – 8 Months	Babbles/ uses jargon/ uses some simple single words/ uses sound and gesture to express emotion  	
SKILLS TO DEVELOP			
Awareness of vision, sound, touch movement and smell - Make eye contact and smile – help child focus  - Sing and make sounds 	- Awareness of cause and effect - Recognition of sound - Focus Help child to: - Look at objects and talk about them  	Develop comprehension of words and symbols: - Attention to words  - Ask child to fetch things / to do things / find things 	Production of words that is understood Awareness that language is way of acquiring needs/wants Activities where child can produce words 

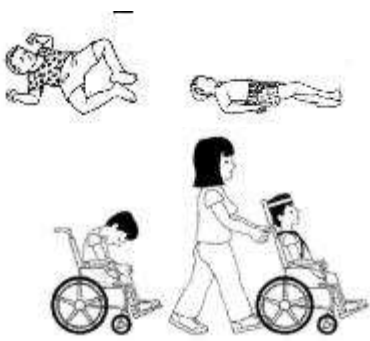
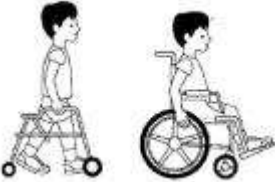
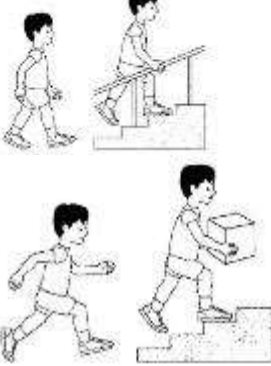
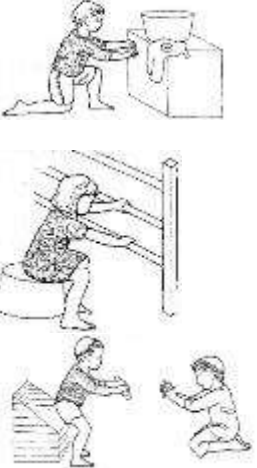
Unresponsive	Increased responsiveness	Listens and begin to understand/ respond	Communicates through words and gestures
 - Say child's name, smile  - Mobile with objects hanging 	- Listen and look towards sound/ make sound happen  - Finger rhymes/ songs  - Make things happen (cause-and-effect)  - Look for objects – half hidden (object permanence)	 Encourage imitation and turn taking    Songs and rhymes 	- Which one is missing  - Tell stories and ask questions 

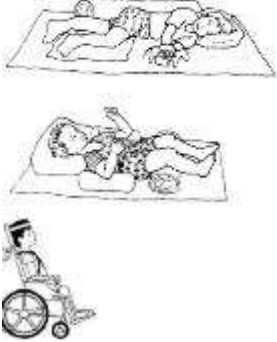
Unresponsive	Increased responsiveness	Listens and begin to understand/ respond	Communicates through words and gestures
	 - hidden (object permanence)  - Peek-a-boo  • Repeat sounds and gestures  • Talk about what you are doing/ things you see Name objects/ actions	• Stimulation of play with objects (functional play)  • Name objects and actions /talk about them  • Name parts of body and ask child to touch them  • Give choices between objects, by naming them / let child point to object of choice 	

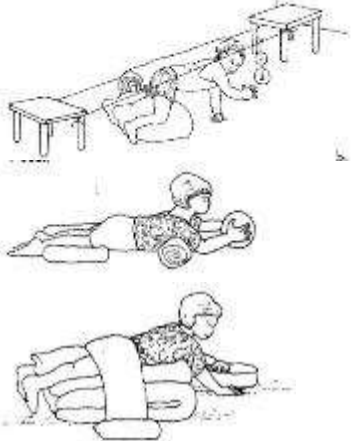
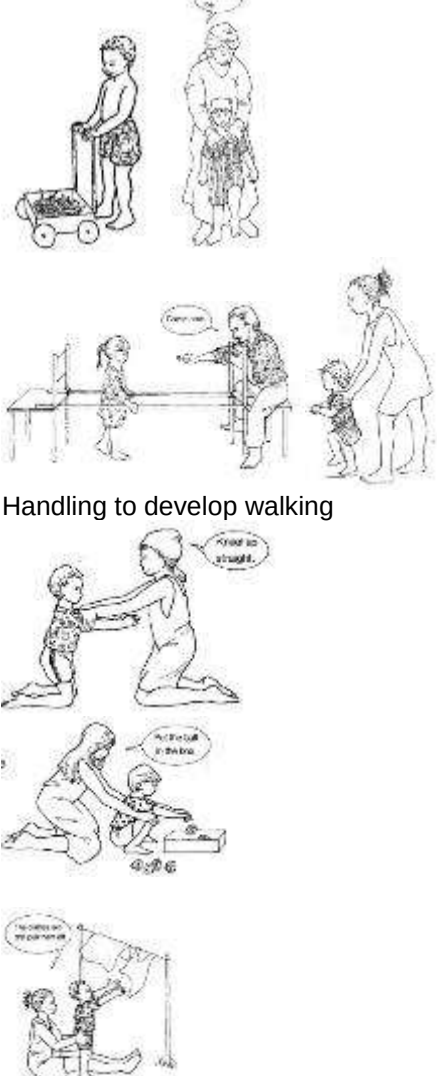
Unresponsive	Increased responsiveness	Listens and begin to understand/ respond	Communicates through words and gestures
		• Talk about where things and people are  • Tell simple stories showing actions and objects where possible 	

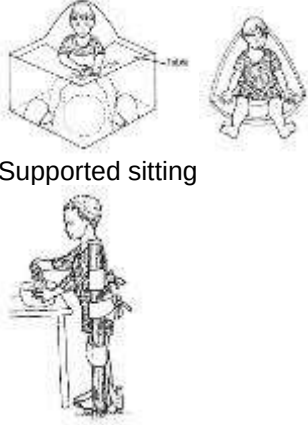
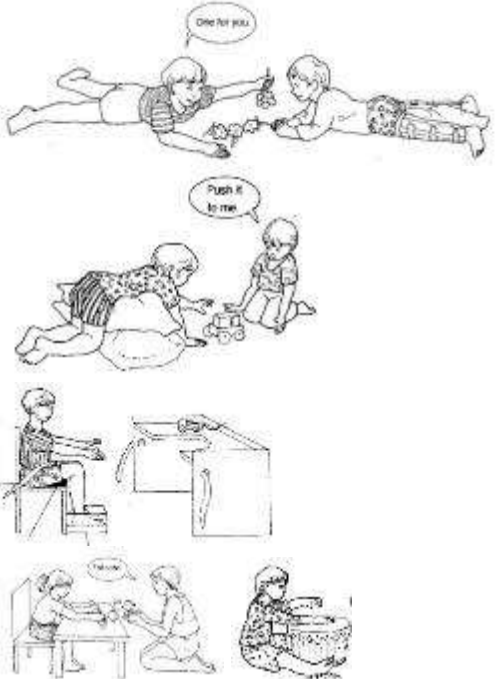
ANNEXURE 2:

PICTURE BASED OVERVIEW OF ATYPICAL AND NORMAL GROSS MOTOR FUNCTION

Physical abilities	- Transported in a manual wheelchair. - no independent movement; unable to maintain antigravity head and trunk postures	- Self-mobility with limitations; - May use powered mobility - limited floor mobility	- Experience limitations - Walks using a hand-held mobility device (age 2 – 4 independent floor mobility)	Walks without limitations
				
Skills to develop: Focus of activities: Activities to: - Prevent contractures - Promote comfortable handling - Position for ADL's - Position for play - Stimulate movement	Skills to develop:  Carrying Handling	Skills to develop: 	Skills to develop: 	Skills to develop: 

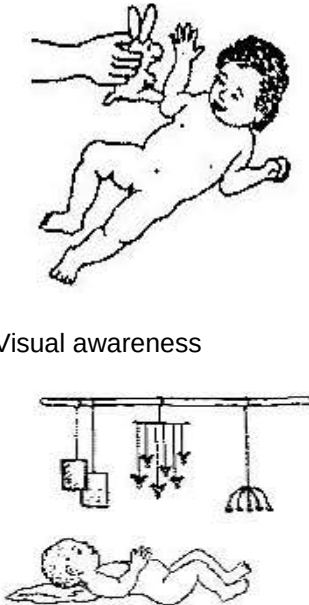
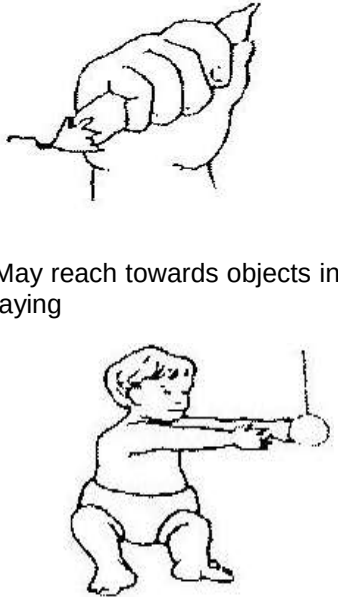
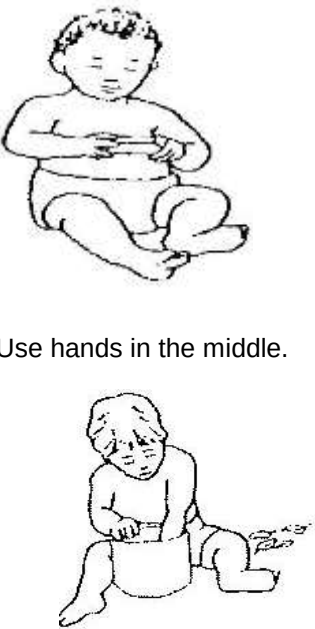
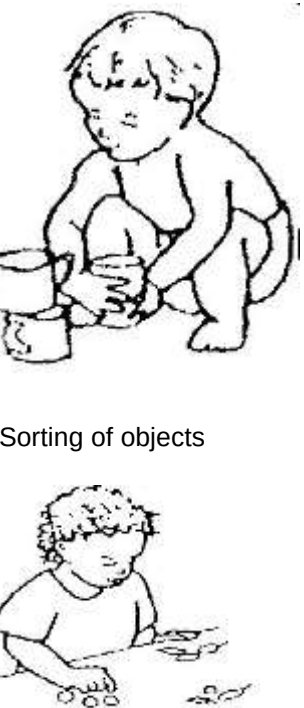
Physical abilities	- Transported in a manual wheelchair. - no independent movement; unable to maintain antigravity head and trunk postures	- Self-mobility with limitations; - May use powered mobility - limited floor mobility	- Experience limitations - Walks using a hand-held mobility device (age 2 – 4 independent floor mobility)	Walks without limitations
Develop postural endurance and physical skills	 Positioning Focus of activities: Position child to: - Prevent contractures - Promote comfortable handling - Position for ADL's - Position for play	 Carrying Handling to develop head control	 Handling to develop sitting Handling to develop standing	

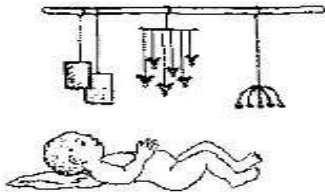
Physical abilities	- Transported in a manual wheelchair. - no independent movement; unable to maintain antigravity head and trunk postures	- Self-mobility with limitations; - May use powered mobility - limited floor mobility	- Experience limitations - Walks using a hand-held mobility device (age 2 – 4 independent floor mobility)	Walks without limitations
		 Positioning  Rolling	 Handling to develop walking Handling to develop balance	

Physical abilities	- Transported in a manual wheelchair. - no independent movement; unable to maintain antigravity head and trunk postures	- Self-mobility with limitations; - May use powered mobility - limited floor mobility	- Experience limitations - Walks using a hand-held mobility device (age 2 – 4 independent floor mobility)	Walks without limitations
		 Supported sitting Supported standing Focus of activities: Stimulate active voluntary movements Position child to: - Prevent contractures - Promote comfortable handling - Position for ADL's - Position for play and communication - Develop head and trunk control	 Positions for play Focus of activities: - Participate in physical activities - Involvement in ADL's through maintaining postures - Attainment of physical skills and independence in ADL's	 Focus of activities: - Participate in physical activities - Attainment of physical skills and independence in ADL's

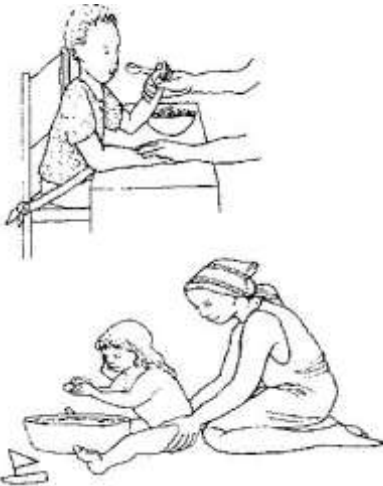
Annexure 3

PICTURE – BASED OVERVIEW OF FINE MOTOR DEVELOPMENT

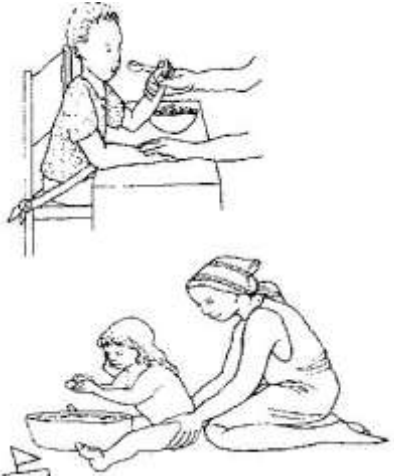
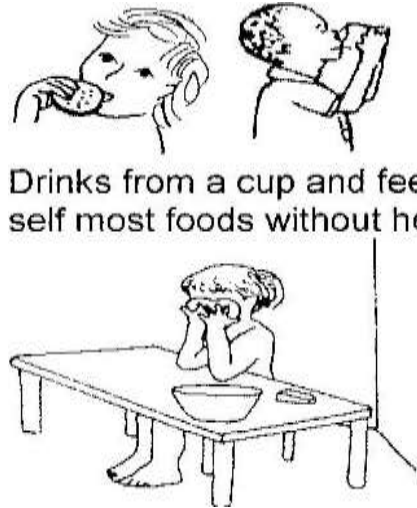
• The child has no active movement on his hands or fingers. • Does not reach or handle objects	• Begins to use hands • Holds objects when placed in his hand. • Use one hand	• More functional and appropriate use of both hands	• Hand function is developed
 Visual awareness	 May reach towards objects in sitting or laying	 Use hands in the middle.	 Sorting of objects

PICTURE – BASED OVERVIEW OF PERCEPTUAL SKILLS			
- The child has no active movement on his hands or fingers. - Does not reach or handle objects	- Begins to use hands - Holds objects when placed in his hand. Use one hand	More functional and appropriate use of both hands	Hand function is developed
Visual awareness 	Attention to self, people, objects Developing body awareness  	Imitates and copy people Points to objects  	Follow simple instructions and learns simple tasks. 

Annexure 4

PICTURE – BASED OVERVIEW OF SELF CARE SKILLS			
Fully dependent	Needs constant/ lots of support	Needs little support	Independent
			 Drinks from a cup and feeds self most foods without help 

PICTURE – BASED OVERVIEW OF SELF CARE SKILLS

Fully dependent	Needs constant/ lots of support	Needs little support	Independent
			 Drinks from a cup and feeds self most foods without help

SKILLS TO DEVELOP: SELF CARE

Encourage reaching towards objects with facilitation



Facilitate touch of different textures



Playing with different materials (water, sand, mud)



Playing with different materials (water, sand, mud)



Annexure 5: Example of a BASELINE SCREENING ASSESSMENT TOOL (David Werner – Disabled Village Children p.292 – 293)

DISABLED VILLAGE CHILDREN

Name: _____

Birth date: _____

Date: _____

EVALUATION OF A CHILD'S LEVEL OF PHYSICAL DEVELOPMENT

Note: Although on these guides physical and mental skills are separated, the two are often closely interrelated.

These charts show roughly the average age that a normal child develops different skills. But there is great variation within what is normal.

RECORD
SHEET
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(page 1)

PHYSICAL DEVELOPMENT	Average age skills begin	3 months	6 months	9 months	1 year	2 years	3 years	5 years	What to do if a child is behind
Head and trunk control	 lifts head part way up	 holds head up briefly	 holds up head and shoulders	 turns head and shifts weight	 holds head up well when lifted	 moves and holds head easily in all directions			Activities to improve head and trunk control (see p. 302).
Rolling		 rolls back to belly	 rolls back to belly	 rolls over and over easily in play					Activities to develop rolling and twisting (see p. 304).
Sitting		 sits only with full support	 sits with some support	 sits with hand support	 begins to sit without support	 sits well without support	 twists and moves easily while sitting		Work on sitting. Special seating if needed (p. 308).
Crawling and walking		 begins to creep	 scoots or crawls	 pulls to standing	 takes steps	 walks	 runs	 can walk on tiptoe and on heels	Activities to improve balance (see p. 306).
Arm and hand control	 grips finger put into hand	 begins to reach towards objects	 reaches and grasps with whole hand	 passes object from one hand to other	 grasps with thumb and forefinger	 easily moves fingers back and forth from nose to moving object	 throws and catches ball		Eye-hand activities. Use toys and games to develop hand and finger control (see p. 305).
Seeing	 follows close object with eyes	 enjoys bright colors/shapes	 recognizes different faces	 eyes focus on far object	 looks at small things/pictures	 Sees small shapes clearly at 6 meters (see p. 453 for test).			Have eyes checked (see p. 452). If poor, see Chapter 30.
Hearing	 moves or cries at a loud noise	 turns head to sounds	 responds to mother's voice	 enjoys rhythmic music	 understands simple words	 hears clearly and understands most simple language			Have hearing checked. If poor, see Chapter 31.

Birth date: _____

Date: _____

EVALUATION OF A CHILD'S LEVEL OF MENTAL AND SOCIAL DEVELOPMENT

MENTAL DEVELOPMENT	Average age skills begin	3 months	6 months	9 months	1 year	2 years	3 years	5 years	What to do if a child is behind
Communication and language	 cries when wet or hungry	 coos when comfortable	 makes simple sounds	 uses certain sounds for different things	 begins to use simple single words	 begins to use words together	 uses simple sentences		Speak and sing often to child. If needed, develop alternatives to speech (p. 313).
Social behavior		 smiles when smiled at		 begins to understand and respond to "NO!"	 begins to do simple things when asked	 likes to be praised after completing simple tasks	 interacts with both adults and children		Consider trying behavioral approach to social behavior (see p. 149).
Self-care	 sucks breast	 takes everything to mouth		 chews solid food	 begins to feed self	 drinks alone from glass	 takes off simple clothes	 toilet trained	Encourage child to help self if possible. Use behavioral approach to learning (see p. 350).
Attention and interest		 smiles when smiled at	 brief interest in toys and sounds	 develops strong attachments to caretakers	 takes longer interest in toys and activities	 sorts different objects	 builds playthings with several pieces		Early stimulation activities (see Chapter 35). Provide toys and 'fun' objects.
Play	 grasps things placed in hand	 plays with own body	 plays with simple objects	 begins to enjoy first social games (peek-a-boo)	 imitates and copies people	 begins to play with other children	 plays independently with children and toys		Guided play, lots of stimulation and interaction with other children.
Intelligence and learning	 cries when hungry or uncomfortable	 recognizes mother	 recognizes several people	 looks for toys that fall out of sight	 copies simple actions	 points to things when asked	 follows simple instructions	 follows multiple instructions	Early stimulation (p. 316). Lots of toys, talk, and step-by-step training.

Put a circle around the level of development that the child is now at in each area.

Put a square around the skill to the right of the one you circled, and focus training on that skill.

If the child has reached an age and has not mastered the corresponding level of skill, special training may be needed.

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(page 2)

CHILD DEVELOPMENT

ANNEXURE 6

EXAMPLE: INTEGRATING THE LEARNING PROGRAMME INTO THE DAILY PROGRAMME	
ARRIVAL AND GREETING	
Greeting all – Using of appropriate greeting phrases and names of carers and friends. Packing out bag – bag in cubby hole/ lunch box out / message book on table. (<i>labelling of cubby hole and objects with photos and names</i>) Hanging up jackets/ taking off shoes and packing away shoes in own cubby hole; rolling up socks. <i>Constantly make children aware of their own names and their friends' names.</i> Structured free play 2 or 3 well marked play areas Dolls/house corner/ cars / blocks – <i>put container with blocks in a hoop so that child/ children can play sitting by the hoop and keeping all the blocks inside the hoop. This will support organising, packing up and general discipline.</i> Tip: Introduce activities through songs. – <i>this gives the children time to adjust/ get used to change. It gives children opportunity to participate in song/ actions. – use pictures indicating different routines and activities of the daily programme</i>	
SELF-CARE ROUTINES	
Washing hands Toileting Potty training - <i>training to be done in co-operation with parents – consistency in routine at home and school/centre.</i> Brushing teeth Methods to be used: • Interactive song (pictures and actions) - <i>to announce the activity that would follow</i> - <i>to prompt sequence of the routine activities</i> - <i>Sing songs very slowly to give opportunity for slow response.</i> - <i>Songs can be repeated a few times to enhance memory and participation.</i> • Interactive story (pictures and actions) of each self-care routine - <i>to prompt sequence/steps of the routine activities</i> - <i>to enhance proper self-care routines; independence and basic hygiene.</i> • Discussions of self-care routines (<i>using pictures/ objects/ story and songs</i>) • Practice/ participate using the correct steps while prompted by carer/pictures/song. <i>Daily supervision and observation of self-care routine activities are important to enhance development of independence and basic hygiene.</i> (See Learning Programme section on specific self-care routines and activities)	

BREAKFAST / SNACK / LUNCH TIME

Laying table: placemats with photos of each child

Work chart with children's photos and name to indicate who's turn it is to lay the table; hand out food; clear the table; wipe the table; etc. Discuss work chart daily.

Methods to be used:

- Interactive song (pictures and actions)
 - to announce the activity that would follow
 - to prompt sequence of activities
- Interactive story (pictures and actions)
 - to prompt sequence/steps of the activities
 - to enhance independence and basic manners
- Discussions of healthy foods/ table manners (*using pictures/ objects/ story and songs*)
- Practice/ participate using the correct steps while prompted by carer/pictures/song.

Making of own sandwich with repeating short sentences, choices and actions. (pictures and objects)

For example: Spread the butter/marmite/ jam/ peanut butter

Correct use of utensils (knives, forks) – spreading/cutting; eating with fork

Matching/ sorting of utensils/ food (objects/pictures)

Identifying food/ spreads on bread by naming/ showing / tasting / smelling

Awareness of colours, shapes and counting

MORNING RING

Interactive Greeting song: (actions + photos)

– greeting game/song using photo cards – child responds to photo shown and name called

Story: (*See story time section*)



Interactive story

Weather

Discuss weather – let the children look out the window/go outside to look

Use pictures and put up on the daily chart (have additional matching and sorting picture cards to play a game)

Days of the week

Song and week chart to show sequence of the week

Discuss today/ yesterday/ tomorrow

Link the day to specific events (Sunday = church/ Monday = horse riding)

Sequence of the daily programme

(use separate pictures for each activity or routine)

Discuss the sequence of the daily programme – as activities are about to change to the following activity – make the children aware of it by singing a song/ using a rhyme or asking 1 child to show on the chart what activity is to follow.

GROSS MOTOR

Always model and discuss gross motor activities before expecting the child to perform.

Ask the child to tell what is expected of him, once the instructions are given – to ensure child understands, but also to enhance language.

Play games that include their names –“rotten egg” – nominate a child, and give him a photo of a friend. He runs round the circle and gives photo to correct child, etc.

Exploring body movements and body in spatial relationships – build an obstacle course use outside jungle gym – as the child is climbing on/ in or out of obstacles, say what the child is doing and have the child repeat it.

Hopscotch – draw a hopscotch activity on the floor with chalk, but instead of only circles draw different shapes/ colours – give instructions using shapes/ colours and counting. For example, “jump only in yellow squares; How many yellow squares are there”

Music and movement – copying of movements/ actions to songs/ music is always fun and most children enjoy participating

TIP: use gross motor activities in between other activities as ways of motivating better participation/ enhancing children to focus/ give better attention.

TABLE / MAT ACTIVITIES (FINE MOTOR/ Sensory)

Correct positioning at table/ mat

On carpet – use hoops to help organise – put blocks in hoop – the child must ensure that blocks stay in the hoop and is not scattered around.

Depending on time available 1 – 3 activities – children can be divided in 2 or 3 smaller groups

Do activities with the child/ group and use as opportunities to discuss, ask questions

Create awareness of colours/ shapes/ counting/

Use opportunities to match/ sort/ count/ identify

Integrate activities with theme/ discussions: for example when discussing and talking about healthy foods - do a craft activity with paint/ paper tearing and making mosaic fruit pictures

- make fruit when playing with clay
- look for pictures, identify, cut and paste – fruit collage
- Making of picture books – paging through and discussing of picture books

(see section on fine motor activities in Learning Programme)

STRUCTURED FREE PLAY

Play with the children and use time to observe their interactions with their peers

Create opportunities for exploring and participating in activities/ games that will enhance development of skills to match; identify; name colours/ shapes.

Play counting games while building blocks; playing with cars or playing house – making tea.

Use hoops to organise play areas

Create opportunities for interaction with one another

Some children might need support in learning how to play (building a tower)



Water play – bathing a doll – naming body parts; discuss steps to bathing

MUSIC RING

Music and movement

Learning to listen to music and respond through dance and movement

- copying movements of carer/ others
- react to specific music in specific ways – soft music = walk on toes / loud music = loud stomping of feet

Musical instruments (bought and home-made)

Can be made during fine motor craft activities – shakers / drums/ etc.

Listens to music and use instruments appropriately – stops when music stops.

Picture cards of the instruments – child may only use his instrument if the picture is shown – stops if picture is put down and waits for his turn when picture is shown again.

STORY TIME

Short story with 1-liners per picture. (A-5 pictures/ A4 or A-3 pictures)

Make use of interactive storytelling with actions/ sounds/ catchy phrases and prompt participation.

Ask questions and repeat story.

Prompt children to copy the storytelling.

Provide opportunities for children to tell the story with the use of the pictures.

Keep it short, repetitive sentence/phrase, about things they understand.

Ask questions about colours/ counting/ identifying things in the pictures

Tell the same story for a week/ two weeks to give children the opportunity to learn the story and copy the storytelling.



ANNEXURE 7

TOPIC: MY BODY- IDENTIFY BODY PARTS

(GENERIC/ INTERACTIVE GROUP)

COMMUNICATION	VISUAL PERCEPTUAL AND COGNITIVE SKILLS	LIFESKILLS
Listening and Speaking • stories; • songs; • listen and respond • participate (actions/words) • talk about object, pictures, topic	• perceptual skills • figure ground • spatial skills • matching • counting • colours • rhymes	• sensory skills • gross and fine motor skills • social and emotional skills • self-care and activities for daily living (ADL)
MATERIAL: • objects, photos, pictures (posters/flashcards) • Mirrors (full length fixed to classroom wall and small handheld mirror) • Theme/topic table (in discussion area) • Dolls (baby dolls with clothes that can be removed) • Poster size picture of boy and/or girl (can be purchase or homemade) • Separate laminated face pictures (A4 size) + flashcards made from the mouth, ears, eyes nose, hair (set of 4 – 8) • Flashcards of face parts and body parts (photo cards work best for awareness and transitional groups/ picture cards can be used for the interactive groups. • Face and body puzzles (see text box for instructions) • Play dough • Bath time/body parts story		
INTRODUCTION: (Story/ song/ game) • Song: Hockey Pockey (adapt the song (words and tune) to suit the needs of the group) • Use a hoop or draw a big circle with chalk on the floor. The children stand around the circle facing one another. • Use actions that the children must copy • Sing the song very slowly to give the children the opportunity to copy the song and actions. • The length of the song should depend on the age and level of functioning of the group and can be adapted accordingly.		

Song:

I put my 1 foot in (place foot in circle) (left and right are difficult concepts, even more so when all are standing in a circle)

I take my 1 foot out (take foot out of circle – wait for children to respond)

In and out. In and out. In and out (put foot in and take out - wait for all to copy actions)

Turn around and around and around and around (place arms in the air while turning slowly till all is facing the middle of circle again).

1 – 2 – 3 – 4 – 5 clap, clap (clapping hands while clapping)

Ohhh hocky pocky (hands on hips; shake body/bum while bending forwards and up again) x 3

1 – 2 – 3 – 4 – 5 clap, clap (clapping hands while clapping)

Repeat from beginning, but replace body part with similar (other foot or both feet – then jump in and out) or different body parts namely hand/head/bum.

DISCUSSION TIME

Vocabulary:

- Initially - eyes, nose, mouth, ears, hair.
- Extended to – body parts: hands, arms, feet, legs, fingers, toes, stomach, back, bum
- Children are seated on the carpet/chairs in a semi-circle in the discussion area facing the carer.
- Show and tell children to put hands on their heads/ hands on their knees/ hands in their laps. (this will help control the group and get them to pay attention.)
- Ask children to point to their nose, “show me your nose” – *carer should not point to her own nose at first, but wait for the children to respond first.*
- Once most have pointed to their noses, say “yes that is your nose” - carer can touch 1 child’s nose.
- Call a child that is not paying attention, or is pointing to the wrong body part to watch, “Jamie, look, this is my nose, come show me your nose” – in order to get the child’s attention and encouraging the child to copy the action. Give support where needed. (See table for different awareness-, transitional- groups). A mirror can also be used, where the child and carer look in the mirror while touching the mentioned facial part. A full mirror is needed for whole body!
- Repeat with other body parts.
- This is followed by asking children to name body parts: 1 child can stand in front facing group while carer touches a body part and ask the group/individual to name the body part.
- Use doll so that child can show body parts on the doll.
- Children functioning on a higher level can move on to showing body parts on the poster on the wall.

TABLE ACTIVITIES/ FINE MOTOR (1 to 2 per day)

Main activity:

Clay and Laminated A4 face picture:

- Children roll out small clay balls in hands.
- Roll out snakes/long strings with clay
- Ask child to put clay balls on specific face part, i.e. eye
- Then press clay ball down flat with thumb.
- For mouth and hair use clay strings.
- (hand over hand support where needed)

Pasting face part pictures on A4 picture of face.

Sub-activities:

Face puzzle

- Let child place body part puzzle piece on the A4 picture of the face (can put clay or sticky press balls on the back of the pieces to stick to laminated picture.

Sorting flash cards of body parts

CONSOLIDATION:

Consolidation can be integrated in various activities within the Daily Programme such as :

- Music and movement -
- Game (i.e. "Simon Says" – during gross motor activity or outside play later in the day.
- Story telling – use 1 story for at least 4 weeks in order for children to remember the story and tell the story along with the carer.

Story: Bath time

- (own stories can be created)
- Tell story of bath time showing pictures of washing a specific body part.
- As story is being told – children act out the washing of the specific body part.
- Use short repetitive sentences, in order for the children to start telling the story with the carer and to anticipate next part.
- For example: Bath time story

Dad calls Amy; "come Amy, time to take a bath!"

Amy: "I wash my face' (*all copy washing of face*) – *picture of girl washing her face.*

Story continues with Amy saying what she is washing next (*All copy each body part being washed*) – *but not feet!*

End of story: Dad says: " your face is clean", (*touch face*) – *continue with all body parts in the same way* "but your feet are dirty!!!!!" (*show shocked reaction on face when looking at feet*). Amy forgot to wash her feet!!

ADAPTED: AWARENESS GROUP

1. only face – with full hand-over-hand support making use of the mirror (eyes/nose/ears/mouth/hair) (see LP: Communication and Language: Listening and understanding:
2. add hands, feet, legs and arms
3. add stomach, back, bum

ADAPTED: TRANSITION GROUP

Same as awareness group, but with hand-over-hand support only as needed (less support)

INTEGRATION IN DAILY PROGRAMME:**Self-care routines:**

- Toileting – talk about body parts during toilet routines (“show me your foot”/ “what is this?”
- Brushing teeth – look in mirror and discuss face (show me/ what is)
- Washing face/hands
- dressing/undressing – taking of jacket/ shoes and socks during the day

Gross motor activities :

- indoor/outdoor play with various objects (balls/bean bags/swings/obstacle course) – talk about what he/she is doing with his body in relation to objects. I.e. kick the ball with your foot; crawl on your knees under the table; lie on your back.

Social skills:

- privacy when dressing/using toilet (private parts)
- keeping safe (no touching of private parts by others – see LP – Life skills: Sense of self and sexuality)

- The same topic and sub-topic should be repeated on a daily basis for at least 4 – 6 weeks to give children more opportunities to develop skills, learn and apply knowledge.
- As children progress sub-topics can be added/extended.

ANNEXURE 8

RESOURCE LIST: Organisations (NGOs) delivering services, training and resource material for children with SPID

Organisation	Telephone	e-mail	City/ Province	Programme
Children's Disability Centre Director: Nerina Nel	011 484 1908	info@cdcjhs.co.za	Johannesburg Hospital	Fishbowl Training Programme
Sunshine Organisation (NPO)	011 642 2005 011 642 2008	info@sunshine.org.za	Johannesburg Hospital	START programme
Pathways-Pretoria (NPO)	082 5789 117 082 866 7112	pta-pathways@mweb.co.za Danita Nel	Pathways centre 690 Rankdoring Moreletta Park Pretoria	Pathways programmes (resource material and training) Stimulation kits and manual
Western Cape For Intellectual Disabilities (WCFID)	021 5104686/7	Director.wcfid@kingsley.co.za Tessa Wood	Alexandra Hospital Cape Town	Resource material and training workshops
Cape Mental Health Organisation (CMH)	021 5104 686/7	learning@cmh.org.za	Cape Town	Learning for Life training programmes
Uhambo Foundation	021 797 8239	info@uhambofoundation.org.za	2 Hemyock Road Plumstead, Cape Town	Stimulation kits and manuals
Cerebral Palsy Association Eastern Cape (EC)	041 583 2130	info@cerebralpalsy.org.za http://www.cerebralpalsy.org.za info@hambisela.co.za www.hambisela.co.za	54 Admiralty Way, Port Elizabeth, 6001 Eastern Cape	Hambisela Programme for children with CP (training for carers)